



PARLIAMENT OF TASMANIA

TRANSCRIPT

HOUSE OF ASSEMBLY

ESTIMATES COMMITTEE A

Hon. Jeremy Rockliff MP

Monday 5 June 2023

MEMBERS

The Chair of Committees (Chair)

Mr Young (Deputy Chair)

Ms White

Ms O'Connor

OTHER PARTICIPATING MEMBERS

Dr Broad

Ms Dow

Dr Woodruff

IN ATTENDANCE

Hon. Jeremy Rockliff MP, Premier, Minister for Health, Minister for Mental Health, Minister for Tourism, Minister for Trade

Ministerial Office representatives

Vanessa Field	Chief of Staff
Justin De Domenico	Deputy Chief of Staff
Amy Hills	Principal Adviser
Sandy Wittison	Principal Adviser
Andrew Johnson	Senior Adviser
Ned Whitehouse	Senior Adviser
Rosita Gallasch	Adviser
Megan O'Brien	Adviser

PREMIER

Department of Premier and Cabinet representatives

Jenny Gale	Secretary
Craig Limkin	Associate Secretary
Todd Crawford	A/Deputy Secretary, Policy and Delivery
Noelene Kelly	A/Deputy Secretary, People, Performance and Governance
Amanda Russell	Director, State Service Management Office
Justin Thurley	Chief Information Officer
Todd Babiak	CEO, Brand Tasmania

Department of Communities representative

Mel Gray	Deputy Secretary, Wellbeing, Strategy and Engagement
----------	--

HEALTH PORTFOLIO AND MENTAL HEALTH PORTFOLIO

Department of Health and Department of Mental Health representatives

Kathrine Morgan-Wicks	Secretary
Shane Gregory	Associate Secretary
Tony Lawler	Chief Medical Officer and Deputy Secretary Clinical Quality Regulation and Accreditation
Dale Webster	Deputy Secretary Hospitals and Primary Care
Sonj Hall	Deputy Secretary Community Mental health and Wellbeing
Jordan Emery	Chief Executive Ambulance Tasmania
Craig Jeffery	Chief Financial Officer
Michelle Searle	Chief People Officer
Francine Douce	Chief Nurse and Midwifery Officer
Mark Veitch	Director of Public Health
Lisa Howes	Director Office of the Secretary
Jessica Jordan Hoverman	Manager Office of the Secretary

TOURISM PORTFOLIO

Tourism Tasmania representatives

Sarah Clark	Chief Executive Officer
Mark Jones	Chief Operating Officer
Edwina Morris	Director, Office of the CEO

Department of State Growth representatives

Kim Evans	Secretary
Mark Bowles	Deputy Secretary, Business and Jobs
Jenna Cairney	Executive Director, Business Tasmania
Angela Conway	A/Deputy Secretary, Business Services
Claire Fitzgerald	Director, Tourism and Hospitality Support

TRADE PORTFOLIO

Department of State Growth representatives

Kim Evans	Secretary
Mark Bowles	Deputy Secretary, Business and Jobs
Lara Hendriks	Executive Director, Trade
Angela Conway	A/Deputy Secretary, Business Services

The Committee met at 9 a.m.

CHAIR (Mr Tucker) - Welcome everyone. The time scheduled for Estimates for the Premier is three hours. We will take a short break at 11 a.m. for morning tea. Members will be familiar with the practice of seeking additional information, which must be agreed to be taken by the Premier and questions handed in writing to the secretary. I remind you that microphones are sensitive so I ask you to be mindful of Hansard and be careful when moving your folders, documents and water glasses around the table. Also, it is difficult for Hansard to differentiate when people are talking over each other, so I ask that members speak one at a time to assist with this, please.

I won't be keeping time on the questions and answers, but I will be keeping an eye on the questions asked to be kept to less than one minute, and that answers are less than three minutes.

Premier, would you like to introduce the people with you at the table and make a brief statement?

Mr ROCKLIFF - Thank you very much, Chair, and thank you for the opportunity to be here with the Committee today. I have Ms Jenny Gale, the Secretary of the Department of Premier and Cabinet and Mr Craig Limkin, to my left, the Associate Secretary of the Department of Premier and Cabinet. I have a short opening statement.

The Department of Premier and Cabinet (DPAC) is the central agency of the Tasmanian Government and the department provides a broad range of services to Cabinet members of parliament, government agencies and the community. It works closely across the state sector, the community, local government, the Australian Government and other state and territory governments and, in particular, it supports me in attendance of National Cabinet.

The Government's key priorities in the 2023-24 Budget focus on building a strong economy and a safe and caring community. It is a carefully considered Budget and I want to quickly outline some of the achievements in the past year within my portfolio and inform the committee on some of the commitments made in the 2023-24 Budget to deliver our Government's plan to keep building Tasmania's future.

Most importantly, is our Government's response to the Commission of Inquiry into the Tasmanian Government's Response to Child Sexual Abuse in Institutional Settings. the commission is expected to make wide-ranging recommendations by 31 August 2023. Our Government is committed to adopting the recommendations of the commission of inquiry's final report.

We have not waited for the commission's final report to make improvements where it is clear from the hearings that have taken place that they need to be made. We are already progressing 30 interim responses and publicly reporting their progress on the DPAC website.

The Budget provides a down payment of \$30 million for urgent actions in the 2023-24 financial year in response to these 30 actions, and in preparation for the final report of the commission of inquiry.

PUBLIC

The Government is also progressing our commitment to a Wellbeing Framework which is a first whole-of-Government framework for Tasmania. We are inviting Tasmanians from every walk of life to tell us what 'wellbeing' means to them, because we know wellbeing means different things to different people.

Our Government is also developing a Sustainability Strategy for Tasmania, which will assist us all in thinking about and making decisions for the future. We will have more to say about that strategy later this year.

The Department of Premier and Cabinet also delivers our whole-of-Government, comprehensive Child and Youth Wellbeing Strategy, *It takes a Tasmanian Village* - another first for our state - which we launched in 2021. We are now well into the second year of the strategy and there has been significant progress across all of the action areas. I look forward to updating the committee today on recent milestones and action highlights.

This year, DPAC has also supported many flood-impacted communities devastated by the October 2022 floods. The department manages, coordinates and administers recovery and resilience activities and funding. I can also provide an update today on the significant work undertaken supporting those communities.

DPAC has also provided our Government with legislation to restore the House of Assembly from 25 to 35 seats, to come into effect at the next state election.

The Budget includes \$3.39 million allocated in 2023-24 and 2024-25 to support the procurement process and construction works associated with the office accommodation fit-out works within the Salamanca building to accommodate an additional 10 members.

As members know, during 2022, the Anti-Discrimination Commissioner Sarah Bolt undertook a review to examine workplace culture in the Tasmanian ministerial and parliamentary services. Our Government took a strong stance on the importance of getting this right because, as a parliament, we must lead by example. The Budget provides funding over two years of over \$740 000 for an independent project manager to implement recommendations arising from the *Motion for Respect: Report into Workplace: Culture in the Tasmanian Ministerial and Parliamentary Services*.

Lastly, the Department of Premier and Cabinet is supporting governments to establish strategic regional partnerships to support and work in collaboration with Tasmania's regions to plan for the infrastructure and services they need to thrive and attract investment for sustained prosperity and growth. On the west coast, it was announced earlier this year as the first council area we'll be partnering with. I look forward to providing an update to the committee today on this work.

Ms WHITE - Good morning, Premier, I wanted to ask about the motion the parliament agreed to where it ordered the Government to table documents and advice in relation to the stadium and the AFL agreement. There are a number of matters that we regard to be outstanding in relation to that. Can you explain why you didn't share information provided to the Government from Treasury, from the Department of State Growth, from Macquarie Point Corporation, TasWater, the RSL and others, who, no doubt, would have been feeding into the process around your decision to locate a stadium at Macquarie Point?

PUBLIC

Mr ROCKLIFF - As I said very clearly last Thursday night, Motion 125 was supported by myself and our Government in the interest of providing the required information we can to the House within the time frame. The minister for Sport, Nic Street, tabled a document pertaining to Item 1 of the motion on that day. Since then, departments have been undertaking a search to provide Government with documents pertaining to Item 2.

On Thursday, to the best of my knowledge, I tabled all documents provided to me that were identified as part of the search. The documents provided to House were the documents that were provided by departments. What I was able to provide the House was the briefings that pertained, what I was unable to provide the House were the briefings that pertained to Cabinet in-confidence and discussions. And, of course, you would understand that.

We spoke about the RTI, as referenced by yourself, and the RTI references in an email between departmental personnel. Emails between departmental personnel are frequent, commonplace and do not constitute formal advice to Government. This is an important distinction.

I've committed to transparency. I delivered that when I delivered the AFL agreement. I'm delivering what I promised in parliament as well. As I said, in terms of the confidential or in-camera briefings, we are seeking advice from the Clerk and looking to organise those at the earliest opportunity, and look forward to that. To the best of my knowledge, all those documents were tabled by departments.

Ms WHITE - Premier, did you receive Treasury advice about the proposal to both sign the agreement with the AFL and to construct a stadium at Macquarie Point?

Mr ROCKLIFF - I'm not going into Cabinet -

Ms WHITE - I'm not asking you to. I'm asking you to explain if you received advice.

Mr ROCKLIFF - I'm not going into any Cabinet deliberations. We've worked through this very methodically as a Cabinet. I will not be discussing any Cabinet deliberations in this committee.

The Budget has the stadium investment there, as well as the AFL team. So clearly, we have been in discussions at a Cabinet level around this matter. The confidentiality of Cabinet is a long-established precedent and I won't be breaching it. I have released the agreement and the agreement is there for everyone to see. I've been very open and transparent about the investment that we're making and I will continue to be.

Ms WHITE - I'll ask you again, Premier: has your Government received advice from Treasury in relation to both the AFL agreement and the stadium at Macquarie Point?

Mr ROCKLIFF - Well, I'm not going to go into in Cabinet deliberations -

Ms WHITE - I'm not asking you to do that.

PUBLIC

Mr ROCKLIFF - I'm not going to go into Cabinet deliberations. Of course, we've had discussions with Treasury given we have the investment in the Budget.

Ms WHITE - Have you received advice from Treasury in relation to the AFL agreement that supports you signing a document like that?

Mr ROCKLIFF - I am not going into any matters pertaining to in-Cabinet discussions.

Ms O'CONNOR - Why can't you just say you received advice from Treasury?

CHAIR - Order.

Ms O'CONNOR - If you didn't that is a problem.

Mr ROCKLIFF - We went through a budget process which very clearly outlines the investment we are making in the AFL team and Macquarie Point Stadium. We are in the very early stages with Macquarie Point Stadium. We have only recently received the funding commitment from the federal government. Treasury, as has been pointed out, names the stadium as a budget risk.

Ms WHITE - It does. The Treasury has named the stadium as a budget risk.

Mr ROCKLIFF - And others, of course.

Ms O'CONNOR - It's only public money.

CHAIR - Order

Mr ROCKLIFF - It's an investment, Ms O'Connor. We can argue and agree to disagree on where that investment should be placed. My view is very strongly that this is an investment that is there to grow the economy, support jobs, provide an opportunity and turn an area that has been underinvested in and desolate into something special.

Ms WHITE - Government has been in power for nearly 10 years, you have no one else to blame but yourself there.

Premier, I want to ask another question in relation to the advice that you withheld from the parliament last week. There was no documentation or advice shared from Macquarie Point, from the Department of State Growth, from TasWater or from TasPorts. Can you explain why there was no information shared with the parliament given all of those entities have a direct influence of whether the stadium can be built at that location?

Mr ROCKLIFF - I have tabled all the advice -

Ms O'CONNOR - No you haven't.

Mr ROCKLIFF - that I was advised to table with respect to departments to the best of my knowledge. I have done that. I tabled the first lot of documents in the morning and then came back in the afternoon with another document with the matters around the roofed

stadium. We are in very early stages at the moment. We have only just received the funding for the stadium, a Macquarie Point precinct chair. There will be a lot of work, a lot of scrutiny, a lot of planning. The Project of State Significance process we have committed to will also ensure a thorough level of scrutiny and planning.

Ms WHITE - We received a Right to Information document that shows in a briefing you gave to Cricket Tasmania in December last year there were a significant number of details that Macquarie Point Development Corporation shared with your Government about matters that needed to be resolved on that site. I have no doubt that information has been provided to you either in part or in whole. Why did you withhold that from the parliament?

Mr ROCKLIFF - I have not withheld anything from the parliament. You have been given an RTI. If you are unhappy with the responses provided by an independent RTI officer you have every right to appeal.

Ms WHITE - I am disappointed with your response to the parliament, which ordered you to provide those documents.

Mr ROCKLIFF - I have tabled all matters relating to departments to the best of my knowledge. If I haven't I will continue to table. To the best of my knowledge I have tabled all that advice.

Ms O'CONNOR - Premier, I am finding it quite hard to believe that your agencies, particularly Treasury and the Department of State Growth, did not provide minutes and briefing notes to the minister, therefore advice to the minister that fell outside the Cabinet process. Do you and your agencies fully understand the principles of a Westminster parliament where if the parliament says to the executive you will table these documents, it is on the Executive to meet the demand of the parliament.

My question to you is: are you saying that all departmental advice from Treasury and Finance, State Growth and Veterans' Affairs potentially is captured by Cabinet-in-confidence, or does it not exist?

Mr ROCKLIFF - Clearly through the budget process we've discussed the matters and investments we're making around AFL and indeed the stadium as well. It is very much in its infancy. We've only just secured funding from the federal government.

Ms O'CONNOR - Is it GST exempt?

Mr ROCKLIFF - We've just secured funding from the federal government for \$240 million for the stadium at Macquarie Point precinct, Ms O'Connor, and without that funding, which essentially came in late April, now the work starts.

Ms O'CONNOR - That is not my question, with respect.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - With respect to you, we have tabled all the advice to the best of my knowledge from departments, as instructed by parliament. I am very well aware of the

PUBLIC

Westminster system, Ms O'Connor, and we have been very open and transparent about this project. We have detailed the AFL agreement. It is out there for people to see.

As to the investments we're making, I was very upfront with you a number of times in terms of the annual investment we are making, the investment into the high-performance centre and our commitment around the \$375 million to the stadium itself to make this work. I've got nothing to hide. This is a very open and transparent process and I can't think of another project that's been so heavily scrutinised that has only really just been secured funding so it can go ahead.

Ms O'CONNOR - Thank you, Premier, but you didn't go to the question. This is a folder of the materials you tabled on Thursday and there is no advice in there from departments, and many of them are on the public record -

Mr ROCKLIFF - Which highlights our transparency.

CHAIR - Premier, please let Ms O'Connor ask her question.

Ms O'CONNOR - A number of them are reasonably heavily redacted. All of the advice that would have come to ministers in the form of minutes and briefings would have gone to you, they would have gone to the Treasurer, the Minister for State Growth and the Minister for Veterans' Affairs, at a bare minimum. There would be multiple departmental minutes and pieces of advice and they can't all be captured by Cabinet-in-confidence, yet not one piece of departmental advice was tabled on Thursday. How do you explain that?

Mr ROCKLIFF - I have explained it, Ms O'Connor. I have tabled, to the best of my knowledge, all the advice I have been provided with and if there is more, I am happy to table that as well. As I say, it is to the best of my knowledge.

Ms O'CONNOR - Okay.

Mr ROCKLIFF - We have been dealing with stakeholders, Ms O'Connor, and you mentioned the veterans -

Ms O'CONNOR - They're not happy with you.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - There is a lot of work to do with stakeholders, including veterans, and I received some correspondence just the other day from veterans who do support the stadium or are at least open-minded to the stadium, and I think it is a huge opportunity. I have met with the veterans on site at the Cenotaph and looking at the various visual aspects and the like. I am happy to continue doing that and that will form an important part of the planning process, because a project of state significance works with the economic, environmental and social aspect of that planning process. No doubt the social aspect is very important to the local community members as well, but I see that as an opportunity.

Ms O'CONNOR - Premier, did you fully brief all of your ministerial colleagues on the licence deal and the proposed funding of the stadium before you signed that agreement?

PUBLIC

Mr ROCKLIFF - I'm not going to go into any discussions we've had with Cabinet.

Ms O'CONNOR - I am not asking you to talk about Cabinet. I am asking you if all your colleagues knew?

CHAIR - Please let the Premier answer.

Mr ROCKLIFF - We have discussed this a lot in Cabinet. My colleagues have been kept informed along the way. They are very supportive of the deal that we have signed.

Ms O'CONNOR - Are you sure about that?

Mr ROCKLIFF - Yes, I am. I am very sure all my colleagues are very supportive of the AFL agreement. It is a huge opportunity.

Ms O'CONNOR - The stadium.

Mr ROCKLIFF - The stadium? Oh, absolutely, very confident, because it is part of the deal. We can't have an AFL team without a stadium -

Ms O'CONNOR - We have two good stadiums.

Mr ROCKLIFF - Well I can go through a lot of reasons why -

Ms O'CONNOR - I read the paperwork, it's alright.

Mr ROCKLIFF - Did it convince you at all?

Ms O'CONNOR - Nope.

Ms WHITE - Premier, was the AFL deal formally considered by Cabinet.

Mr ROCKLIFF - Cabinet has been briefed along the way. I am not going into any discussions around Cabinet. It is a very important Westminster system. We have discussed a number of matters regarding the AFL agreement, stadium and high-performance centre within Cabinet. We will continue to do so.

Ms WHITE - The reason I ask is because one of the excuses you give for not sharing documents that were ordered by the parliament for you to share is that it has been deliberated on by Cabinet. You weren't able to clearly let me know if the AFL deal was formally considered by Cabinet. Surely that's a yes or no answer?

Mr ROCKLIFF - I am not going to go into any deliberations around Cabinet. I've kept all of my colleagues informed around the deal. It's a good deal for Tasmania, which was demonstrated the other day by John Olsen, chairman of Adelaide Crows, who called it the deal of the century and made a number of comparisons-

PUBLIC

Ms O'Connor - Another bloke in a suit from the mainland telling us what is good for us.

CHAIR - Please let the Premier answer.

Mr ROCKLIFF - Mr Olson put the deal into perspective in terms of the contributions the AFL are making.

Ms WHITE - Premier, you would understand why we need to understand this because you have had two members defect from your Government, citing a lack of transparency. I think it follows in a long history of your Government lacking transparency. When the former premier Peter Gutwein announced in his state of the state Address that he was building a floating stadium at Regatta Point, had that been a decision Cabinet had considered? Were you even aware of it before it was announced?

Mr ROCKLIFF - Sorry, going back to the Regatta Point stadium.

Ms WHITE - When the former premier announced that in his state of the state Address, was that the first time you had heard of it?

Mr ROCKLIFF - I will have to go back and recall the conversations I had with the former premier around the stadium as presented in that particular time -

Ms O'CONNOR - He really dropped you in it.

Mr ROCKLIFF - As Premier, when it came to the Regatta Point stadium, I wanted to see if there was an opportunity -

Ms WHITE - That wasn't my question. My question was whether the first time you'd heard of it was when premier Peter Gutwein announced it in his state of state Address.

Mr ROCKLIFF - I have had lots of conversations with the premier. I am not going into matters around Cabinet. The state of the state Address is discussed in Cabinet.

Ms White - Is it?

Mr ROCKLIFF - Referred to in Cabinet. Usually it is delivered the day after we have a Cabinet meeting. A number of those matters would be addressed there. I am not going to go back and talk about my predecessor's stadium announcement. What I am interested in is the fact that we have worked out a way to have a stadium, not at Regatta Point. That floating stadium could well end up being very costly. What I have done is take another look at Macquarie Point to see if it's possible to fit it there. It does.

Ms O'Connor - Just.

Ms WHITE - Is that not just a simple yes or no - was the AFL deal considered by Cabinet?

PUBLIC

Mr ROCKLIFF - I am not going to go into any Cabinet matters regarding the decision-making process. I won't disclose that.

Ms O'Connor - You can say yes or no.

Mr ROCKLIFF - What I can say is that the stadium and the AFL investment is in the Budget.

Ms O'CONNOR - Only part of it.

CHAIR - Order, Ms O'Connor.

Mr ROCKLIFF - We included that in the Budget. Clearly, we've discussed the stadium and the AFL agreement in some detail -

Ms WHITE - But that's after the fact.

Mr ROCKLIFF - because it's in the Budget.

Ms WHITE - Yes; but that's after the fact, because you'd already signed the agreement. My question is whether it went through the Cabinet before you signed it.

Mr ROCKLIFF - No, we've prepared the Budget, and it is quite a lengthy preparation exercise. We don't just put in numbers the day before it's released. We're fully aware of the discussions in Cabinet about the stadium, as well as the investment we're making to the AFL.

Ms WHITE - It sounds as though there is a lack of transparency about this - not only with the public, but also with your colleagues. The Chair of this committee defected from your party because of it. You can't deny that's a massive problem for you.

Mr ROCKLIFF - I've been open and transparent about the project when the information is ready to be released. It's a positive project; it's a project that we have fought for over many decades. The first four years is in the Budget, in terms of dollars, and it mentions residual in the fifth year as well; and there is a standard infrastructure project profile. I recognise the issues you raise about transparency, but we have agreement out there. The Budget is there; the numbers are there. I recognise that it's a controversial and much-talked about investment.

Ms O'CONNOR - Unpopular.

Mr ROCKLIFF - Stadiums around the country have been unpopular, Ms O'Connor, at this particular stage of their idea and construction; and then the community ends up coming on board when they can see the economic and social benefits. I anticipate that Tasmanians will see that benefit with this stadium as well, should it go through the appropriate planning processes.

Ms WHITE - One of the elements of the AFL Club Funding and Development Agreement that you failed to be transparent about is Schedule 6 - the Ground Occupancy Agreement. Is that available for us to look at yet?

Mr ROCKLIFF - My understanding is that those matters haven't yet been finalised, so it's still under discussion.

Ms WHITE - When do you expect that will be finished?

CHAIR - I call Mr Young.

Mr YOUNG - Thank you, Chair. Premier, given the commission of inquiry has delayed its report until the 31 August, you've previously said that the Government wouldn't wait before starting to make improvements. Can you outline the progress of work across the Government to keep children safe, and can you also outline how the recommendations from the commission's final report will be managed in implementation?

Mr ROCKLIFF - Thank you, Mr Young. No question is more important question, and there is no greater priority than keeping our children safe. That's why we established the Commission of Inquiry into the Tasmanian Government's Responses to Child Sexual Abuse in Institutional Settings. The commission has brought to light mistakes and failings which have occurred across multiple governments over time. We must ensure we hear, learn and, of course, we act so we never repeat those mistakes.

As you said, the commission is now expected to make its recommendations by 31 August 2023. Our Government is committed to adopting all the recommendations of the commission of inquiry's final report, but we have not waited for that final reports before making improvements. We are already progressing 30 interim responses, and we are publicly reporting their progress on the DPAC website so all Tasmanians can see clearly what we're doing, how we are doing it and the status of those initiatives.

I briefly mentioned that in my opening statement. For example, we committed to legislation to create a new crime of failing to protect a child or young person. Legislation was drafted and has passed both Houses of parliament, receiving Royal Assent on 20 April this year. It is now law in Tasmania. That's the level of detail we are providing in our transparent disclosure of our actions, and it will be ongoing.

Our Budget provides \$30 million to continue those 30 interim responses, which are, of course, actions in response to issues raised during the hearings of the commission and also to ensure there's opportunity for agencies to request further funds once the final recommendations of the report are released

Importantly, the secretary's board, which is all heads of agency across Government, has endorsed a new Keeping Children Safer structure, a dedicated unit within the Department of Premier and Cabinet which will provide whole-of-government oversight of actions in response to the commission of inquiry's final report, as well as policy support, monitoring and coordination. The commission is a once-in-a-generation essential opportunity to effect real change for children but it's the delivery of the commission's recommendations, it's putting in place the right systems, practices and cultures that will ensure a lasting change. As Premier, I'm very committed to ensuring that happens and the recommendations, we look forward to acting on once they're released.

PUBLIC

Ms O'CONNOR - Premier, when your Government was thrown into minority by the defection of two of your members, you said that you'd be reaching across the Chamber to make that work. There is absolutely no evidence of that yet. From the outside, and from the inside too, it looks like your Government is not coping with being in minority. We saw evidence of that on Thursday night in the House when it all fell apart. I will just remind you -

Mr ROCKLIFF - I don't need reminding, Ms O'Connor.

Ms O'CONNOR - in the four years of the Labor-Greens government we never lost control of the floor once. You've lost it twice in as many weeks. What are you going to do as Premier to provide stability in your changed circumstances? Will you be changing anything about the way you and your Government operate?

Mr ROCKLIFF - Well, you know, I can point to minority governments in the past -

Ms O'CONNOR - That didn't lose control of the floor twice in two weeks?

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - There is no doubt we are a minority. We have been a majority. But Tasmania is in a very good place. We've got 56 000 more jobs that we've created since 2014. Our unemployment is at an all-time low - half of what it was under the previous government. Job vacancies remain high and we're leading the nation in wages growth. That's the first time in a decade. So, our plan has delivered really fantastic economic conditions. We're investing in areas that Tasmanians would expect us to invest in in terms of those key services as well. But, of course -

Ms O'CONNOR - I'm more talking about how you'd operate-

CHAIR - Ms O'Connor, please let him answer.

Ms O'CONNOR - Thank you, I'm waiting for him to, Chair.

CHAIR - Well, please let him answer in silence.

Mr ROCKLIFF - I've seen where we had a previous member for Clark and the way-

Ms O'CONNOR - She was a handful.

Mr ROCKLIFF - and the way that matters were. All respect for Ms Hickey, we've been friends for a very long time, still get on very well and I enjoyed working with Ms Hickey. We had a number of decisions made in the parliament at that point of time at which the parliament came together and sometimes not so. Absolutely, my intention is to reach across the Chamber, working with both Mrs Alexander and Mr Tucker, and anyone else who wants to work with our Government for the betterment of our society and -

Ms O'CONNOR - Just to be clear though, when Ms Hickey was in the Chair, although she could be unpredictable, it -

Mr ROCKLIFF - This is a commentary. It's like reading editorial.

Ms O'CONNOR - it delivered nation-leading gender reforms. It was still technically a majority Government. Now you're in a minority government. Given the events of the first couple of weeks in parliament of your minority Government, I'm just wondering what lessons have been learnt and how you as Premier and your ministers might change the way you operate in order to deliver the stability that Tasmanians deserve, that businesses are looking for and that sort of thing?

Mr ROCKLIFF - All our ministers work with other members of parliament. I am often having discussions with other members of parliament of all political persuasions about various needs in the community and receive correspondence as well, which we act on. In supporting individual members of parliament to represent their constituents - and my expectation is that every minister takes that very seriously -

Ms O'CONNOR - A bit more humility, perhaps?

CHAIR - Order, Ms O'Connor.

Mr ROCKLIFF - I am not sure what you are getting at there. I am not sure if that is an attack on me for not being humble.

Ms O'CONNOR - I think you are a relatively humble person.

CHAIR - Order, Ms O'Connor.

Ms O'CONNOR - He was provoking a conversation.

CHAIR - Please, let the Premier finish his answer.

Ms O'CONNOR - You are a humble person, relatively.

Mr ROCKLIFF - I recognise where we are in terms of our Government. As Premier, I will be delivering stable government for the people of Tasmania. In order to achieve that, we will be working together to do that.

Ms O'CONNOR - Working together with who?

Mr ROCKLIFF - With all members of parliament.

Ms O'CONNOR - Okay.

Ms WHITE - I am going to conclude that the AFL agreement did not go through Cabinet before it was signed and, therefore, is not a Cabinet document and should have been provided in full when the parliament ordered you to provide that, along with other information that should have been provided.

Mr ROCKLIFF - You have the agreement.

PUBLIC

Ms WHITE - It is highly redacted. And there are other documents that are important for us to see as a parliament that you were ordered to provide that you failed to do. You are hiding behind Cabinet in-confidence and, frankly, I don't buy it.

Mr ROCKLIFF - I reject your assertion. Clearly, the Budget demonstrates that there have been long discussions in Cabinet, numerous discussions around the AFL agreement and the stadium, because it is in the Budget.

Ms WHITE - One of the things I wanted to understand is what modelling you rely on to say that your stadium is going to help pay for hospitals and schools? What work have you done to draw that conclusion?

Mr ROCKLIFF - It is enabling infrastructure, Ms White. When you invest in key infrastructure such as that, that supports jobs, that supports private investment or encourages private investment that grow the economy. People pay their taxes through greater consumption or through their land tax, or other areas of revenue-raising that governments do through a growing economy. You cannot have 4200 jobs in construction and the flow-on effects not benefit the local economy.

Ms WHITE - The original proposals for that Macquarie precinct, which you proudly put a press statement out on about last year, showed there would be 9000 jobs created with the original plan that had been agreed to by the Macquarie Point Development Corporation. You are, in fact, cutting in half the number of jobs that could be created on that site.

Ms O'CONNOR - That's what happens when you make stuff up.

CHAIR - Order, Ms O'Connor.

Ms WHITE - How do you feel about smashing 4000 jobs?

Mr ROCKLIFF - That's 4200 jobs in construction and -

Ms WHITE - Four thousand fewer than what you proposed if Macquarie Point had been developed under the alternative vision, according to your own press statement.

Mr ROCKLIFF - You criticised that previous vision as well, as I understand it.

Ms WHITE - You haven't provided any evidence of any modelling you are relying on to show that schools and hospitals will receive increased funding if you build a stadium. Have you got any information that you can share with the committee about what modelling or assumptions you are basing that on?

Mr ROCKLIFF - Ms White, of course, when you build infrastructure, roads, the Bridgewater Bridge, for example, it creates economic activity and provides an opportunity for a caring community and allows us to invest in those key services.

Ms O'CONNOR - It is called trickle-down economics.

CHAIR - Ms O'Connor.

Mr ROCKLIFF - You don't support economic growth?

Ms O'CONNOR - I don't support trickle-down economics.

CHAIR - Order, can you please let the Premier answer the question.

Ms WHITE - Premier, it was put to Mr Swain in the PAC committee the same question I have just put to you, and he said, 'There is no more more modelling that would show that'. So the question, so that I don't take anyone out of context here, was from the Chair, who asked if there was modelling that you can provide to the committee that backs up that statement you have just made publicly and now here for the committee, which is that the stadium will pay for schools and hospitals? Mr Swain said, 'What I've just talked about is the business case and its attachments, which are in front of the committee'. The Chair said, 'There is no more modelling that would show that?', and Mr Swain said, 'No, not that I'm aware of'.

That business case Mr Swain references shows that the tax receipts to government are equivalent to \$300 000 a year in payroll tax. That will pay for three nurses. It will not fund schools and hospitals. So what modelling are you relying on to make such baseless claims?

Mr ROCKLIFF - The business case is there for everyone to see.

Ms WHITE - It's a dog.

Ms O'CONNOR - It sure is.

CHAIR - Please let the Premier answer the question.

Mr ROCKLIFF - Come on, Ms White, really.

Ms WHITE - It is, it's a shocker. It loses money.

Mr ROCKLIFF - It doesn't. Honestly, it creates \$2.2 billion of economic growth over a couple of decades and supports private investment, employing people, which is great for the economy. You can't tell me we'd have more funding in our coffers if we sat down and twiddled our thumbs and invested in no infrastructure. Over the course of the last 10 years we've invested record amounts in infrastructure and have got record amounts of people employed.

Ms WHITE - Do you agree that that business case you're referring to shows that over the life of the project, so 20 years, it loses \$300 million?

Mr ROCKLIFF - What I recognise is that we are growing our economy by some \$2.2 billion. I point you to the fact that your own federal Leader, Mr Albanese, has said exactly what I've said around that investment of infrastructure and growing the economy.

Ms WHITE - In the precinct, which is about investing in other productive infrastructure like the port, housing and transport. We haven't got to those matters yet but I

PUBLIC

would love to know how you're going to fund those, because there wasn't a cent in the Budget for them.

Mr ROCKLIFF - Well, we've got the enabler for the stadium as part of the urban renewal and in terms of objectives, it is to make people's lives easier and more connected and maintain the character of our communities. It gives people more choice about where they live and move around. This is just the start of an enabling project of a significant investment, but it allows further investments, including housing, public transport, the Derwent ferry network expansion which has been invested in by some \$20 million, the rail corridor I've spoken about, the private investment that will come to feed off the stadium opportunity.

The information I have been given suggests that the stadium economy around the nation is some \$8 billion. We don't have part of that now but we should be having part of that. Why should Tasmania miss out on an opportunity like that, Ms White? We shouldn't.

CHAIR - I am going to give myself the call because I am allowed to enter into any questions.

Premier, can you provide the date when the decision was made to support the stadium by the Cabinet?

Mr ROCKLIFF - I'm not going to go into Cabinet deliberations, Chair. The Cabinet supports the stadium because we have a budget to suggest that we support the stadium. In developing the Budget it is very clear that the stadium is supported.

Ms O'CONNOR - Chair, I can tell you that at the very least a decision had been made in February last year because the former chief of staff to the former premier was commissioning consultancy works on the stadium in February 2022, some time before you persuaded us to join the bid.

Premier, I want to take you to the cost-benefit analysis final full report of November last year, which was tabled in the House on Thursday. On page 8, there is a redaction about life cycle capital costs and that is the ongoing annual capital costs of maintaining the stadium. There is only one redaction on this page and it is about the assumptions of the overall construction costs on government each year from 2029. Could you confirm that says '100 per cent'?

Mr ROCKLIFF - Look, I will of course attempt to answer as many questions as I can regarding this project and the Macquarie Point precinct. It is a matter for State Growth, however, and this is Department of Premier and Cabinet Estimates-

Ms O'CONNOR - This is your signature infrastructure project.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - I am sure the Minister for State Development, through the State Growth Estimates, would be more than happy to take a number of questions relating to this, but I have been very generous with my time in speaking about a range of matters -

Ms O'CONNOR - That's not the way it works.

Mr ROCKLIFF - Well, it is the way it works, because we are scrutinising the Department of Premier and Cabinet budget, Ms O'Connor, and I am happy to take questions on a whole range of matters. Look at all these briefs I've got on really important stuff. I'm more than happy to take questions on the responsibilities I have for the Department of Premier and Cabinet, but I am sure our minister would be more than happy to answer.

Ms O'CONNOR - Yes, but we ask these questions of you because you signed the deal with the AFL, so you can't fob it off to your Minister for State Growth.

Mr ROCKLIFF - Oh, come on.

Ms O'CONNOR - Also in this report is a large black box where it says 'cost-benefit analysis'. This is the report that finds that over 20 years there will be \$306 million in losses and your return on investment is 50 cents on the dollar in public funding. Do you know what that box covers up?

Mr ROCKLIFF - Well, it says cost-benefit analysis, Ms O'Connor. I am sure we can work through this when it comes to State Growth Estimates -

Ms O'CONNOR - You tabled these papers.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - In the interests of transparency I did. I have highlighted the agreement. There is a lot of work to be done now on this, Ms O'Connor. We've only just secured federal funding with the \$240 million and the work really starts now. It's important to recognise that fact and I look forward to progressing that work and the economic, social and environmental planning aspects will be worked through as well.

Ms O'CONNOR - I just think, in the interest of transparency, to have a whole page on the cost-benefit analysis blanked out and you are unprepared to say what might be under that is not very transparent, but moving right along because you are not going to, can you confirm that the \$240 million in Commonwealth funding for urban renewal, which you said earlier that the money had arrived, has there been a successful pitch to exempt that money from future GST distributions?

Mr ROCKLIFF - Can I just go back to your last question? Securing the AFL licence and project funding marks the beginning for what is a highly complex, multi-year project, similar to the Bridgewater bridge. After review and acceptance of a detailed business case, the development of the design section of the preferred contract and completion of the planning and approval process will all need to occur in parallel.

The next key steps are for the Macquarie Point Development Corporation to prepare a high-level precinct plan for Macquarie Point, identifying the key considerations and target outcomes from the development of the precinct, inclusive of a stadium. With the precinct plan prepared and the stadium's planning and design team assembled, work can then commence on developing both a detailed precinct master plan and a stadium master plan, the

latter to be based on a detailed functional brief which sets out the services the stadium needs to be able to deliver to different users. This will be developed into a preferred development option, which will be described in a reference design that satisfies the functional brief.

Ms O'CONNOR - That's not relevant to my question.

Mr ROCKLIFF - It is.

Ms O'CONNOR - It's not.

Mr ROCKLIFF - The reference design will provide the details necessary for progressing and planning approvals processes and subsequent procuring the main contract to develop the new stadium. This will enable commencement of an early works construction package at the end of 2025, a commencement of the main works in 2026. A comprehensive stakeholder consultation and communication plan will be developed and implemented at the beginning of these processes, to ensure that timely information is communicated, consultations undertaken and feedback received and considered throughout the development and delivery of the stadium.

As the Treasurer's letter says, further to our recent discussions - and a number of discussions have taken place. To answer your question on the 240 - no, we haven't received feedback from the Treasurer's letter as yet. About the GST exemption matter. We are still working through that.

Ms O'CONNOR - You referred earlier to the Bridgewater bridge but that's principally Commonwealth-funded. The implications for the state Budget, compared to the stadium; it's really an apples and oranges comparison. You won't tell us what was in the redacted section of the cost-benefit analysis; but can you perhaps tell us how concerned Treasury was about the projected cost of the stadium; how the funds would be procured; and the medium to long-term implications for the state Budget? That's not divulging Cabinet-in-confidence; that's your advice from Treasury.

Mr ROCKLIFF - Treasury has stated the Budget risks in the Budget papers, as well.

Ms O'CONNOR - But, you proceeded anyway.

Mr ROCKLIFF - They have a responsibility to highlight a number of risks and every budget, of course, has a risk. I've worked with budgets all my life on the farm, and some of the risks are drought and flood and all sorts of things which are budget risks. Falling commodity prices is an example.

Ms O'CONNOR - This is public money; it's not your farm's budget.

Mr ROCKLIFF - I'm just highlighting the comparison around risks. In every budget - irrespective of whether that's a business budget or a state budget - has an element of risk attached to it. The Bridgewater bridge is 20 per cent funded by the state Government so there are significant state Government funds into that; recognising, of course, the \$375 million that we are putting into the stadium, Ms O'Connor -

Ms O'CONNOR - And the rest of it, over time.

Mr ROCKLIFF - but, with this record health spend in this Budget, we will invest an average \$8.3 million a day; that's \$375 million we will spend and invest into health every 45 days. But, with the infrastructure into the stadium, we invest that once - so, that is a comparison. This is not tomorrow; that investment is over the course of the next five or six years. Our infrastructure investment, if my memory serves me correctly, with the Budget over the forward Estimates, was \$5.6 billion; it's now just under \$5 billion, so we're spending less on infrastructure, including the stadium.

Mr YOUNG - When you first became Premier, you said you were committed to developing a wellbeing framework. What will it look like and how is it being progressed?

Mr ROCKLIFF - Thank you, Mr Young. I appreciate your question and the interest in wellbeing, which is very important to me. When completed, this will be Tasmania's first whole-of-Government wellbeing framework. The reason we're doing it is to understand the factors that contribute to wellbeing for Tasmanians and to ensure that informs policy, Government priorities and Budget decisions. We are starting by inviting Tasmanians from every walk of life to tell us what wellbeing means to them, because we know it means different things to different people.

In partnership with the University of Tasmania, through its Institute for Social Change, I am pleased to announce today the launch of an online survey to hear from Tasmanians and to understand what is important to their wellbeing now and into the future. The development of the wellbeing framework will continue throughout much of 2023, with a set of draft domains for all consultation produced before the end of this year. Consultations will take all forms including with community partners like libraries, prisons, local councils and neighbourhood houses, so we can engage those who find it difficult to participate. This will enable all Tasmanians to have their say on what wellbeing means to them and their hopes for the future of our state. The statewide survey asks two simple questions about wellbeing and the future, and more information will be available at www.wellbeingframework.tas.gov.au

Our Government recognises that the prosperity of future generations is equally as important as present-day wellbeing, and with careful planning, we can have both. We anticipate we may hear things about physical and mental health; social connections; economic security; environmental quality; and cultural vitality. These things will help us build the domains of Tasmania's framework. The consultation process is part of Tasmania's commitment to listen to the voice of all Tasmanians and put this at the heart of decision-making. Once developed, the framework will assist us to provide investment, policy and decision making to areas which will make a real difference.

Ms WHITE - Premier, when is the precinct plan due for Macquarie Point?

Mr ROCKLIFF - To the best of my knowledge, in October.

Ms WHITE - What happens if you don't provide that to the federal government by October?

PUBLIC

Mr ROCKLIFF - The precinct plan? Well, I'll work to provide that to the Tasmanian people and I'm looking forward to that. It's an exciting opportunity and I can inform the committee that the Macquarie Point Development Corporation is currently working to develop a precinct plan and consultation is currently underway. Anybody is encouraged and information about the project and on how to provide input is on the corporation's website: www.macpoint.com/precinctplan. The plan will provide a high level outline of the key use areas of Macquarie Point and linkages to surrounding areas. The precinct plan will inform the development of a multi-purpose stadium on site, and an update to the current master plan.

Work is under way, as I said, to develop a draft precinct plan by October. I understand that is our deadline, not that of the federal government - which will be shared after it's presented to me for consideration and review. It is anticipated that much of the existing masterplan will be retained, with amendments to reflect an updated site layout and facilities to support the multipurpose stadium.

We aspire for the Macquarie Point precinct to be a place to gather, to celebrate and the to reflect through the arts, culture, sports, events and entertainment. It will be a vibrant extension of Hobart's CBD and waterfront. It is a once in a lifetime opportunity, set to deliver an extraordinary precinct for all Tasmanians and a destination for visitors. The updated masterplan will provide the framework for the development of the site, including what can and cannot be built. The updated masterplan will be used to seek an update to the planning scheme.

The site will include a Truth and Reconciliation Park at Macquarie Point -

Ms O'CONNOR - How are you going to fit that?

CHAIR - Order, Ms O'Connor.

Dr BROAD - Postage stamp.

CHAIR - Order, Dr Broad.

Mr ROCKLIFF - Its development will be guided by the original owners of lutruwita and palawa people. The corporation will work directly with the Tasmanian Aboriginal Community to progress this project. The multipurpose stadium will be part of Macquarie Point's arts, entertainment and sport offerings. The multipurpose stadium will offer a space to host a range of events, exhibitions, festivals performances and experiences all year, and will be a place for business and organisations to flourish. We will also make sure there is space for the development of Antarctic and science facilities as part of Tasmania's unique role as Australia's gateway to Antarctica and the Southern Ocean.

Ms WHITE - Premier, you've indicated the masterplan and the precinct plan are due by October, including an update to the planning scheme. Does that mean that work will need to be done before parliament can consider the POSS?

Mr ROCKLIFF - Physical work you mean?

Ms WHITE - No, you said the master plan and the precinct plan are due by October, including a change to the planning scheme. You said it would include an update to the planning scheme. Does that work need to occur before the parliament consider the Project of State Significance?

Mr ROCKLIFF - The master precinct plan will continue. The answer to your question is, no.

Ms WHITE - When do you expect the parliament to consider a Project of State Significance for that?

Mr ROCKLIFF - We are working on the draft amendment to the legislation now. I look forward to bringing that to the parliament as soon as practicable. I made a very clear commitment that's what we will do. As soon as possible.

Ms WHITE - Would you not agree it would be sensible to wait for the precinct plan and the master plan for the site before the parliament is asked to consider a project that we do not even know will fit?

Mr ROCKLIFF - I expect the draft amendment, I am advised, next session, given we have the Budget session and those changes to the act.

Ms WHITE - In spring?

Mr ROCKLIFF - Yes, August, the spring session of parliament.

Ms WHITE - Premier, you also indicated that the stadium is enabling infrastructure for housing, port and transport. When will those items receive funding. The last cost estimate for the port infrastructure was \$240 million. The analysis for the activation of the rail corridor for the rapid transit buses was about \$400 million. There has been no cost estimate for the housing. It is 120 units, so you could assume that is between \$30 million and \$50 million. There is no money in your Budget for any of those projects. What time line are you considering for developing those associated infrastructure projects?

Mr ROCKLIFF - That is a good question. The stadium is the enabler. The whole precinct is much more than a stadium, it is all the areas that you mentioned. You have to appreciate that this is intergenerational infrastructure. With the port rebuild, the housing and the transport infrastructure, we are talking significant investments. When it comes to all the infrastructure that will make up the part of the precinct plan, I anticipate that some of the timing of that infrastructure will be in the precinct plan.

Ms O'CONNOR - There are now two motions sitting on the table, one from the Greens and one from your former Liberal colleague, Mr Tucker, which give parliament the capacity to vote on members having a second and final vote on the stadium as a Project of State Significance. What is your response to those motions? Given your commitment to Mr Tucker to enable the second vote to happen, unless someone from the Cross Bench brings it on, would you be proactive about this in getting a legislative amendment up?

PUBLIC

Mr ROCKLIFF - I have been proactive. Our Government is looking at that draft amendment now. I understand the motions put forward and the reasons why. I have already given that commitment. I can't speculate on how motions will proceed or not through the House. I have committed to the parliament, the public and Mr Tucker an amendment to the act to allow the two votes to proceed.

Ms O'CONNOR - That would be, as you said to Ms White earlier, in the spring session. That would come in August?

Mr ROCKLIFF - Yes.

Ms O'CONNOR - I want to go to some of the finances for the stadium. This is something that a lot of people are talking about. Part of the public concern about this is the loading up of debt onto the Tasmanian people. We have a stadium that you say will cost \$715 million but, on the evidence of other stadia that have been built, it will cost significantly more than that.

Mr ROCKLIFF - Except that this is a 23 000 boutique stadium, Ms O'Connor. I stand to be corrected but I think Optus Stadium is 60 000 seats.

Ms O'CONNOR - Okay, I'll take that on board. But Macquarie Point is, as you know, a very tight site and it has significant remediation issues for any build. So, we have a stadium that may or may not cost \$1 billion, we have a forward projection of \$900 million or so out of GST receipts, we have cuts through the efficiency dividend to agencies and we have spiralling net debt. Do you think it's a fiscally responsible position to take, given the alarms I am certain were set off by Treasury, to sign a blank cheque for a stadium when the state's finances are in such a difficult place, and for the foreseeable future?

Mr ROCKLIFF - I reject the assertion that it was a blank cheque. It clearly wasn't.

Ms O'CONNOR - It was, because you promised to pay 100 per cent of cost overruns, so it is a blank cheque.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - You are ignoring the fact of the cheque that the AFL is providing, of some \$360 million to Tasmanians.

Ms White - They would provide that to any club.

CHAIR - Order, Ms White.

Dr Broad - Over how many years?

CHAIR - Order, Dr Broad.

Mr ROCKLIFF - Over 10 years.

Ms O'CONNOR - But the question still stands.

Mr ROCKLIFF - That's right, and it's interesting. It's about injecting some \$120 million investment as a result of the AFL team and everything else that goes with it. Ms White, I have to agree with your former colleague, Mr O'Byrne, when he made a very good speech to parliament. Most of it. I didn't agree with the stuff he said about the Rockliff Liberal Government but I agreed with what he said about the opportunities the AFL team will bring to Tasmania.

In the context, though, that our expenditure for infrastructure and the forward Estimates has lessened by some \$600 million, I believe it is affordable, it is sustainable, it will generate economic activity and jobs, and transform a precinct that would have to be invested in anyway.

Ms O'CONNOR - Our colleague in the upper House, Ruth Forrest, is possibly, across the parliament, the most knowledgeable about the intricacies of budget management. In an opinion piece she had published in the *Mercury* a couple of weeks ago, she states:

I cannot recall such wanton recklessness in any of my 18 years of analysing government budgets.

So clearly the alarm was raised by Treasury, the numbers speak for themselves, you have got seasoned budget analysts like Ms Forrest saying this is recklessness. What is your response to Ms Forrest?

Mr ROCKLIFF - I don't agree with Ms Forrest. I recognise that people might like to place that investment elsewhere but I don't see it as recklessness when you're investing in enabling infrastructure in an area that is ripe for investment, an area that, while over the course of the last 10 years there has been some work done -

Ms O'Connor - Of your Government, dithering.

CHAIR - Ms O'Connor, let the Premier answer, please.

Mr ROCKLIFF - Ms O'Connor, a lot of work has been done, in actual fact, around the planning and the remediation. It is not fair to say that nothing has happened. But with a bit more remediation to go, it is now ripe for investment and I believe this is a good investment, as they have been in other states of Australia. Why can't we have an opportunity not only to secure our own AFL team but also to be able to take part in what is an \$8 billion stadia economy.

I have some information: when I said I think we were expecting the amendment to the act to be in the next session, that could be late June.

Ms O'CONNOR - In the last sitting week, potentially. I thought you might want to knock it off early.

Mr ROCKLIFF - Possibly. We're getting some advice now but I don't want be criticised for not being factual.

PUBLIC

Ms O'CONNOR - Just for absolute clarity on that point, are you saying that the amendment to the State Policies and Projects Act will contain the order that the stadium be declared a PoSS (Project of State Significance) as well as the mechanism for the second vote?

Mr ROCKLIFF - The stadium will declare a PoSS, yes.

Ms O'CONNOR - I'm just trying to get some clarity for the whole table and then we can move on from this question. You have now said that it is possible that parliament will see the amendment, correct me if I am wrong, in this session, so the last sitting week before the break. Is it your understanding that what that amendment will be will be both the declaration of the stadium as a PoSS and the provision for the second vote?

Mr ROCKLIFF - It's just the amendment, at this stage, to allow the order.

Ms O'CONNOR - The second vote. So, the order will come in in August?

Mr ROCKLIFF - Later this year.

Ms O'CONNOR - Later this year, thank you.

Ms WHITE - Premier, I wanted to ask about -

Mr Rockliff - DPAC?

Ms WHITE - Maybe later.

Ms O'Connor - We have some DPAC questions, don't worry about that.

CHAIR - Order.

Ms WHITE - I wanted to ask about the tight time line that the AFL and you are signed up to in the agreement for the delivery of the stadium, and whether you are worried that you are already falling behind that. In one of the documents that you tabled last week it showed when the master plan and the precinct plan needed to be delivered, which was this year by about now, and that is already slipping to October. So, you are falling behind the schedule that the Government identified you needed to meet and there are penalty clauses in that document you signed with the AFL. Are you worried that you are already falling behind the schedule in order to start to build something on that site, like you signed up to and said you would?

Mr ROCKLIFF - No. There's a number of areas and conditions attached to the agreement, obviously. The state has agreed to using its best endeavours to have the TA facility completed by 31 December 2025 -

Ms White - Where is that going to be?

Mr ROCKLIFF - and achieving practical completion by 31 October 2027.

PUBLIC

The state has agreed to completing 60 per cent of the build of the stadium by 31 October 2027, four years away and a bit more. And achieving practical completion by effectively 2029, 31 December 2028, subject to the Commonwealth funding condition and all relevant statutory approvals being obtained.

The contract includes a sunset date for practical completion of the stadium by 31 December 2028 and the AFL has the right to terminate the AFL licence agreement, which results in the termination of the funding agreement, if the stadium does not achieve practical completion by that date.

This is a backstop date. If by 31 October 2025, the Tasmanian Government believes it will not reach 60 per cent completion of the stadium by 31 October 2027, it may provide the AFL with a notice requesting an extension for the licence conditions' sunset date, so that date of October, or a waiver of the 60 per cent construction requirement. Any delay in the Hobart stadium reaching the 60 per cent completion threshold is likely to delay the club's entry into the AFL competition, potentially.

The obligation to meet the time frame regarding the TA facility is limited to making best endeavours. However, practically, this may delay the club's entry into the VFL in preparation for entry into the AFL, such as the VFL and VFLW, and this milestone is a condition to the AFL licence becoming unconditional. Further delays of this milestone may result in the club's entry into the AFL competition being delayed as well, but I am confident that once we proceed we can meet those timeframes.

Ms WHITE - Can you confirm that the executive director of Major Stadiums, Andrew Finch, is no longer in that role?

Mr ROCKLIFF - I'll seek some advice on that although it's not my portfolio responsibility, it's a matter for State Growth.

Ms WHITE - You don't know?

Mr ROCKLIFF - I can seek some information for you as to the status of Mr Finch, but it is a question for State Growth.

Ms WHITE - Are you aware that the Goods Shed on the Macquarie Point precinct site has been nominated for heritage listing more than once? What's the current status of that asset?

Mr ROCKLIFF - I will seek some information for you on that. Is this regarding the Hobart Brewing Co?

Ms WHITE - No, the Goods Shed. They had entered into a contract to take the Goods Shed on as a small business operator, but given you announced the stadium, their plans were scuttled.

Mr ROCKLIFF - I am advised that in April 2022, Hobart Brewing Co was issued a lease for the Goods Shed which required redevelopment of the space. The corporation has

PUBLIC

had regular engagement with the Hobart Brewing Co, I am advised, since it was announced that Macquarie Point is the preferred location for a multipurpose stadium.

Ms WHITE - My question was about the heritage listing of the Goods Shed.

Mr ROCKLIFF - The Goods Shed remains available for events, and the Hobart Brewing Co continued to manage the facility and operate from the red shed on site. I am advised the corporation is in discussion with the Hobart Brewing Co around options in relation to the lease of the Goods Shed.

Ms WHITE - My question wasn't about the Hobart Brewing Co. It was about the listing of the Goods Shed on the Heritage register. Are you aware that it has been nominated more than once? What is the current status of that assessment?

Mr ROCKLIFF - That's a matter for State Growth; I am sure they will be able to assist you with the exact status of that.

Ms WHITE - Wouldn't you think it would be important for you to understand whether the building was heritage listed before you went ahead and built a stadium on it?

Mr ROCKLIFF - As I say, there were discussions with the Hobart Brewing Co.

Ms WHITE - I'm not talking about the Hobart Brewing Co.

Mr ROCKLIFF - Yes, well -

Ms WHITE - The Goods Shed - have you been down to that site, do you know what I'm talking about?

Mr ROCKLIFF - I do.

Ms WHITE - It's a very big, large, old shed which I believe has been nominated more than once for heritage listing.

Mr ROCKLIFF - Okay, I'll take you at your word. This is a matter for State Growth. We are in DPAC Estimates and I have been very accommodating talking outside the DPAC budget Estimates, Chair, I am sure you would agree, but there are a number of matters across the portfolio area. Nomination for heritage doesn't mean it's been listed.

Ms O'CONNOR - That's why I asked for an update.

Mr ROCKLIFF - My understanding is that it isn't on the Heritage register; that is the information that I have.

Mr YOUNG - Premier, I have a DPAC question for you. You have previously said that the Government would establish strategic regional partnerships to work with regions on their future and growth as well as community wellbeing. Can you provide an update on the first partnerships established with the west coast?

PUBLIC

Mr ROCKLIFF - I can indeed, Mr Young. We recently had a Community Cabinet on the west coast which was very enjoyable. Our Government recognises that each Tasmanian region and their people are unique; they have different economies and communities, different strengths but also different challenges, so we are not taking a one-size-fits-all approach. Like our jobs hubs, which are helping to flexibly deliver local jobs in local communities, we are also taking a place-based approach to delivering stronger regional economies and communities through our strategic regional partnerships.

The partnership approach is aimed at identifying local challenges, supporting practical solutions and helping regions to coordinate and outline the long-term framework for investment and growth, but also better government services within the area. The west coast region has been identified as our first partnership. Discussions have commenced and we aim to deliver the first partnership by the end of 2023.

Scoping has identified three key issues facing the region, including accommodation and housing for workers in the mining and renewable energy sectors but also in health services for the attraction and retention of staff and the provision of essential services from a liveability perspective and economic diversification. As a result, the Department of Health is developing a statewide staff accommodation model to more efficiently manage accommodation demand and provide a consistent standard.

The next steps for the west coast will be bringing together key regional decision-makers and formulating practical and achievable solutions. Funding of \$3.57 million has been provided over two years in the 2023-24 Budget, including \$1.35 million in 2023-24 and \$2.2 million in 2024-25 to support the partnerships, including identifying options for worker accommodation, and further partnerships will be identified in consultations with councils and regional bodies soon.

Ms O'CONNOR - Premier, among the documents that you tabled on Thursday is a fascinating document that asks, 'Will it fit?'

Mr ROCKLIFF - The fact sheet.

Ms O'CONNOR - The fact sheet, which asks, 'Will it fit?' We have scaled out the measurements and think the stadium itself will just fit. Can you confirm that there is a question mark over whether there is space for a concourse and access for emergency vehicles in the design so far? Our understanding is that it doesn't quite fit when you put a proper concourse in there. Do you know anything about this?

Mr ROCKLIFF - I didn't anticipate your level of expertise in these matters in terms of doing your own work on that.

Ms O'CONNOR - No, it's just measuring up the numbers that were given and putting it in the diagram.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - This will all be worked through in the precinct plan. Remember, with all due respect, that the design brief is still to be done and we will ensure that the matters

you raise will meet all those requirements. Having secured the funding just over a month ago, we are now at that very beginning stage, as I highlighted this morning in an answer to Ms White, where the precinct plan, the design of the stadium and all those matters can now take place with consultation, which is exciting. Our own Tasmanian design of an iconic boutique stadium, relevant to Tasmania, attracting investment - it's a very exciting proposal.

I recognise the challenges in being able to bring everyone on board with this but I have to say there are a lot of people who support the stadium and the AFL team. They recognise the fact that you can't have an AFL team without the stadium and they are very excited about the opportunities it will bring. Your questions are very legitimate, as are the questions from the RSL, as are the questions around a reconciliation park and other matters, which will be worked through in the design stage of not only the stadium but also the precinct.

Ms O'CONNOR - Thank you. Can we move on now to the commission of inquiry? This is specifically to DPAC.

As you are aware, we had an exchange of letters late last year where the Greens suggested that some money be allocated in this Budget prior to the handing down of the commission of inquiry's recommendations just to be sure you'd be ready to implement them as soon as the report is handed down at the end of August. We have the DPAC updated disclosure log as at Friday, which shows that now 43 state servants have been suspended over child sexual abuse allegations. Whether they are historical or contemporary is unclear.

There are four ED5 investigations that have found a breach of the code of conduct in relation to child sexual abuse allegations and there's been a sanction determined. Could you talk us through how it could be that so many state servants now have been suspended or faced a sanction as a result of the evidence put to the commission of enquiry?

Mr ROCKLIFF - I've spoken about the importance of the safety of our children and young people. Mr Young asked that question before. On 23 March this year, the secretary's board endorsed a new Keeping Children Safer government structure to facilitate the Government's commitment to implementation of immediate actions and respond to issues raised during the hearings of the Commission of Enquiry into the Tasmanian Government's Responses to Child Sexual Abuse in Institutional Settings and to include the establishment of a child and youth safety and wellbeing subcommittee of the secretary's board, providing whole-of-government oversight of actions and facilitate strategic outcomes in response to the commission of inquiry's final report.

What you're referring to is very important. We all, when we get the opportunity, should offer our deepest and heartfelt apologies to all victims and survivors of past child sexual abuse that occurred in relation to agencies across our state.

Ms O'CONNOR - Hear, hear.

Mr ROCKLIFF - It was an important moment when the parliament officially apologised to victims and survivors.

Ms O'CONNOR - Could you address the substance of the question in relation to the number of state servants who've either been suspended or sanctioned. That's a very high number of state servants who are part of the fallout of the commission of enquiry.

Mr ROCKLIFF - The latest disclosure, which was updated on 2 June and as I understand as of 3 June. Since October 2020, there have been 43 suspensions of state servants as a result of allegations of child sexual abuse, 22 related to historical allegations and 21 related to contemporary allegations. I'm advised 27 of the 43 cases are located in the north and 16 in the south. Of the 43 cases, all have been referred or are known to Tasmania Police. In 41 of the 43 cases, notifications have been made to the relevant regulatory body. The further two cases were matters involving an employee whose employment was not subject to any regulatory body.

Of the 43 employees, four have been found to have breached the code of conduct in relation to contemporary child sexual abuse allegations. Eleven investigation determinations have found no breach in relation to child sexual abuse and 10 of the 11 employees have returned to duty as the findings of the relevant investigation found it was appropriate for this to happen. The remaining employee has resigned.

Ms O'CONNOR - That sounds like quite an efficient system of dealing with potential historical or contemporary child sexual abuse in the State Service. What level of sanction is applied to the four state servants who were found to be in breach of the code of conduct under ED5 investigations? Are they all employed still in the State Service?

Mr ROCKLIFF - I can't provide individual circumstances around that.

Ms O'CONNOR - We're not looking for identifiers.

Mr ROCKLIFF – We have some information here. Do you mind if I hand over to our secretary, Ms Gale?

Ms GALE - Through you, Premier, there is a range of sanctions that heads of agencies can apply when somebody is found to have breached the code of conduct. They range from counselling through to termination, depending on the nature of the allegation and what has been found. So, it's a matter for each head of agency to make those determinations based on the particular circumstances of the case that they have been considering.

Ms O'CONNOR - Thank you, Ms Gale. One final question. Of the four ED5 investigations where a breach of the code was confirmed, without identifying individual state servants, are all four of those state servants still employed within the State Service or have some been suspended or their positions terminated?

Ms GALE - Through you, Premier, when there are only four there is a possibility that state servants could be identified by providing that kind of information. In relation to the routine disclosure you will see on the right-hand side of the table that there is information about the numbers of state servants who have returned to duty. The table currently shows that 10 have been returned to duty.

Ms O'CONNOR - Ten of the 43?

Ms GALE - That is correct.

Ms O'CONNOR - My final question on this round of questions. So, \$30 million is allocated in this year's state Budget to implement the recommendations of the commission of inquiry but the Budget shows that there has been \$1.3 million for child safety that was expected in this Budget which has been pushed out to 2026-27. Also delayed funding of nearly \$3 million to the Transition to Independence program, which supports young people exiting care and delayed funding for the school support package including the north-west support school of \$20 million a year. That adds up to almost to \$30 million. Is the \$30 million allocated to the Commission of Inquiry's recommendations coming from other parts of the Budget that relate to children and young people?

Mr ROCKLIFF - The \$30 million is for, as I outlined today in Mr Young's question, the areas that we are already actioning, around 30 recommendations. I want to make sure I have this right. Then the commission of inquiry recommendations will come out and those funds can then also support immediate actions. Responsible agencies can immediately draw on those funds. We expect towards the end of the year that the true investment required to support the roll out of the recommendations of the commission of inquiry will be brought to parliament. That would reflect a forward Estimates figure where that investment will be applied.

What was important in the \$30 million is that agencies can immediately implement some of the recommendations and some of the 30 that we have already highlighted over the course of the last year.

Ms WHITE - Premier, the Budget position is deteriorating. You and I discussed in this output last year the prospect of an efficiency dividend. I will remind you that when I asked you about whether you'd looked to apply an efficiency dividend last year you said, 'That is not on our agenda at this stage'. Premier, what has changed? There is now a \$300 million efficiency dividend built into your Budget?

Mr ROCKLIFF - A lot has changed, Ms White. I recognise why the question is being asked. You might have had an exchange with my predecessor in the 2019 budget. The efficiency dividend was put forward in the budget then. Agencies were then working with various ministers on how that might be applied. Then COVID-19 hit and that efficiency dividend was taken off the table. We have invested between \$1.5 billion and \$2 billion to keep people alive, well and in work.

Ms WHITE - I asked this question of you last year. This wasn't pre-COVID-19.

Mr ROCKLIFF - Yes; and I'm providing some background for you. We've made significant investment through COVID-19. This Budget highlights a very measured response ensuring we bring back the efficiency dividend, albeit starting in 12 months' time; and that will be 60 cents in every \$100. We have a challenging budgetary position, as does the federal government and other states of Australia. Victoria's budget was handed down just prior to ours and it highlighted their massive challenges.

We have the second lowest net debt per capita in the country, behind Queensland, but we still have to ensure that we manage our expenditure in a very sensible and measured way. That's why we have the efficiency dividends starting in 12 months' time. It gives agencies and ministers time to work through those matters. It is worth putting in context that this is a modest adjustment we can achieve through backline efficiencies, but also achieve through digital enhancements. There is a significant investment into digital enhancement, particularly in my area of responsibility, which is health. To put it into context, the Commonwealth Government, on average, has a 1.5 to 2 per cent efficiency dividend each and every year. What is important as well, in our discussions with agencies, is that frontline services will be protected.

Ms WHITE - Premier, you sit on the Budget subcommittee - you shared that with us last year -

Mr ROCKLIFF - In a very open and transparent way.

Ms WHITE - Good on you. As I recall, the Deputy Premier, the Minister for Sport, Mr Street, and the Attorney-General joined you on that subcommittee; unless the membership has changed. You told us last year that you're making decisions about how to deal with the growing record debt, because on the current trajectory under the risk report provided by Treasury we're tracking to nearly \$30 billion in debt, which is one of the worst case scenarios set out in that report. Can you rule out making further cuts beyond \$300 million you've earmarked in this year's Budget?

Mr ROCKLIFF - I can rule it out this year, because we've got our efficiency dividend. We have no intention, because we're starting the efficiency dividend in 12 months' time. Our forward projections would mean that we don't have any need for further cuts. I can rule out further cuts because we've got our forward Estimates where the efficiency dividend is embedded. We want to invest in further services and continue to grow our economy.

Ms WHITE - Can you rule out any new taxes?

Mr ROCKLIFF - Any new taxes? We've new taxes in the Budget.

Ms WHITE - There's some levies though, aren't there?

Ms O'CONNOR - What's wrong with that?

Ms WHITE - It's is a sneaky way of getting around saying you don't have new taxes.

Mr ROCKLIFF - No. As at a few days ago, we are the lowest taxing state in the country. That's the information I have; compared to Victoria - they're taxing like there's no tomorrow. We are being very responsible in our taxation regime, ensuring we have the revenue to fund our essential services -

Ms O'Connor - The stadium.

Mr ROCKLIFF - This is why we are saying to Victorian businesses, come on down to Tasmania. What are you shaking your head, shadow treasurer?

Dr BROAD - You're kidding yourself. They're poaching our teachers; are you happy with that?

Mr ROCKLIFF - Come on.

Ms O'Connor - And our healthcare workers.

CHAIR - Order.

Mr ROCKLIFF - We've got healthcare workers coming internationally. You have to start talking up Tasmania, Shane. You're very negative all the time, so negative.

Dr BROAD - Because you're running the show.

Mr ROCKLIFF - No. Tasmania's in a very good position.

Ms WHITE - Last year when we had this same discussion, you said that there's importance for reform in the area of taxation. because we talked about the sustainability of the Budget. Is that still your intention, to progress tax reform, and can you outline your thinking?

Mr ROCKLIFF - I'm interested in a level of reform in this area and there's been some discussion about the fire service levy, as we realise.

Ms WHITE - Sure has.

Mr ROCKLIFF - I'm not going to rule out reform in that area because it's an existing levy; but we will take advice on that. Surely, you're not saying we should not reform at all?

Ms WHITE - I'm asking you to explain what your thinking is.

Ms O'CONNOR - With the tax reform agenda.

Ms WHITE - At the moment, all I have is a brief comment from you from last year - no further information for 12 months. Here we are again, and I'm asking you once more to outline your thinking about what sort of tax reform you are looking at in Tasmania?

Mr ROCKLIFF - I have mentioned the levy as one of those areas. We're interested in ensuring that we remain very competitive in terms of our taxation revenue and our levy revenue. As it was highlighted, we are the lowest in the country, which is a good position to be in. We want to stay that way, but we also want to ensure that we are funding our essential services - and the best way to do that is to grow our economy.

Ms WHITE - So, you don't know. Are you going to start a process for tax reform or are you just going to vaguely talk about it?

CHAIR - Order, Ms White. Please let the Premier answer.

PUBLIC

Mr ROCKLIFF - This is an area that the Treasurer would be very interested to talk to you about.

Ms WHITE - You have nothing, really. You're not announcing a process; you're just talking about it for another 12 months, and it is very vague and no commitments.

Mr ROCKLIFF - Our Budget, given that we have invested so heavily throughout COVID-19 and when you compare our Budget -

Ms WHITE - I'm talking about your previous statements stating there would be tax reform, and there hasn't been.

CHAIR - Ms White, please let the Premier answer.

Mr ROCKLIFF - Not only our level of taxation regime being the lowest in the country, as highlighted by the report, but also the second-lowest net debt per capita. That is a very strong position to be in, despite the fact that we've had to invest upwards of \$2 billion throughout the COVID-19 years.

CHAIR - Premier, a question from the me, the chair - Christine Milne has put a tweet up:

Jeremy Rockliff admits higher Tas power prices because of Marinus but will consider an energy dividend from profits to offset price increases. But what if it doesn't make a profit? Not only higher prices but debt servicing as well. How about not proceeding?

Do you think Marinus will bring down power prices, and if it doesn't, do you think Marinus will make a profit that you will be able to provide an energy dividend?

Ms O'Connor - I never thought I'd see the day - Christine Milne's tweet being quoted in Estimates from an Independent.

CHAIR - Thank you, Ms O'Connor. Premier?

Mr ROCKLIFF - Yes, and thanks for the question. This is a very good question for our minister for Energy as well, Chair. We've been very good and responsible in our support of Tasmanians when it comes to energy price increases. That's most evidenced by this Budget, with \$45 million matching the \$45 million of the federal government. Also, Mr Tucker, last year with the Winter Bill Buster payments, we are supporting Tasmanians through those energy price hikes.

Marinus will bring greater investment in the renewable energy sector - an estimated investment of some \$7 billion into wind power generation and other, and also secure our energy future, so it's not only about ensuring the price is stable and we don't have the huge price hikes they are experiencing on mainland Australia, and putting downward pressure on electricity prices, it's an important part of the discussion, but also it's about supply and about industry attraction as well.

Our connection to the national grid is also critical to support our role in helping Australia and the rest of the world meet its emissions targets. Climate change doesn't recognise state lines and borders, so we all have to act when it comes to that. Marinus is about our renewable energy self-sufficiency for Tasmanians. It's not undermining it, it's about our self-sufficiency.

More pertinent to your question, with Marinus Link Tasmania will have access to a greater diversity of renewable energy sources, putting downward pressure on electricity prices, with independent modelling showing lower prices in a world with Marinus compared to one without. The Australian Energy Market Operator's integrated system plan, ISP, confirms that Marinus Link will deliver a net benefit of \$4.5 billion and more jobs, attracting billions of dollars of investments in regional areas of Tasmania.

I am pleased that the Australian Government has listed Marinus Link and associated projects under its Rewiring the Nation program to provide low-cost financing to fund the construction of these projects. We are going through the FID process at the moment, the final investment decision process, and we are expecting a final investment decision in 2024 confirming the project remains in Tasmania's best interests.

Mr YOUNG - Premier, it seems to me that the State Service would be an excellent career choice for young Tasmanians. Can you tell me how the Government is working to ensure jobs are available for younger Tasmanians in the State Service?

Mr ROCKLIFF - Thank you, Mr Young, and I also agree with you and others who have said that the State Service is a great option for young Tasmanians because there is a range of different job opportunities and also because younger Tasmanians help ensure a diverse workplace and a solid future workforce. We have a range of ways to entice young people into the State Service, from school-based traineeships, cadetships and graduate programs, including in regional areas. The Tasmanian State Service school-based traineeship program supports young people transitioning from school to work. In the past year, seven agencies recruited school-based trainees in areas such as business administration, ICT, automotive customer service, sport and recreation and animal care. Since the 2019 pilot of this program, we have employed 30 school-based trainees and it means students can gain a nationally recognised qualification through working in the State Service while staying at school to complete their TCE in years 11 and 12. The program also supports increasing Tasmania's grade 12 retention rate.

We also provide cadet and graduate career opportunities across agencies. TasGraD is where university graduates are employed across agencies and participate in a development program designed to build foundation skills for a successful career. In 2023, there are 34 roles for graduates to participate in. In addition, 39 graduates who participated in the 2022 TasGraD program continue to be employed in the State Service. Since the TasGraD program commenced in 2017 a total of 188 graduates have been employed in the State Service through the program, and while there is no age limit, most are young Tasmanians.

The Department of Treasury and Finance and the Department of Natural Resources and Environment Tasmania also employ cadets, and this year there are 33 who will undertake university study in conjunction with their work duties. Further recruitment is planned for August 2023, with a further four cadets to join NRET. All up, in December 2022 there were

PUBLIC

4450 young people under the age of 30 working in the State Service, representing 13.27 per cent of the workforce, an increase of 9.19 per cent since last year, which is great, and nearly double that of 2014.

Ms O'CONNOR - Premier, back to the commission of inquiry. Obviously we can't be sure what recommendations the inquiry will be handing down, but there's some areas of specific inquiry that might point you as Premier to places that need reform.

Mr ROCKLIFF - And have done.

Ms O'CONNOR - Yes, I accept that. One of the issues that has come up through the commission of inquiry is the lack of robust protections for whistleblowers who might report child abuse or inappropriate behaviour with children amongst state servants. If the commission of inquiry recommends changes to the whistleblower act, will your Government be moving on that?

Mr ROCKLIFF - I anticipate we will if it's recommended. I'll be very interested in the recommendations of the commission of inquiry and if there's a recommendation to do that, absolutely.

Ms O'CONNOR - Do you think the whistleblower protections we have in place now are adequate?

Mr ROCKLIFF - I have no doubt that ensuring we have the most robust whistleblower protections possible is a good idea and if there is a way to improve them, we will.

Ms O'CONNOR - Another area of legislative reform that may be required is to the State Service Act, potentially the process around ED5s. Should the commission of inquiry find there are deficiencies in the State Service Act and the process of ED5s, will you commit to amending that act and strengthening it to protect children and young people?

Mr ROCKLIFF - My understanding is that we've done some work in this area. It is one of the interim actions of the 30 recommendations and employment direction 5 is currently being reviewed initially. This was with consideration of the recommendations from the 2021 independent review of the Tasmanian State Service, so from my memory there were 77 recommendations from that and we are acting on the first stage of that, which is around 29 recommendations from memory, and this was part of that. I stand to be corrected on that but that's my memory.

It was then expedited to support the Keep Children Safer priority actions as part of the Government's interim response to the commission of inquiry made in February this year, noting reference 8 of Keeping Children Safer is to review and rewrite employment direction 5. State Service review recommendations 55 and 56 are directed specifically at making improvements to ED5 and the management of code of conduct processes within the agencies. As part of the review, all the employment directions are subject to revision and ED5 was specifically expedited for review as part of the Keeping Children Safer response. The ED5s will be revised as part of this response initially and continue to be updated as part of the State Service review implementation and the process for managing conduct and behaviour in the State Service.

I have some further information which may well be helpful. I am pleased to report that we are well on the way to realising these proposed changes subject to further stakeholder consultation, including with the unions. The draft ED5 for consultation with the stakeholders is being developed with input from agencies and further stakeholder discussions are continuing. After consultation and final review, I intend to issue a revised ED5 to meet the Government's commitments to undertake this work, noting the response to the Keeping Children Safer commitment has been extended until October 2023 to ensure that all recommendations from the commission of inquiry can be considered where relevant.

The key changes to the proposed draft ED 5 include greater flexibility. This follows recommendations from the Independent Review of the Tasmanian State Service that head agencies should be able to tailor an investigative process based on the circumstances surrounding an alleged breach by changing the 'must appoint an investigator' to 'may commence a process consistent with the ED and may appoint an investigator'.

The current requirement to always appoint an investigator can be disproportionate to the circumstances. For instance, where an employee admits to certain behaviour, an investigator may not be warranted. This may accelerate dealing with the matter and consideration to improving the timeliness of matters being dealt with. Is there anything further to raise, Ms Gale?

Ms O'CONNOR - Just on that - and you, Ms Gale, may have something to contribute to this - what became really tragically apparent during the evidence to the commission of inquiry was that there has been a culture within the State Service, a culture, for example, at places like Ashley Youth Detention Centre, of not taking seriously enough allegations of the mistreatment or sexual abuse of children. Or, in the circumstances surrounding the Launceston General Hospital, of cover-up internally within management of manifest problems and harm to children. Are you confident that these changes that you have committed to making will deal with that culture which is residual and damaging to children? It is a culture of cover-up.

Mr ROCKLIFF - I am confident but also it has to change. I am confident it will. But can I say, in that sense, it is not enough. It just has to change.

Ms O'CONNOR - How do you make absolutely sure of that?

Mr ROCKLIFF - We are doing all we can in terms of - you mentioned the LGH. In February this year - I can throw to others who have some more information - we announced the critical steps being taken to strengthen child safety in our hospitals and our health settings, as our Government implements the recommendations from the Child Safe Governance Review. Following the release of the report of the Child Safe Governance Review, the governance advisory panel, our Government has committed to all 92 recommendations in full. Over 30 recommendations have already been completed and strong progress is being made across a number of other recommendations.

Customised and mandatory training is being rolled out to all staff, focusing on mandatory reporting, professional boundaries and identifying signs of grooming as well.

Ms O'CONNOR - Within Health or just at the Launceston General Hospital?

Mr ROCKLIFF - The Department of Health in its entirety. The Department of Health is progressing a complaints management project. I know we can probably talk about this during our Health Estimates. But I am advised that phase 1 has been completed, which includes the establishment of the new independent and centralised statewide complaints oversight unit to review the current state of complaints management and design the future of complaints management in the department.

We were also working on Item 6, in terms of the interim response the Commission of Inquiry - encourage and support staff to raise child safety concerns. That is underway. The Keeping Children Safer working group has commenced mapping of agency staff child safety educational resources so they may be shared and tailored to departmental needs to support the wider cultural change and staff training across the service.

If I can talk to schools, Ms O'Connor, as well, it is extremely important. The Safeguarding in Schools model was approved by the Minister for Education, Children and Youth in October last year. The Office of Safeguarding Children and Young People has been established to drive long-term cultural change and continuous improvement across the Department for Education, Children and Young People.

I am pleased to say that safeguarding leads commenced their roles in all state government schools at the beginning of the 2023 school year. These leads will help plan and implement strategies to support the wellbeing of students at each government school to foster a child-centred culture where safeguarding our children is everyone's responsibility.

Compulsory mandatory reporting training has been rolled out to all staff at the Department for Education, Children and Young People. A range of mandatory professional development is also provided to all staff working in the areas of Children and Families, and Youth Justice.

Initial compulsory training is understanding, preventing and responding to child sexual abuse. It was developed and undertaken with the school principals and safeguarding leads towards the end of last year. Similar compulsory training for all departmental staff has commenced.

CHAIR - Do you want to add more to the answer?

Ms GALE - I was just going to add a bit more information in relation to the ED5 dates and where that's going. One of the really important learnings out of the Commission of Inquiry was that a lot of the practice in agencies wasn't trauma-informed. Therefore, that's an important part of updating the ED5 in these first steps. In relation to that, we are rolling out trauma-informed training across the State Service, starting with senior officers. The whole-of-government panel for ED5 investigators' tender documents also requires investigators to be trauma-informed.

The other part that I think is worth mentioning today is that the ED5, as was explained through the hearings, is part of an employment framework. The other big learning was that

PUBLIC

the processes really looked at things from the employee's perspective in terms of natural justice, procedural fairness and so on. It didn't give much guidance at all on keeping complainants informed about the process. So, that is another important step that we are taking in relation to making those changes for the process prior to the final report being provided.

CHAIR - As the time is a couple of minutes past 11 a.m., we will have a short five-minute break and come back at 11.07 a.m.

The Committee suspended at 11.03 a.m. to 11.09 a.m.

Ms WHITE - Premier, I have a copy of a letter here that was written by you on 20 February this year where you offered Mr Tarrant Derksen, director at New Norfolk Distillery \$1.2 million in grant funding for his project. It was provision of a grant to a project that was subject to four different requirements that needed to be met. I was interested in understanding what the process was for you determining this project would be the recipient of a \$1.2 million grant.

Mr ROCKLIFF - It's a good project, and a lot of areas and caveats around that. This is really a question for the Tourism Estimates.

Ms WHITE - It's on your letterhead as the Premier of Tasmania.

Mr ROCKLIFF - It is part of my Tourism responsibility -

Ms WHITE - It's not clear that's where the money was coming from.

Mr ROCKLIFF - Our visitor economy one of our state's biggest strengths, delivering a record spend to our economy, supporting our regional communities and the jobs that come with them. Our products and experiences are the backbone of our tourism industry and we must continue to foster innovation and growth so that we can remain competitive as a destination to attract visitors who will stay longer. This is in the Tourism Estimates and part of the responsibility -

Ms WHITE - This is about a process. The question I have is not about the project but about the process.

Mr ROCKLIFF - The process is outlined there.

Ms WHITE - How did they get offered a \$1.2 million grant?

Mr ROCKLIFF - The process has been outlined. Nothing has been expended in this Budget. One such proposed project is the New Norfolk Distillery Willow Court revitalisation project. This project aims to restore the buildings in Willow Court into an innovative visitor experience for tourists and locals alike, with a focus on a cellar door, tasting room, distillery, restaurant, function space, as well as a courtyard and external works.

The project aims to increase visitation to the Derwent Valley, building on its reputation as a must-see Tasmanian destination. As I've informed parliament on a previous occasion, no

allocations of funding have been made to this project. The distillery successfully applied for a \$2 million loan under the Tourism Development Loan Scheme to assist with its project in February 2022. The original loan was approved, subject to conditions including the project receiving federal government funds, the transfer of properties from the council and an equity contribution by the proponents and appropriate approvals.

This loan was assessed by the independent Tasmanian Development Board, which is at arm's length from the Government, however, the proponent was unsuccessful in securing the Australian Government funds at the time. In recognition of the project's merits, in February this year I provided the New Norfolk Distillery with a letter of intent for a grant of up to \$1.2 million towards the project. The grant would be conditional and these conditions were made very clear, including the transfer of land from the Derwent Valley Council to the New Norfolk Distillery with the New Norfolk Distillery demonstrating an ability to finance and complete the project. I am advised the proponent needs to raise significant additional funding which would involve satisfying the independent Tasmanian Development Board via its loan process and other terms and conditions commensurate with a proposal such as this.

The proponent has applied for a \$2 million loan under the Tourism Development Loan Scheme and is being assessed at arm's length from the Government by the TDR board. This process includes due diligence being undertaken on the project. As such, no allocations have yet been made from the Tasmanian Government. The Department of State Growth continues to work through these conditions with the proponent. I note the comments of the proponents in the media over the past week where they have withdrawn their offer to buy the buildings at Willow Court due to deliberations of council over the sale. The department will discuss this development with the proponents, as one of the conditions was the transfer of the land.

Ms WHITE - Premier, you've outlined the process in some of your answer. They have applied to the TDR board for a \$2 million loan. They were unsuccessful. Then you wrote them a nice letter offering \$1.2 million as a grant, not a loan. What was the grant program that you were going to offer them money through? Were other people able to make applications for grants under the same program or did you just hand-pick this particular proponent?

Mr ROCKLIFF - Again, this is another Estimates. Given you have raised it, I thought it was a worthwhile investment for a rural and regional community with a number of caveats. Not much has happened with Willow Court for a long time. I have not visited Willow Court for quite some time but it's an opportunity for the local council or the local community. I thought it was worthy of investment.

Ms WHITE - You will be aware that Justin Derksen was a Liberal candidate at the last election and is currently employed as an adviser in the ministerial office in your Government, that administers projects like this because he works for the minister who reports to the Department of State Growth.

Mr ROCKLIFF - I'm not going to get into any personalities and -

Ms WHITE - I understand he's a brother of the proponent. How was this conflict managed?

PUBLIC

Mr ROCKLIFF - Well, there's no conflict. I don't -

Ms WHITE - He's an adviser to the minister whose department gave them money.

Mr ROCKLIFF - And I haven't spoken to Mr Derksen who works for Mr Barnett at all. Surely you are not going to go down that personality track?

Ms WHITE - You gave the money from a grant funding round that nobody else could access.

Mr ROCKLIFF - I haven't given any money

Ms WHITE - You gave them a nice letter that offered \$1.2 million.

Mr ROCKLIFF - With a number of conditions attached to it, because I'm investing in rural and regional Tasmania. A worthy project that has community support. But, given the media comments and the number of conditions that have not yet been met, it's not part of this Budget. You'll notice the media comments more recently about this matter; there was a project that I thought was worthy for support in rural and regional Tasmania, in the interest of jobs.

Ms WHITE - There are lots of worthy projects that are probably more deserving of support in regional and rural Tasmania. How do they get access to a grant of \$1.2 million from your Government in a program that wasn't publicly advertised?

Ms O'Connor - Have a family member run as a candidate.

Mr ROCKLIFF - I'm not going to get into any personalities on this and play personal politics -

Ms WHITE - Even if there weren't personalities of this nature involved, there are clearly conflicts. This doesn't look good, where you're offering a \$1.2 million grant for a project that didn't go through any competitive process.

Mr ROCKLIFF - But a number of caveats were put on that loan. No dollars have been provided.

Ms WHITE - That's not because you didn't want to, though; it's because of other factors.

Mr ROCKLIFF - I believe in a lot of worthwhile projects around Tasmania. The *Cartela* was a good example of that, and we were also investing in that as a tourism opportunity.

Ms WHITE - Do you often provide letters that would provide grant funding to projects across the state without there being any competitive process to award them, or assessment to understand if they're the best value for taxpayer money?

PUBLIC

Mr ROCKLIFF - No; but again, no dollars have been provided, not in this Budget; and a number of caveats were put in place.

Ms WHITE - What made this one different, then?

Mr ROCKLIFF - As I say, I've invested in key areas of responsibility. The *Cartela* is a good example of that. This is another investment to support rural and regional Tasmania, which I believe has support out there in the community, and no dollars have been provided.

Ms WHITE - This will be my final question, if I may. Your Government has a problem with providing money to different projects where there are perceived or real conflicts of interest and here's another example. When are you going to clean this mess up? The Integrity Commission has called it out. It doesn't seem that you've learnt from the sports rort scandal at the last state election, because this letter was sent on 20 February this year.

Mr ROCKLIFF - I absolutely reject your assertion. We invest in key projects. I'm not going to go back into the matters pertaining to the 2021 election where candidates for our team had a very legitimate reason to work with local communities -

Ms White - Family connections

Mr ROCKLIFF - and support their needs.

Ms O'CONNOR - Premier, I have some questions about your work on National Cabinet. On what evidence did you base your push with former New South Wales Premier Dominic Perrottet to remove five-day isolation on -

Mr ROCKLIFF - That's a National Cabinet decision, Ms O'Connor -

Ms O'CONNOR - You led the charge, with Dominic Perrottet. The public record is very clear about that. Both of you went in and argued for the removal of five-day isolation. Did you have evidence?

Mr ROCKLIFF - And agreed to by the Prime Minister and every other Premier.

Ms O'CONNOR - I'm still waiting. Was there an evidence base that enabled you, as Health Minister and Premier, to argue that it was safe to remove mandatory five-day isolation?

Mr ROCKLIFF - AHPPC and advice from the Chief Medical Officer at the time supported the National Cabinet decision, if my memory serves me correctly.

Ms O'CONNOR - That is interesting. If you have a policy of forced infection, I am sure that is true. Did you lead the charge with former New South Premier, Dominic Perrottet, to remove five-day isolation, knowing it would lead to an increase in infection, reinfection, disability and death?

Mr ROCKLIFF - Ms O'Connor, we have been working very hard and transitioning out of an intense COVID-19 environment. Rules and regulations have lessened since the height

of COVID-19. That was another step in that process. I am happy to take this on board again in Health.

Ms O'CONNOR - Don't worry, Dr Woodruff is looking forward to having a chat.

Mr ROCKLIFF - I'm pleased about that.

Ms O'CONNOR - As you know, Premier, Tasmania has the highest COVID-19 transmission rate in the country, by a significant margin. I noted what you said before, about the importance of keeping children safe and well. When you are constantly allowing them to be reinfected with a novel coronavirus, that is not possible. According to the most recent Tasmania Health information, for the fortnight ending 25 May, five to 17-year-olds had the highest rate of COVID-19 infection per 1000 people in Tasmania. Do you think it is satisfactory and responsible to allow the continued infection and reinfection of children with a novel coronavirus that is vascular, that causes long-term sequelae, brain damage, increases in childhood diabetes and kidney issues along with a whole range of other compounding sequelae? Do you think that is keeping children safe and well?

Mr ROCKLIFF - You are merging two very important areas of responsibility.

Ms O'CONNOR - This is about National Cabinet, where you have been completely irresponsible. But, yes, carry on.

Mr ROCKLIFF - National Cabinet has made a very clear decision. All the Premiers and the Chief Ministers sit within National Cabinet, including the Prime Minister. I understand your point of view -

Ms O'CONNOR - No, you don't. I don't think you have allowed it to permeate your consciousness, otherwise you would not be conducting yourself this way as Premier.

Mr ROCKLIFF - Ms O'Connor, I understand your view on matters relating to COVID-19 and related safe behaviours. We have spoken about it and I have listened to your public comments.

Ms O'CONNOR - Have you read any of the research?

Mr ROCKLIFF - I have read a lot of research, particularly when it comes to long COVID-19. That is why we have our own areas where we are reaching into the long COVID-19 support for individuals, which I announced in Sheffield last year.

Ms O'CONNOR - If you had read the research on long COVID-19, you would understand that it relates to viral persistence in the body. How can you get up in parliament as the Premier and Minister for Health and not confirm that COVID-19 is airborne - you refuse to - and then tell people, as you did last week, that in order to protect themselves from COVID-19 they should wash their hands and take personal responsibility,?

Mr ROCKLIFF - There is an area of personal responsibility for all of us.

PUBLIC

Ms O'CONNOR - It used to be called public health, before public health was sidelined by your Government and every other government in Australia.

Mr ROCKLIFF - We take advice from public health.

Ms O'CONNOR - And you ignore it, don't you?

Mr ROCKLIFF - No, we have always worked very closely with public health.

Ms O'CONNOR - Is Dr Veitch hiding in a cupboard somewhere in the Department of Health? Where have you put him?

Mr ROCKLIFF - Dr Veitch is not in a cupboard. He is still working and we have worked with the Director of Public Health along the way since the start of COVID-19.

Ms O'CONNOR - Just one final question on this line of questioning. Can you confirm that neither yourself nor your Director of Public Health has taken questions from the media at a COVID-19 press conference since July last year?

Mr ROCKLIFF - I couldn't speak for Dr Veitch. I get COVID-19 questions in media conferences from time to time.

Ms O'CONNOR - It's very reactive, though, isn't it? You're not doing anything proactive to help Tasmanians minimise the risk of infection, reinfection, disability and death. What are you doing?

Mr ROCKLIFF - We're investing significantly in COVID@homeplus and we're still reinforcing COVID-safe behaviours.

Ms O'CONNOR - Once people are infected.

CHAIR - Ms O'Connor, please let the Premier answer.

Mr ROCKLIFF - We are encouraging vaccination. We have a winter flu and respiratory illness strategy which we launched last week, if my memory serve me correctly.

Ms O'CONNOR - You know COVID is not just a respiratory illness, don't you? It's a vascular illness, it gets into every organ in the body.

Mr YOUNG - Premier, you recently launched the kids care clinics at the Kingston Neighbourhood Centre in my electorate of Franklin. This is a key part of the Child and Youth Wellbeing Strategy. Can you provide a short update on the strategy and the highlights from the second year of the action plan?

Mr ROCKLIFF - Thank you, Mr Young. The kids care clinics are providing accessible comprehensive health assessments for vulnerable children in local communities across the state and it is a very valuable new service for Tasmanian children and their families. A total of \$6 million has been allocated over three years for these clinics and they focus very much on early identification of health and wellbeing concerns. Children can be

referred to kids care clinics through a range of ways, including the Strong Families, Safe Kids advice and referral line and maternity services, for example, unborn alerts, and from Aboriginal community-controlled health organisations.

The first clinic was launched on 29 March at Kingston, with nine clinics now in operation, six in the south at Bridgewater, New Norfolk, Clarendon Vale, Kingston, Cygnet, Clarence and the Royal Hobart Hospital; two in the north at Ravenswood and Georgetown; and one in the north-west at East Devonport. Extra clinics will be established at St Helens and Queenstown later in 2023.

In 2021 we launched the \$100 million Child and Youth Wellbeing Strategy, It Takes a Tasmanian Village. This was another first for our state and we are now well into second year of the strategy, with significant progress. I will touch quickly on some highlights: the statewide expansion of the Bringing Baby Home program, including additional residential and in-home capability to support vulnerable parents in the north; Playgroup Tasmania and Families Tasmania supporting parents through the implementation of volunteer-led supported playgroups and [inaudible] villages in rural and regional areas; commencement of the second stage outreach model from Child and Family Learning Centres, with the appointment of a family engagement worker in Latrobe; continued support for young parents through the Supporting Expecting and Parenting Teens program, with 84 young parents currently participating in the program; increased access to information and support for our community kinship carers through a dedicated porthole on the FindHelpTAS online service directory; participation of over 50 children in kids' day events recently held at the Mary Hutchinson Women's Prison and Ron Barwick Prison; launch of the [inaudible] Tasmanian project which includes the provision of baby packs for all Tasmanian babies born from 1 January 2023 and a website with information links and Tasmanian stories to support parents in the early years; implementation of the Youth Connector program at the Glenorchy Jobs Hub which has supported 46 young people into employment since November 2022; formation of a palawa child-safe and supported policy partnership working group to help reduce the overrepresentation of Aboriginal children and young people in our Child Safety and out-of-home care systems; and the recent launch on 9 May 2023 of Your Engagement Place, or YEP, by the Commissioner for Children and Young People, which provides opportunities for Tasmanian children and young people to have their voices heard and share their views on topics that are important to them.

Ms WHITE - Premier, in your state of the state Address this year you made a number of different announcements, including for a state demographer. Do you remain committed to appointing a state demographer? What is the timeline for appointment? Where is the allocation in the Budget?

Mr ROCKLIFF - Yes, I remain committed to that and we're working with the Department of State Growth on that matter.

Ms WHITE - I couldn't see any allocation in the Budget for a state demographer, or mention of a state demographer in the budget papers.

Mr ROCKLIFF - But we are committed to it, and we will -

Ms WHITE - When? This year, 10 years?

Mr ROCKLIFF - Within the next 12 months we will work through that. I can get some information for you if you'd like.

Ms WHITE - Do you need me to put that question on notice? I also didn't put a question on notice earlier when I asked -

Mr ROCKLIFF - It's for the Department of State Growth, not DPAC, but you can put a question on notice with respect to that matter of state demographer, which will be when you expect the person to be appointed.

Ms WHITE - Yes. I wanted to ask as well about the commitment you made to a demand and feasibility study for new convention facilities in Launceston, which you also announced in your state of the state Address. I haven't, at this point in time, seen any further development on that, there was no allocation again in the Budget for this and I don't believe anything has been circulated publicly. Do you remain committed to doing that?

Mr ROCKLIFF - Yes, we can cover that in the Tourism output.

Ms WHITE - There are lots of other things I'd like to cover in the Tourism output too. I'm asking about this because you made this statement in your state of the state Address.

Mr ROCKLIFF - We are committed to it.

Ms WHITE - You are committed to it? So when will that work start?

Mr ROCKLIFF - We will work with relevant agencies to ensure that work does start.

Ms WHITE - So you haven't started it yet?

Mr ROCKLIFF - I will seek advice as to exactly where that project or that commitment is at.

Ms WHITE - Do I need to put that question on notice?

Mr ROCKLIFF - If you would like to.

Ms WHITE - Okay. You also talked in your state of the state Address about a whole-of-government strategy to address the digital divide through innovative solutions. There wasn't an announcement about that strategy that I could see in the Budget. Do you remain committed to a whole-of-government strategy to address the digital divide?

Mr ROCKLIFF - Yes.

Ms WHITE - Why was there no money in the Budget for it?

Mr ROCKLIFF - Our Digital Future, the Tasmanian Government's digital transformation strategy, articulates the Government's commitment to helping and inspiring our communities, businesses, industries and government agencies to develop digital maturity.

It establishes the Tasmanian Government's vision, priorities, principles and objectives for digital transformation across three critical priority areas, the community, the economy and government. The Tasmanian State Service review also aligns closely with the priorities for digital government outlined in Our Digital Future. The review noted that digitisation and digitalisation represents a core strategy that could help secure efficiencies in the delivery of government services and improve the experience of working in and with government.

The review recommended a three-tiered strategy for digital reform and the central allocation of funding to incentivise the digitisation of manual process to bring existing services up to the standard that meets current community expectations; implementation of a functional leadership model across the key digital platforms that provide the foundation for digitalisation across the State Service; and to support whole-of-government coordination and governance of digitalisation with a renewed mandate to drive whole-of-government consistency and improvements in the foundations of digitalisation. The Government will increase its commitment to digital transformation in 2023-24 with \$78.6 million allocated for investments in ICT to support service delivery, up from \$68.7 million from the 2022-23 budget. And through 2023-24, we will continue to progress strategic investments through the digital transformation priority expenditure program, the digital health transformation program and Project Unify with the Department of Police, Fire and Emergency Management.

Over the forward Estimates, the Government has allocated \$311 million for investment in ICT. This is in addition to ongoing operational expenditures for agency ICT services. And we have made an investment in the digital divide; there is a commitment there of \$150 000 this financial year, 2023-24 Budget.

The Government has committed \$4.3 million over four years in the 2021-22 budget to begin development of the digital Service Tasmania portal. This aligns with recommendations of the independent State Service review and will provide Tasmanians with a secure and easy-to-use access point for government services, accessed through a single log-in. The development of the portal has been led by a local Tasmanian company, in partnership with major industry players, which will ensure the solution is secure, built on best practice and tailored to the needs of Tasmanians. Service Tasmania established a customer experience program in 2021 to drive this and key supporting initiatives under this funding, which are the new Service Tasmania -

Ms WHITE - Are you delivering a whole-of-government strategy, which is what you announced in the state of the state? This sounds like funding allocated to different departments to digitise their records or improved their navigability.

Mr ROCKLIFF - This Budget provides funding associated with the digital divide through development of a Tasmanian government digital inclusion strategy. This will include the development of an agreed and coordinated set of priorities for digital inclusion across government and with external stakeholders. It will establish a mechanism to work with the community and industry to create a sustainable and viable inclusion strategy and to validate performance indicators that will allow the Government to measure the value of specific efforts -

Ms WHITE - And that's due when?

Mr ROCKLIFF - Twelve months' worth of funding.

Ms O'CONNOR - I want to talk to you about conversion practices and acknowledge up front your personal commitment to this reform. On 17 May it was a year since the Tasmanian Law Reform Institute released its report which recommended a prohibition on harmful conversion practices. It's a year later. On 5 June, are you personally committed to this reform? When do you think we will see draft legislation? Do you think it will be a Government bill or a private member's bill?

Mr ROCKLIFF - Of course, we want Tasmania to be a place where everyone feels supported, valued and encouraged to be the best they can be. I've said that a number of times. I've considered the Tasmanian Law Reform Institute's report. I have met personally with people who have been subjected to conversion practices in the past. I've always acknowledged the harm and distress that these practices have caused and I do so again today.

I am, of course, aware that other jurisdictions have introduced various laws in relation to this matter. This is precisely why the Tasmanian Law Reform Institute has looked into this matter to consider options and areas that might need review in terms of our laws. Their report is very extensive, as you know.

It is a complex matter and it is important that we get this right. The departments of Health and Justice have been undertaking policy analysis to properly evaluate the TLRI's recommendations and provide further advice to the Government around reform. We will make those decisions on the basis of expert advice and best practice.

Ms O'CONNOR - Can I ask for more detail there? When you say 'make those decisions', what sort of decisions are you talking about? Because you've committed to the reform.

Mr ROCKLIFF - Commitment to reform.

Ms O'CONNOR - But I thought your Government or you had decided that this reform would be implemented.

Mr ROCKLIFF - Yes.

Ms O'CONNOR - So what decisions do you have to make?

Mr ROCKLIFF - The decisions of the aspects of the legislation. So, there is the ban need. But there is also the need, in a policy and legislative sense, to ensure that we're not impinging on people's rights to freedom of speech and prayer, and those matters that have been raised by others in the community.

Ms O'CONNOR - More recently on radio you said:

I remain committed to banning that practice, but we want to make sure that, of course, people have the opportunity of making sure that freedom of speech is protected, and the churches' right to pray and all those matters that seem to be concerning some people in the community.

What do you mean by this? Are you suggesting that, for example, the Australian Christian Lobby (ACL) will have an influence on the form of that legislation?

Mr ROCKLIFF - They would have a right, like every other Tasmanian, to comment on it when it's out for draft consultation.

Ms OCONNOR - But what do you mean by that statement you made on radio? Are you flagging a sort of weaker ban on conversion practices that would still allow some churches to try to 'pray the gay away'?

Mr ROCKLIFF - No, I wouldn't put it that way at all. I have committed to the banning off conversion practices. I remain committed to the banning of conversion practices. I want to ensure that in that ban we're not impinging on people's right to freedom of speech, for example. I know that's been a key concern of a number of people in the community,

Ms O'CONNOR - I think your diary says you had a meeting with the Australian Christian Lobby?

Mr ROCKLIFF - I did. You read my diary, in an open and transparent manner.

Ms O'CONNOR - It's very good to see the diaries become public. But, in that meeting with the ACL - which I don't think reflects the churches more broadly - did you make it clear to them that you didn't believe that you can pray the gay away? And did you make it clear to them that attempting to convert someone to not being who they are, is harmful? Or did you just sit and listen to their concerns?

Mr ROCKLIFF - I don't want to talk about private meetings I've had with individuals in terms of what is said -

Ms O'CONNOR - What about what you said?

Mr ROCKLIFF - because of privacy. We had some robust discussions. You'd be well aware of the leaflets with my head on them that have been put in mailboxes. That didn't sit well with me and I made my feelings known, as you'd expect me to do.

Ms O'CONNOR - A final question on this line. Given the difficulties you have within your Cabinet, and now minority; and that at least one of the members who defected is concerned about the ban on conversion practices, because she's very conservative; are you prepared to push this reform through as a private member's bill?

Mr ROCKLIFF - I want to come good on my commitment.

Ms O'CONNOR - That's right. So, what you going to do about it?

Mr ROCKLIFF - I haven't made a decision with respect to that matter, but I would like to present a Government bill. That would be my aim.

Ms O'Connor - This year?

PUBLIC

Mr ROCKLIFF - In this term of parliament. I'm not pushing it away. I'm very mindful of the fact that I've been open with Advocates for Change, that I'm still committed to reform, but -

Ms O'Connor - But you don't want to blow up your Government over it?

Mr ROCKLIFF - No, that's not what I was going to say.

Ms O'Connor - Sorry. I was just trying to be helpful.

Mr ROCKLIFF - There are a lot of competing interests.

Ms WHITE - You haven't tabled legislation at a rapid rate. You've barely tabled any.

Mr ROCKLIFF - Well, that's another matter. But I can assure you, Ms O'Connor, that I am committed to reform in this term of parliament. I've done some work on this. I've met with a number of stakeholders from both sides of the debate, and I am committed to reform.

Ms O'CONNOR - Thank you.

Mr YOUNG - Premier, last year in October there were severe floods in parts of the state and a lot of residential and commercial properties impacted as well as damage to government infrastructure. Can you please provide an update on assistance provided to those impacted communities?

Mr ROCKLIFF - I can, and thank you. The Budget provides a \$30.4 million over two years to fund a recovery package primarily designed to address infrastructure and economic recovery. It includes funding to assist impacted communities with the costs of clean up and restoration. This includes funding of \$19.4 million in 2023-24, along with a further \$10.9 million in 2024-25 for disaster assistance for flood-impacted communities, continuing recovery activities in response to the 2022 flooding events. This is a significant increase in funding over last year's Budget. The funding will go to support the reconstruction of important state and local government-owned environmental, recreational and community assets.

The October floods resulted in significant flooding across 18 local government areas. Evacuation centres were opened in Railton, Latrobe, Deloraine, Ulverstone and Launceston. The Government also provided temporary emergency accommodation in hotel rooms to 10 vulnerable evacuees until they were able to return home.

Our Government works in partnership with the Australian Government in situations like this. Commonwealth-state disaster recovery funding arrangements were activated. Grants were made available to assist individuals and families, primary producers and small businesses, not-for-profit organisations, local governments and local communities in their recovery. Financial assistance packages were designed to help with displacement from homes, to assist with the clean-up and restoration and rebuilding of infrastructure and assets.

PUBLIC

It's now been six months since the flood event and the majority of financial assistance packages have closed or have been paid or are still being assessed. Applications for primary producers remain open. Our Government continues to work with mayors of affected councils through flood recovery advisory groups established which allow us to ensure that assistance is meeting the community's needs and to better prepare for such future events.

The Government's total committed expenditure to 31 March this year is over \$14 million and includes over \$2 million in grants to the community and approximately \$12 million to reconstruct damaged recreational community and tourism infrastructure. In addition, joint funding has been provided to support the reconstruction of public assets such as roads and bridges. Our Government also activated a new Small Business Recovery Critical Support Grant for small businesses impacted by the full road closure of St Marys Pass and the Sideling. The cost of this will be borne fully by the Tasmanian Government.

I visited a number of properties affected as a result of the floods and went to a place in Deloraine - I think it was a building construction business -

Ms White - A haulage business.

Mr ROCKLIFF - Yes, it was. It had all the different flood levels on the walls from 2006, 2011, 2016 and then the flood in 2022. From memory, this was the highest flood of all those floods. Deloraine was severely impacted.

Ms O'Connor - More coming, unfortunately.

Ms WHITE - Premier, when your Government was making a real debacle about the removal of the Burnie Court from the Burnie CBD, former mayor Kons put in a request for revitalisation funds for the CBD. The former premier, Peter Gutwein, agreed, and I believe you did too that your Government would look at funding to help revitalise the Burnie CBD. There was discussion about a funding amount of about \$20 million. There's no money in the Budget to help revitalise the Burnie CBD. Can you explain why you have gone back on your word?

Mr ROCKLIFF - I can't account for discussions with my predecessor but I have spoken to the mayor of Burnie more recently about the revitalisation. We are very keen to assist where possible once that is well-consulted with the community. They have their own plans. We have a significant investment of \$86 million into the court there, which is in the Budget and is a big investment.

Ms WHITE - And in the CBD, which is a good decision.

Mr ROCKLIFF - Yes, in the CBD, which will add value to the CBD urban renewal project, if I can put it that way; the mayor may have a different way of describing that. When I look at urban renewal I like to look at places such as Devonport, which was very controversial at the time where we invested as a state, and the federal government invested as well, as did local government, and that was a very controversial project which has since been embraced. Private investment has come to support what has been public expenditure of funds as well and I actually see Mac Point in a similar vein.

PUBLIC

Ms WHITE - Burnie is in Braddon, your own electorate. I believe you joined with the then premier in committing at looking at funds for the Burnie CBD revitalisation. You have not put any money in the Budget for it. How much longer do the residents of Burnie need to wait?

Mr ROCKLIFF - I will be very interested to see the plans for urban renewal in the Burnie CBD and have a discussion with the mayor and the council on what might be possible. I am very conscious of the fact that as a state government we invested in the city of Devonport, invested in the Paranple centre, the new library, and there was federal government funding committed as well as local government. I would be very interested to have a conversation.

Ms WHITE - I want to ask about questions that were raised at the TasCOSS forum about the inadequacy of your Budget to support the community services sector. I want to understand if you agree with your Minister for Community Services and Development that the funding for Community Services announced in the Budget was inadequate?

Mr ROCKLIFF - Firstly, I support minister Street, who is doing a fantastic job with all his portfolio responsibilities. He is very committed, engages with all across the sector and has a very good working relationship with all the organisations he represents. I am very proud of the work he does.

What our Budget provides is significant cost-of-living relief over the next four years, straight into the pockets of Tasmanians who need it most. I have mentioned before that there is some \$347 million in concessions together with the state's \$45 million contribution to the national Energy Bill Relief Fund. This year's Budget also provides \$2.1 million in community funding support to help ease the pressures of rising costs of service delivery. This amounts to a total indexation rate of 4 per cent this year for all community service organisations receiving indexation.

On top of this, through our negotiations with the Australian Government we have ensured that the community sector organisations are eligible to access the national Energy Bill Relief Fund to which we are providing \$45 million. This means that many CSOs will be able to access the \$650 in energy relief to reduce their operational costs. Further, our Government will provide long-term certainty in funding to CSOs by moving to five-year funding agreements by the end of 2024, which is very important in terms of workforce development and those matters.

Ms O'Connor - Very good.

Mr ROCKLIFF - Given what TasCOSS have said about the high cost of recruitment and retention of staff in the community sector, long-term funding agreements can go a long way to significantly reduce those costs that often run into the hundreds of thousands of dollars. Alongside this, the Tasmanian Government is completing a comprehensive review of funding models for CSOs to ensure that funding provides long-term certainty, deliverables and outcomes are clear and service provision can be flexible and adaptable. There are multiple components to this work which require detailed analysis and also needs alignment with national policy such as a standalone index for the community sector. Indexation is only one aspect of this work. Peak body funding is another aspect, as is the outcomes framework.

We want to ensure there is an equitable outcome for all CSOs through this work and we are committed to getting this right and working closely with the sector to ensure the sustainable, efficient and effective delivery of community services in Tasmania into the future.

Ms WHITE - The cost of living is putting increasing pressure on households and that means these services are being placed under unprecedented demand. Will your Government commit to reviewing its current budget decisions and work with the community service organisations? You have said they will have to cut jobs or reduce services, and at a time when most Tasmanians cannot afford to see a reduction in these services it's quite critical right now and a lot of these CSOs are really stressed about the position they find themselves in and can't wait until the end of 2024.

Mr ROCKLIFF - Minister Street is working closely with the organisations and I will too as well. I respect the people involved, I respect TasCOSS and have spoken with them at a public forum as well earlier this year, where I raised the matter of indexation and the matter of having a national index rate for community service organisations, and I wrote to the Prime Minister regarding that as well. I will always be willing to sit down with organisations. The next budget process commences shortly and I am very interested to hear the views of those who represent the community services sector so well.

Ms O'CONNOR - Premier, you often talk about Tasmania having the world's best-practice salmon industry. We would like to understand why you don't make those corporations pay more. The Norwegian government - which country arguably has the world's best-practice salmon industry and has a strong social licence - has announced a 35 per cent tax on cash flow. Given that you recently attended a dinner that cost \$4500 a ticket with one of the Batista brothers, are you more inclined to do what JBS asks of you, or in terms of the finances of the salmon industry will you do what is the right thing to do by Tasmania's Budget? The Norwegian government argued that those natural resources belong to the people of Norway and therefore the resource rent of those resources needed to be reinvested in the Norwegian people.

Mr ROCKLIFF - That is a good question, and we have reached into that general area in terms of cost recovery. We provide a range of services and support for Tasmania's salmon industry and some of these services are partially recovered from the industry through existing fees and charges. The access provided to Tasmania's natural resources enables industry success and profitability. Cost recovery is an important aspect of ensuring that regulatory services relating to the salmon industry are well resourced. One of the principles underpinning our Government's new salmon plan is a review of fees and charges to ensure full cost recovery, and an appropriate return to the Tasmanian community, so that is part of our salmon plan that has been recently released. The guidelines for costing fees and charges published by the Tasmanian Department of Treasury and Finance set out that fees and charges should initially be costed on a full cost recovery basis, and that social implications of applying the fee or charge should then be considered when determining whether a government subsidy is warranted for some users.

Ms O'CONNOR - Can I just pull you up there because we've got 90 seconds left? Henry Batista was in the *Mercury* the week before last calling for a complete rewrite of the rules and regulations that apply to the salmon industry so that JBS can expand. Are you

PUBLIC

prepared to rule out bowing to JBS's will and not weakening the rules and regulations but strengthening them around the salmon industry?

Mr ROCKLIFF - We have strengthened them.

Ms O'CONNOR - What are you going to do in relation to his call for a rewrite of the rules?

Mr ROCKLIFF - We're not going to rewrite the rules.

Ms O'CONNOR - Have you told him that, because he paid \$4400 for a ticket to have dinner with you and probably expects that investment to pay off?

Mr ROCKLIFF - Ms O'Connor, we have strengthened continuously the rules and regulations around salmon farming in Tasmania -

Ms O'Connor - And yet the Maugean skate is almost extinct.

CHAIR - Order, Ms O'Connor, let the Premier answer the question.

Mr ROCKLIFF - proudly, I have to say, led by myself as minister for Primary Industries between 2014 and 2018, when we did have some quite considerable reform. We've spoken about that. We will continue to base regulation, transparency and environmental monitoring on a continuous improvement basis.

CHAIR - As the time is now past 12 p.m., we will now be moving from Budget Estimates for the Premier to Minister for Tourism.

Ms White - Sorry, I thought we got the time added on. I forgot. At GBEs you add the time when you have a break. This one you don't.

Mr Rockliff - Don't you. I thought you do add the time on.

Ms O'Connor - I don't think we lost any time, did we?

CHAIR - With the break, we lost five or six minutes there. People will be watching what we are doing so we have to try to finish things on time.

MINISTER FOR TOURISM

CHAIR - I ask the Premier if he could please introduce the persons at the table, names and positions, for the benefit of Hansard. Would the Premier also like to make a short opening statement?

Mr ROCKLIFF - Tourism is one of Tasmania's greatest strengths and a key pillar of our economy -

CHAIR - Premier, could you introduce your staff?

Mr ROCKLIFF - Oh sorry. To my left, Kim Evans, the secretary of the department, to my right, Sarah Clark, the CEO of Tourism Tasmania, Mark Bowles, deputy secretary.

Despite the very significant impacts of recent years, the resilience, grit and determination of our tourism and hospitality operators, and the strategic planning of our government agencies and industry bodies is paying off as we are witnessing strong post-COVID-19 recovery. The latest indicative data from the Tasmanian Visitor Survey shows that total visitor expenditure in the year ending March 2023 was \$3.95 billion. This is the highest annual spend on record, up 55 per cent from when compared to pre-COVID-19 December 2019 figures, despite 3 per cent fewer visitors. These results continue to be driven by Tourism Tasmania's *Come Down for Air* and *Off Season* campaigns drawings visitors to our island.

However, there is no doubt that after a period of exploring their own backyard, Australians are once again feeling more comfortable with overseas travel. This means that Tasmania is again operating in a very competitive marketplace, not only against other Australian states and territories, but also international destinations. This is why our investment in destination marketing is so important to ensure we maintain our market share and travellers continue to visit and spend across the state.

We simply cannot underestimate the importance of our tourism industry for our economy. That's why we are planning for the future. What is critical to the successful tourism offering is our people, and addressing the workforce challenges facing the tourism and hospitality industry is a critical action for both the industry and Government. Our tourism and hospitality workforce -

Ms WHITE - Chair, can I suggest the Premier table his speech? It is very long.

CHAIR - I am sure the Premier will sum it up very quickly.

Mr ROCKLIFF - Thank you, Chair. I had some terrific news to share. The 2023-24 state Budget recognises the importance of our visitor economy and the role of destination marketing. It provides Tourism Tasmania with long-term certainty in marketing Tasmania to domestic and international markets, with \$6 million committed to 2025-26 to continue Government's strong level of investment in destination marketing.

I respect the committee's view and I will finish up here. A critical part of our strategy is to ensure our visitors travel right across the state. That's why we are investing in our regions through infrastructure, our products and experiences as well.

I just want to quickly mention the 2030 Visitor Economy Strategy. It will set out our vision and directions to continue to grow the depth and breadth of our positive impact across the state. It has been developed in partnership with industry through the TICT and supported by the Visitor Economy Advisory Council. Contemporary forecasting, global trends and insights over 500 industry and broader stakeholders have formed the vision and key directions of this important section of the economy to 2030. The directions paper for the strategy was released last week for community consultation. I encourage Tasmanians to read the document and have their say.

There you go, I've cut it in more than half.

Ms O'Connor - Tell your department not to write such a long intro for half-hour Estimates.

CHAIR - Order.

Ms WHITE - Premier, the tourism industry in the north west is left feeling a bit gutted by your failure to secure the funding promised by the federal government for the Cradle Mountain Cableway. The industry secured that \$1 million from the federal government to fund a business case that your Coordinator-General has been dealing with for five years now. It has taken them five years to complete. Will you commit to sharing publicly that business case? And can you table it today?

Mr ROCKLIFF - The Cradle Valley Cableway is a component of the Cradle Mountain Visitor Experience Master Plan. I am assuming you support the cableway?

Ms O'Connor - No.

Ms WHITE - It's had bipartisan support from the outset and that's why it's such a disaster that you've lost the funding.

Mr ROCKLIFF - No. Let's be sensible about all of this.

Ms WHITE - Why did you take five years to develop a business case?

Mr ROCKLIFF - It is comprehensive investment, and comprehensive analysis of potential Cradle Valley Cableway routes has been undertaken over recent years to:

1. Determine what is technically feasible with the least environmental and visual impact.
2. Identify potential cable way alignment concept design, including integration with the gateway village precinct
3. Refine the complete scope of works and associated total project costs.

The cableway business case has been considered by Cabinet and has been provided to the Australian Government. We wrote to both the Prime Minister and the minister, Ms King, ahead of the federal department's program deadline and we remain committed to the project.

Ms WHITE - Did you provide the business case ahead of the deadline?

Mr ROCKLIFF - We wrote to both the Prime Minister - that was on 3 March, and the minister ahead of the federal department's program deadline. We have requested that \$30 million in matched federal funding committed to the Cradle Valley Cableway project be preserved -

Ms WHITE - Did you provide the business case -

CHAIR - Ms White, can you wait until the Premier finishes answering.

Mr ROCKLIFF - while discussions continue between the two governments on the best way to progress the project. At this time, we are yet to receive a response from the Prime Minister.

The Tasmanian Government has completed the business case, which includes a comprehensive analysis of potential cableway routes to determine what is technically feasible and compatible with the World Heritage status of this iconic area.

The previous Australian government had determined to provide its matched funding of \$30 million for this project through the Community Development Grant Program. However, the current Australian Government announced last year that the CDG program would cease and, while it would honour existing projects, it reinforced the condition that all monies were required to be expended by 2026. The Australian Government confirmed that this state would not be extended, meaning it was no longer possible for the state to progress the project under that particular program.

We want to ensure that the commonwealth's committed funds are preserved for Tasmania while we work through the implications of the business case, which estimates the project to now total \$190 million, based on materials and other construction costs in this remote location. Should the business case be approved by the Tasmanian and Australian governments, the project will need to go through extensive levels of environmental and planning approvals and community consultation before it can advance to the construction stage.

Ms WHITE - You said the Cabinet had already approved it. Why does it still need to get approval from the Tasmanian Government?

Mr ROCKLIFF - Tasmanian and Australian governments. We have worked the matters through our Cabinet but the cost is now \$190 million.

Ms WHITE - Premier, the project was announced in 2016. The federal government has given you 10 years to complete it by. I don't think that's unreasonable. The problem is that you took five years to develop a business case and the federal department said that:

Since the department did not receive the required information from the Tasmanian Government by the 28 April 2023, it was unable to complete an assessment and the funding commitment was withdrawn.

Why did you fail to provide the information by April this year? Why has the Coordinator-General's office taken five years to complete the business case?

Mr ROCKLIFF - I am advised the Office of the Coordinator-General provided a briefing on the project, including an overview of the business case, to the department area on 24 March this year. I am talking about the Australia Government department area responsible for the CDG Program. I am advised that the business case was subsequently provided to the Australian Government on 5 May this year and a further meeting between officials and the Office of the Coordinator-General was held on 15 May.

Ms White - It was due in April, so why didn't it provide it by the time line?

CHAIR - Ms White, order.

Mr ROCKLIFF - The Office of the Coordinator-General is awaiting feedback from that meeting.

Ms White - How incompetent are you?

CHAIR - Ms White.

Ms White - Honestly!

Mr ROCKLIFF - The business case has been provided to the Australian Government for the purposes of facilitating discussion on the best way to proceed. At this time, it would be premature for it to be released.

Back in 2017, the Tasmanian Government identified the cableway as a strategic priority and has committed \$30 million to its construction.

Ms O'Connor - Such a priority.

Mr ROCKLIFF - Given the sensitivity -

Ms White - Nothing's happened.

Ms O'Connor - Five years.

CHAIR - Order.

Mr ROCKLIFF - We can talk about this, Ms O'Connor. We still have the challenge of thousands of people accessing a beautiful area and we need to do it in a sustainable way.

Ms White - So it will never happen.

Mr ROCKLIFF - A low-impact cableway is a form of transport that is low impact in terms of carbon footprint. When we're thinking about accessible tourism for people with a disability, it is also a very good mode of transport.

Given the sensitivity and complexities of the project, it was important that we got the business case right. In developing the business case, comprehensive analysis of potential cableway routes has been undertaken to determine what is technically feasible with the least environmental and visual impact. This important work took time to complete and was impacted by the COVID-19 pandemic.

Ms O'CONNOR - Premier, are you able to tell the committee how much the Tasmanian Walking Company, who I note you met with in your diary, charged the Tasmanian

PUBLIC

Government for its costs of marketing the Three Capes Track in the 2022 calendar year and the 2022-23 financial year?

Mr ROCKLIFF - Could you direct that question to Parks?

Ms O'CONNOR - Yes, unfortunately, it does come out of the Parks budget.

Premier, in response to a question from Mr Tucker last week on plantation forests, you went on a reasonably out of character reconstructed rant and referenced the dozens of people who signed this open letter to you calling for an end to native forest logging after Victoria announced it was ending it and ending the rivers of public money into the industry. You called them a lot of privileged people who do not have to think about where the next dollar comes from.

First of all, you do not know anything about the life histories of any of the signatories. As Tourism minister is that how you would describe tourism operators like, Tara who is on the TICT board, Steve Howell from Blue Derby Pods, Ben Ray from Tasmanian eBike Adventures, Simon French from Maydena Bike Park and Greg Irons from Bonorong Wildlife Sanctuary? Do think they are out of touch and privileged?

Mr ROCKLIFF - I am not going to comment of individuals at all, Ms O'Connor.

Ms O'CONNOR - Well, you basically did when you wrote them all off as part of some privileged set.

CHAIR - Ms O'Connor, please let the Premier answer the question.

Mr ROCKLIFF - I did not say 100 per cent of the people there, I am sure I didn't. If I did I stand to be corrected. I was really making the point in comparison to families that have been devastated or are in the midst of being devastating in Victoria as a result of the closure of native forest logging or bringing that earlier from 2030 to 2024. How devastating it is for them. Families have had their lives turned upside down. They will be, at this present time, wondering where their next dollar is going to come from.

Ms O'CONNOR - I understand that and that is why Government has a role in transition.

CHAIR - Ms O'Connor. Please let the Premier answer.

Mr ROCKLIFF - Nothing wrong with expressing a bit of emotion on behalf of hardworking people, albeit in Victoria. I met with many hardworking people between 2010-14 when we had our forest industry devastated.

Ms O'CONNOR - Don't deflect. You stood up in parliament and you have talked about our positive climate profile, which has come about as a result of the forest saved under the Tasmanian Forest Agreement. Surely you can acknowledge the tourism potential of our forests. We have the biggest trees and best trees in the country. We're the big tree state. Do you think people come here to see clear fell and burn sites or do you think there is more to be

PUBLIC

gained in joining what is now a national move, as part of a global move, out of native forest logging? Except for in places like Brazil and the Congo where they still do what we do here.

Mr ROCKLIFF - I do not think you can compare Brazil and the Congo.

Ms O'CONNOR - Yes, you can.

Mr ROCKLIFF - No, you can't.

CHAIR - Ms O'Connor, let the Premier answer, please.

Mr ROCKLIFF - We have a very specialist native timber industry. My view is that people come to Tasmania to experience our wonderful built heritage, our natural environment, our beautiful clean beaches and waters. It is a fundamental part of the attraction. Our tourism industry can co-exist with more traditional resourced-based industries that go about their daily jobs in a very sustainable way in order to protect the longevity of their industry.

Ms O'CONNOR - As you would be aware, Premier, when you talk about sustainability, there's massive loss of carbon. I don't know if you've read Dr Jen Sanger's report, the one your minister's love to denigrate, a massive loss of carbon, loss of biodiversity, loss of water quality as a result of a native forest logging industry, which does not align with Tasmania's brand.

Mr ROCKLIFF - What I am in favour of is a resourced-based sector that is sustainable whether that be salmon or aquaculture, agriculture, mining and forestry, we can all co-exist, Ms O'Connor, provided we respect the resources that we have. I am a great believer in strong natural resource management, I always have been, given my background - water quality, soil management, sustainable timber harvesting. But also, we can demonstrate to the world how well we do this in Tasmania.

Ms O'CONNOR - One last question on this line of questioning. Premier, as you know we export about 1 million tonnes of native forest woodchips overseas each year, which is a massive climate crime. But, Tourism Tasmania's promotion says, 'Come down for air'. How do you think that aligns with the autumn forestry burns, which not only blight the skies but make people with respiratory and cardiovascular conditions in small regional communities around the state, like Huonville, quite sick? Do you think that Tourism Tasmania's 'Come down for air' campaign has authenticity to it, given the autumn burns by Forestry Tasmania?

Mr ROCKLIFF - I do believe it does have authenticity because it is a very successful campaign, which I can speak about.

Ms O'CONNOR - No, just answer the question rather than reading from a brief. Why don't you just answer the question?

Mr ROCKLIFF - Yes, no, I have.

Ms O'CONNOR - Have you?

Mr ROCKLIFF - 'Come down for air', is a play on words. That is an invitation to come down to Tasmania and take a break from the stress and routine. It is a playful twist on a familiar phrase, which means it is good for retention and people would easily understand and get the concept. Tourism Tasmania's campaigns are more than a tag line. We aim to celebrate Tasmania and Tasmanians. The impact of smoke on tourism operators and event providers, of course, is unfortunate -

Ms O'Connor - It is becoming untenable. What are you doing about it?

CHAIR - Ms O'Connor, can you let the Premier answer the question.

Mr ROCKLIFF - I understand there is a commitment from Sustainable Timbers Tasmania to ongoing engagement with operators around the timing of future burns. Our Government is committed to a policy of the two industries coexisting, what I was talking about before. There are good examples of where this is happening, as I understand it.

Ms WHITE - You may have heard Leigh Carmichael on ABC radio, he was on Friday morning, where he said that it took four years for them to get planning approval for the *Transformer* project and they remain committed to developing it, but will wait to see what the construction and supply market looked like in the future rather than build it now. Does this impact on the funding commitment that your Government has provided towards this project? And can you detail exactly how much funding has been provided towards this project?

Mr ROCKLIFF - I will do my best to do that. *Transformer* is a proposed tourism attraction in the Ida Bay State Reserve being developed by Dark Lab. The project is expected to provide significant economic benefits to the Huon Valley region. The Tasmanian Government has committed some \$2.7 million to *Transformer*, with the Australian Government and Museum of Old and New Art, MONA, also supporting the project.

The \$2.7 million to Dark Lab to develop this exciting public artwork in the Huon Valley, named *Transformer*, will drive visitation to the region. In March 2022, the Department of State Growth entered into a grant deed with Dark Lab for the project. The artwork involves an enclosed viewing platform with mirrored interiors which provides visitors with a unique space to contemplate the Tasmanian landscape.

Ms WHITE - Thank you, you have answered one of my questions, which is how much has been contributed. But given that Mr Carmichael indicated they weren't in a hurry to build it, I presume there is a deed of contract between the Government and Dark Lab. What expectations do you have for a time line for when that will be constructed?

Mr ROCKLIFF - I will come to that. *Transformer* will create 28 new jobs FTE during constructions and 28 jobs ongoing; deliver \$4.8 million of economic impact to the region, deliver \$4.2 million economic impact to the region during construction; and attract an estimated 30 000 visitors to southern Tasmania each year.

I was able to learn that on Wednesday 24 May, Huon Valley Council approved Dark Lab's new application for *Transformer*. The department is currently in the process of developing a deed of variation for the project, I'm advised, to extend its completion date

PUBLIC

following the delay related to the approval process. This project will provide, as we say, significant economic benefits to the Huon Valley and I look forward to seeing it progress.

Is there anything further we need to add to that in terms of Ms White's question?

Ms WHITE - What is the likely construction completion time now, given there is going to be a variation to the deed?

Mr EVANS - We are in discussions with them around that deed of variation and, through those discussions, we'll seek their commitment to when that project will be complete. But we'll be very receptive to varying the contract deed to support that important project moving forward.

Ms WHITE - Does it have an impact on the federal government commitment if you vary the time line for delivery?

Mr EVANS - I'm not certain about that.

Ms WHITE - Can I put a question on notice for you to provide that?

Mr EVANS - Yes.

Ms WHITE - Is it possible for you to provide in your answer to that the completion date, or is that something you haven't concluded yet?

Mr EVANS - I don't think we'd be able to provide that in time because we're in discussions at the moment.

Ms WHITE - Okay. Thank you.

Just one quick question before I finish up, Chair. On the Cradle Mountain Cableway, I asked about you releasing the business case. Can I confirm that you won't be releasing the business case?

Mr ROCKLIFF - I'll seek advice on that. The business case has been provided to the Australian Government for the purposes of facilitating discussion on the best way to proceed, so at this time it would be a bit premature for it to be released. But at the right time, based on what I've just said, Ms White, I will seek advice on releasing the business case. But I feel as though -

Ms WHITE - I will put a question on notice and you can provide an answer?

Mr ROCKLIFF - We will take advice from the Coordinator-General on that but please if you would like to put that on notice, that would be good.

Ms WHITE - I also wanted to ask about the funding in the Budget for tourism marketing. There has been an additional allocation of \$6 million across the forwards provided but then it falls off a cliff. Can you explain why you haven't continued to keep funding at that elevated level? I understand it was previously related to increased expenditure

following the impacts of COVID-19. But the market is still incredibly competitive. If we don't continue to maintain our advertising at the current rate, we will lose that market share. So, can you explain your thinking about why you've decreased the funding across the forwards for tourism advertising?

Mr ROCKLIFF - We've maintained the funding for one further year -

Ms White - For one further year but then it does fall off a cliff.

Mr ROCKLIFF - and what we will do is wait for the release of our 2030 strategy and then we'll work with our next budget to see what investment is required that aligns with the 2030 strategy.

Ms O'CONNOR - In your state of the state Address, despite the fact that it was rejected by the Hobart City Council, the planning commission and the Tasmanian Aboriginal community, you recommitted to a cable car on kunanyi. Why, like the stadium, are you so dismissive of proper planning processes, the wishes of the palawa people and also the broad Hobart community, which has gathered in thousands to reject the cable car? And what process will you use to try to jam it through against public wishes and good planning?

Mr ROCKLIFF - There will always need to be good planning.

Ms O'CONNOR - Yes, but the planning commission reached a verdict on the cable car.

Mr ROCKLIFF - Yes, I did confirm in my State of the State Address support for a cable car. We know that the tourism industry is largely supportive of sustainable transport and we are supportive of sustainable transport options. We are currently seeking advice from the Department of State Growth on options to develop a pathway to support the cable car progressing but we are committed -

Ms O'CONNOR - What will those options be? That's the question.

Mr ROCKLIFF - We are exploring that but what I'm interested in is sustainable low-impact transport.

Ms O'CONNOR - But you're not interested in the Aboriginal people's wishes or the Planning Commission's verdict.

Mr ROCKLIFF - Absolutely I am. I'm very interested.

Ms O'CONNOR - But you're prepared to ignore it.

Mr ROCKLIFF - I'm very sensitive to our First Nations people and their views and the planning process, but we still see cableways installed in iconic locations around the globe where sensitive environmental conditions exist. We still have the challenge of accessibility, the challenge of vehicles going up and down the mountain, and if we could find a way of supporting low-impact -

PUBLIC

Ms O'CONNOR - Fast-tracking it. That's what you're talking about, fast-tracking it outside a proper planning process.

Mr ROCKLIFF - Ms O'Connor, some of the greenest, environmentally sustainable places have point-of-interest cableways - that's why they have them.

Ms O'CONNOR - Do you want us to be just like everywhere else? With every gimcrack tourism idea that comes along?

Mr ROCKLIFF - No, I don't, but we can be unique in everything we do. What I'm saying -

Ms O'CONNOR - Nothing unique about a cable car.

Mr ROCKLIFF - whether it be Cradle Mountain or Mount Wellington -

Ms O'CONNOR - kunanyi.

Mr ROCKLIFF - or kunanyi, we still have the challenge of people accessing the mountain in an environmentally sustainable way. I'm interested in low-impact, sustainable transport options for the many thousands of people that will continue to come to Tasmania.

Ms O'CONNOR - It's not low impact.

Time expired.

MINISTER for TOURISM and MINISTER for TRADE

CHAIR - Premier, if you could introduce the people beside you and make a very brief opening statement.

Mr ROCKLIFF - I have Mr Kim Evans besides me, secretary of the Department of State Growth, Lara Hendriks, executive director of Trade, and Mark Bowles, deputy secretary, Business and Jobs.

I have an opening statement which I will make sure is very short. Trade is a portfolio that is a tremendous opportunity and one that I enjoy and it's fair to say that Tasmania has bucked the international trend of recent years which has seen significant trade disruptions and negatively impacted economies. In the face of this, our exports continue to grow. In 2021-22, total exports of goods and services increased by an impressive 23.6 per cent from the previous year. This has taken the value of our exports to more than \$6 billion a year. I'm pleased to advise that with less than a month to go in the current financial year, we are on track to top the \$6 billion mark again.

We are proud to say that our highly successful trade missions and delegations have played a significant role in raising the profile of Tasmanian products. I often say Tasmania has what the world wants and in recognising this, our focus has been on raising awareness on Tasmania as a quality brand, selling quality goods and services at a premium. Our stellar

export performance has been boosted by our continued focus on trade development, targeted trade missions to priority markets and the expansion of international advocacy through our international business development network of Tasmanian trade advocates.

Our Budget provides an additional \$780 000 to continue the Accelerating Trade Grant Program for a further two years. This program provides financial assistance to eligible Tasmanian businesses working to expand their global market presence. In the past 10 months, this program alone has supported new export contracts worth \$24 million, a 50 per cent increase on the prior year and a great outcome for our state.

Across the coming financial year we will also continue to drive further opportunities for new trade and investment through missions scheduled for the United Kingdom, Europe, the United States of America and New Zealand. These missions enable businesses to showcase their capabilities, forge connections with international buyers, distributors and trading partners and expand their global market presence.

Dr BROAD - Premier, earlier this year your minister, Madeleine Ogilvie, visited the United States. Was this related to trade and were any costs incurred by the Government for this US trip?

Mr ROCKLIFF - My advice is that I don't believe any costs were incurred. My understanding was that Ms Ogilvie funded the trip herself.

Dr BROAD - In DPAC's routine disclosures, Ms Ogilvie spent more than \$14 000 on international travel in November. What was this for?

Mr ROCKLIFF - I am advised that was for a trade mission to Indonesia.

Dr BROAD - Was that entire \$14 000 spent on flights alone?

Mr ROCKLIFF - I am advised it was flights, accommodation and transportation.

Dr BROAD - Did she fly first class?

Mr ROCKLIFF - I'm not aware of anyone flying first class.

Dr BROAD - It seems \$14 000 for a trade mission to Indonesia sounds quite excessive for flights and accommodation. Is there a breakdown of that \$14 000 available?

Mr ROCKLIFF - Is that from the Advanced Manufacturing and Defence Industries budget?

Dr BROAD - Are you answering the question? Is there a breakdown of the \$14 000 available of what the money was expended on?

Mr ROCKLIFF - If the question is put on notice I am sure we could provide the breakdown of the \$14 000.

PUBLIC

Dr BROAD - What has been the total cost to Government for trade missions over the last budget period?

Mr ROCKLIFF - The total trade missions actual is \$668 876 in 2022-23.

Dr BROAD - Correct me if I am wrong, but the only line item I can find in the Budget that is directly attributable to the Trade portfolio is the Accelerating Trade program but the funding for that only continues for two years.

Mr ROCKLIFF - That is correct; I have just said that.

Dr BROAD - Why is the funding disappearing after 2024-25?

Mr ROCKLIFF - I wouldn't say it is necessarily disappearing. The Trade Strategy goes to 2025 and we review the strategy as we go. My expectation, Dr Broad, is that the allocation of funds following the review of the Trade Strategy beyond that time will allow us to put what investment is required there.

Dr BROAD - There is no provision in the Budget, so you're pretending to be in surplus in 2025-26. Are programs like this not funded for those two years? Why haven't you put in the \$180 million provision for continuation of time-limited programs to cover very important projects like this?

Mr ROCKLIFF - We review our Trade annual action plans every year. That is a seven-year strategy but there are two to 2025. No doubt we will be doing our due diligence as to where that investment should be applied further in the future.

Dr BROAD - It seems there's no trade output group and you're just lumped into output group 1 - Industry and Business Growth, and in 1.2, which is a lot of different programs. How much of that particular line item can be attributed to the Trade portfolio? Why isn't it outlined in a separate line item in the budget?

Mr ROCKLIFF - If I look at the Industry and Trade Development Portfolio, the Tasmanian Trade Strategy 2019-25 is \$1.25 million; the Trade and Investment Mission Plan 2024 was \$1.065 million; sorry, that is what the Trade Strategy is.

Ms O'CONNOR - Minister, as you know the Greens have been quite critical of the Tasmanian Government for being too heavily reliant on trade with China. But, you would also be aware that the Chinese Government has announced that it is lifting its trade sanctions on timber imports from Australia. Are you able to give a year-by-year breakdown of timber exports out of the port, just for the period of COVID-19 so far, which as we know is not over?

Mr ROCKLIFF - Could you allow us to have a bit of time in doing that?

Ms O'CONNOR - While you're doing that, how much has the Tasmanian Government diversified its trading partners, so that we're not over-reliant on a regime which is prepared to use trade to punish nations it feels have fallen out of line?

Mr ROCKLIFF - The disruption of the pandemic significantly impacted our trade with China, and particularly for markets such as our rock lobster, for example. And so, one of the diversification opportunities - and please, those at the table correct me if I am wrong - was diversifying into Singapore with the Red Dragon Rock Lobster opportunity.

The other proactive action is the trade advocates that we've deployed in the last few years into Japan, Singapore and the United States. That's a proactive example of diversification away from having all our eggs in one basket, as some people might say.

Ms O'CONNOR - It was true.

Mr ROCKLIFF - I was concerned about our over-reliance in China.

Ms O'CONNOR - Any plans to send a trade delegation to Taiwan, for example, so we can support a thriving little democracy?

Mr ROCKLIFF - Tasmania's top goods export markets are currently China, USA, South Korea, Japan and Taiwan. I've been to South Korea and Japan earlier this year. At numbers six to nine, if you like, is Indonesia, Thailand, Malaysia and Vietnam. China's share of Tasmania's goods export market has dropped from 40 per cent in 2020-21 to 33 per cent.

Ms O'CONNOR - It's a good start.

Mr ROCKLIFF - It dropped from 40 per cent to 33.1 per cent in the last two years. We expect the recovery of our trade in international services to continue, and in particular in tourism and education, as conditions change and more international travel resumes. We can expect some downturn of exports in a few of our key destinations from the effects of the global economic challenge, such as South Korea, I'm advised. That's been a significant reduction, but we've been very proactive.

Last year I had a trade mission to Vietnam and Singapore. The focus was food and beverage, but also advanced manufacturing. We had Japan and South Korea this year, which was again food and beverage but also advanced manufacturing - particularly in South Korea.

What I can tell you about is the enormous interest in Tasmania's 100 per cent renewable energy capacity and our renewable investment, and how far behind, if you like, some of the countries in that area are with their renewable energy capacity - and how interested they are in where we are at. I see that a potential export opportunity.

Ms O'CONNOR - Can I just say what a big difference it is between the trade strategy from even a few years ago, when your ministers and the Coordinator-General were going on trade missions to China about twice a year. That's very positive. Do you expect this diversification strategy to continue?

Mr ROCKLIFF - Yes, I do. It needs to continue. China is still our largest export market

Ms O'CONNOR - Sure; but you wouldn't want to be totally reliant them, would you; because they'll flatten us, on a whim.

Mr ROCKLIFF - Diversification is very important and that's part of our trade strategy - particularly putting advocates in key locations. That's been a good investment, in my view.

Ms O'CONNOR - Do we have answers to the question on timber exports?

Mr ROCKLIFF - The Minister for Resources will, I'm sure.

Ms O'CONNOR - I thought it might come under the Trade portfolio.

Mr EVANS - I'm advised that we don't have that data. The exports are transhipped through Melbourne so they are not captured in the available data that we have. But, I'm sure the Minister for Resources -

Ms O'CONNOR - It's not that the data isn't captured; it's just not held by Trade?

Mr EVANS - No, we don't have it.

Dr BROAD - I have some questions about the trade ambassadors. What is the total cost of the Trade Ambassador Program?

Mr ROCKLIFF - The actuals, for 2022-23, for trade ambassadors only: \$453 225.

Dr BROAD - Thank you. How much is salary and how much is other expenses?

Mr ROCKLIFF - The USA Advocate is \$180 000 per annum; the Japan Advocate is \$125 000 per annum; the Singapore Advocate is \$140 000, but it is out of contract now and we're in an open procurement process; and we have a China Business Development Manager, a permanent employee of Austrade contracted to the OCG and with a separate budget.

Dr BROAD - That's all salary? Right. Has there been any turnover of staff, any vacancies?

Mr ROCKLIFF - The Singapore Advocate contract has ended and there is an open procurement process now for a new Singapore Advocate.

Dr BROAD - And China has come to an end?

Mr ROCKLIFF - No, it is Singapore, but it hasn't come to an end; we are going to out to get another person.

Dr BROAD - What assessment has been done of the outcomes? What sort of KPIs are they measured against and what are the outcomes they have or haven't achieved?

Mr ROCKLIFF - The Tasmanian trade advocates and our advocacy models and partnerships provide in-market advocacy, promotion support and representation at international events and meetings, which has resulted in \$4 171 720 worth of new contracts to date for this financial year 2022-23. For the nine months until 31 March this year, 290 Tasmanian businesses were provided with a service, 21 new contracts realised almost \$4.2

million. We have 28 dedicated media and brand promotion activities for Tasmanian businesses in key markets, and 48 investment leads have been explored or introduced.

Dr BROAD - What are their KPIs and are they being met?

Mr ROCKLIFF - Noting that there is still three months of data to be captured to finalise this financial year, the IBDN has still achieved the following outcomes: an additional 81 businesses being supported, an increase of 32 per cent from the prior year; a further 15 contracts being entered into, increasing the value by \$3.7 million, \$3 737 544 and 27 additional investment leads introduced.

Dr BROAD - What are their key performance indicators then, if you want me to spell it out? What are their key performance indicators?

Mr ROCKLIFF - There's key performance indicators in the trade strategy. They produce quarterly data on how many businesses are being supported, how many contracts are being entered into, and how many promotion activities as well.

Dr BROAD - I take it these advocates are part-time. I imagine the salaries you are talking about wouldn't involve them being full-time advocates?

Mr ROCKLIFF - Yeah.

Dr BROAD - What portion of their time is allocated against trade in Tasmania?

Mr ROCKLIFF - In terms of FTE?

Dr BROAD - Yup, well FTEs is one way of looking at it.

Mr ROCKLIFF - I can get that information for you. Ms Hendricks, would you like to talk to the committee around Dr Broad's question?

Ms HENDRICKS - Thank you Premier, you are correct. The US advocate and the Japanese advocate are both on a part-time arrangement. It's about a 50/50 arrangement. They provide quarterly updates on those KPIs. When we have lead-ups to missions, the time isn't for an even two-and-a-half days a week, it truncates depending on the busy-ness. They also represent us at a lot of media events and a lot of expos in markets. That time obviously gets picked up and that is reported on their quarterly reports. The [inaudible] arrangement, which is Udai Panicker who was, until recently, our Singapore advocate, has put in a submission for the current one that is open at market. They were a consultancy rate relationship but that worked out about the part-time hourly rate as well.

Ms O'CONNOR - Premier, does the Office of the Coordinator-General work to you at all as Trade minister?

Mr ROCKLIFF - I wouldn't describe it as working to me, the Coordinator-General has been on trade missions with me-

Ms O'CONNOR - That's right, many, probably more than all the ministers put together really. You don't need to answer that. Has the Coordinator-General led any trade delegations to China in the last couple of years?

Mr ROCKLIFF - The advice I have is no.

Ms O'CONNOR - That's interesting isn't it, because I remember Will Hodgman and Peter Gutwein used to get up and talk about how wonderful it was that another trade delegation had gone to China. There has been quite a significant shift in trade policy, hasn't there?

Mr ROCKLIFF - There is a shift in terms of diversifying our markets, that is correct. You'd be aware of the relationship between our national government and China, which has been challenging in the past couple of years. The new federal government appears to be thawing out that relationship.

Ms O'CONNOR - Let's hope it doesn't come at the cost of our sovereignty. Is there any planned trade delegation to China upcoming or has it just been decided that those trade delegations are not so valuable anymore?

Mr ROCKLIFF - Every trade mission is of value but we are concentrating on the areas that I mentioned in my opening statement, including the United States, which will be early next year, the United Kingdom in early September.

Ms O'CONNOR - Can you confirm we have a really interesting trading relationship with Taiwan in relation to education?

Mr ROCKLIFF - Yes.

Ms O'CONNOR - Isn't there an engagement on educational opportunities and that sort of thing? I will take that on notice. Premier, would you ever consider going on a trade delegation to Taiwan?

Mr ROCKLIFF - It's our fifth-largest goods market.

Ms O'CONNOR - That's right. There hasn't been one to Taiwan in 20 or 30 years. I can't imagine why.

Mr ROCKLIFF - I went to Taiwan around 2012.

Ms O'CONNOR - Good, but not as Premier. Don't you think it would mean a lot to the government of that independent democratic state if a Premier like yourself went on a trade delegation to our fifth-largest trading partner?

Mr ROCKLIFF - Potentially, yes.

Mr EVANS - Just to add, we recently supported the delegation from Fruit Growers Tasmania to visit Taiwan last year.

PUBLIC

Mr ROCKLIFF - Towards the end of this year, we will be formulating the next two-year mission plan to take us into the end of 2025. Our emphasis there will be evident.

Dr BROAD - How many FTEs can you associate to the Trade portfolio?

Mr ROCKLIFF - Fifteen, I am advised.

Dr BROAD - Are there any current vacancies?

Mr ROCKLIFF - Three.

Dr BROAD - What sort of positions are vacant?

Mr ROCKLIFF - Senior director of trade, trade development adviser and trade development support manager.

Dr BROAD - Are you actively recruiting to these positions?

Mr ROCKLIFF - I am advised by Ms Hendriks - two are out at market and the other one will be advertised on 12 June.

Dr BROAD - Referring back to your earlier answer about the \$14 000 spent by Ms Ogilvie to Indonesia, the DPACT notice seems to indicate that that was for flights only.

Mr ROCKLIFF - The majority of it was for flights. Accommodation was \$428. I am advised that Ms Ogilvie flew business class and the flights totalled \$13 978.

Dr BROAD - That seems an extraordinary amount for flights to Indonesia. Why were those flights so expensive? Was it done at very short notice? What other contributing factors are there to having such a huge flight cost?

Mr ROCKLIFF - I will have to take that on notice.

I understand that our Defence Advocate was scheduled and couldn't for personal reasons and Ms Ogilvie stepped in at reasonably late notice.

Dr BROAD - Thank you.

CHAIR - The time being 1 p.m. the budget Estimates for the Minister for Trade has expired. We will resume at 2 p.m.

The Committee suspended from 1 p.m. to 2 p.m.

MINISTER for HEALTH

CHAIR - Welcome everyone here today. I welcome the Premier. Could the Premier please introduce the persons at the table with him for the benefit of *Hansard* and could you also give us a brief opening statement as well?

Mr ROCKLIFF - Thank you, Chair. To my right his Kathrine Morgan-Wicks, secretary of the Department of Health; Shane Gregory, associate secretary; Professor Tony Lawler, chief medical officer and deputy secretary Clinical Quality Regulation and Accreditation; and Dale Webster, deputy secretary Hospitals and Primary Care.

It is a great privilege for me to sit here for my third Estimates hearing as Minister for Health. We know that Health is very challenging and complex and we are working hard to ensure we can meet the ever-changing health needs of Tasmanians now and into the future. We also know the demands of the health system are increasing, which is why we are investing record amounts into Health.

Our investment into Health in the state Budget reflects our commitment to addressing the growing demands for health services across Tasmania. In our Budget, our Government has outlined a \$12.1 billion investment into Health across the 2023-24 Budget and forward Estimates. That equates to almost \$8.3 million on average per day and 34.8 per cent of the total Budget has been allocated to Health services. This means more beds, more staff and more initiative to drive down waiting lists and reduce wait times. We are investing more in our hospitals, in community health care and in mental health care. We are also making significant investments in primary health care to increase access to these services across Tasmania even though this is primarily the responsibility of the federal government.

We are continuing to progress our \$196.4 million elective surgery plan to ensure more Tasmanians get the care they need sooner and I'm pleased to say that our plan is working, with the latest Health dashboard data confirming elective surgery waitlists are at their lowest since 2018. In fact, since January 2021 we have seen the elective surgery waiting list reduce by some 30 per cent. I want to thank and acknowledge our dedicated health professionals for their fantastic efforts.

Additionally, we are providing a \$38 million boost to endoscopy services which will ensure that we can deliver an additional 22 000 endoscopy procedures by 2027, ensuring we can reduce wait times for Tasmanians. We're also taking significant steps to improve the way our outpatient services are delivered through our outpatient transformation strategy. This will ensure that Tasmanians can access the care and services they need sooner.

While these plans are having a positive impact in the short term, we're not losing sight of the need to plan for the future. That is why we are prioritising the development and upgrade of major public hospital sites across Tasmania through a significant capital works program.

In the south, we are progressing our plan for health services in the region as part of the 30-year Royal Hobart Hospital redevelopment and this includes our announcement last week that St Johns Park in New Town will be transformed into a contemporary health and wellbeing precinct. In the north, we are continuing work on the implementation of the Launceston General Hospital master plan including the recently announced new helipad for the LGH. In the north-west we've released a blueprint for hospitals in the region through the North West Hospitals Masterplan to guide the future of health services in the region.

We're also bolstering Ambulance Tasmania with more resources, building new ambulance stations and employing additional paramedics to support these vital services across the state. This includes new ambulance stations in regional and rural areas to support local communities including Bridgewater, Beaconsfield, Queenstown, Longford, Oatlands and Bicheno.

This week we're also releasing the final version of the Long-Term Plan for Healthcare in Tasmania 2040, which sets out our vision for a more connected health system that meets the needs of Tasmanians now and into the future. The plan outlines how we will work in partnership with the Tasmanian community to achieve our vision that all Tasmanians are supported by a health system that is world-class, integrated and innovative.

We also know that in Health our biggest asset is our people. Since coming to government we've employed almost 1400 additional nurses, 390 additional doctors, 310 additional allied health professionals and 220 additional paramedics and dispatch officers. We continue to recruit and plan for the future through the Health Workforce 2040 Strategy to shape a health workforce that meets the needs of Tasmanians now and into the future.

We are also giving Tasmanians an increased say in how we shape health services of the future through initiatives like the Health Senate, which brings together clinicians and people who use our services to provide evidence-based and guidance. We are rolling out innovative programs to boost our workforce in priority areas and this includes our Allied Health Scholarships of up to \$25 000 to boost our allied health workforce across the state. This will see up to 45 health professionals added to the Health Service from 2024, which will provide an immediate boost to the workforce for areas currently experiencing workforce shortages.

These are just some of the key investments we are making as part of our commitment to delivering a contemporary healthcare system that meets the needs of Tasmanians now and into the future, ensuring we can achieve the right care in the right place at the right time.

Ms DOW - Premier, why is your Government committing nearly \$8 million this financial year to the Launceston General Hospital Masterplan and \$15 million to a stadium in Hobart?

Mr ROCKLIFF - We've committed \$115 million to the Launceston General Hospital redevelopment over the course of the forward Estimates and we are also investing in our roads and bridges and all sorts of infrastructure across Tasmania, as well in various areas including schools. I wouldn't make the comparison, Ms Dow, because it is not useful in terms of the extraordinary commitment that we are making in Health.

In fact, we have committed in this Budget \$115.4 million to continue the redevelopment of the LGH, including \$83.7 million towards our \$580 million stage 2 plan. This will see a new purpose-built mental health precinct and expanded acute care zone, surgical short-stay and day procedure unit, and urology, respiratory and infusion services. Our funding commitment is very clear, it is in the budget papers available, and when we look more broadly at our investment into Health infrastructure, we are some \$689 million, if memory serves me correctly, I haven't got the figure right in front of me - \$682 million, thank you Ms Morgan-Wicks. I am pleased to advise that with the completed project, stage 1 of the LGH precinct master plan is now substantially complete. Work on stage 2 projects has already

begun, with this stage including detailed planning works for many of the complex construction projects, including a new mental health services precinct, delivering a new helipad to meet contemporary standards, modernising health records to free up additional space, detailed planning for the new LGH Research Centre to include the Clifford Craig Foundation and Library.

Ms DOW - They're all really important things and I guess that's why I asked the question, because there is only \$8 million there this financial year. It is a \$580 million project. It was your Government's key election commitment at the last state election, where at the time you said that you would ask the federal government to further fund that project, given the significant demand on the Launceston General Hospital with bed-block and the emergency department and pressures there. Did you ask the previous federal Liberal government for funding towards the Launceston General Hospital?

Mr ROCKLIFF - I asked the federal coalition government to support our health services and infrastructure, yes.

Ms DOW - For this particular project, the Launceston General Hospital?

Mr ROCKLIFF - There are a number of projects I wanted to see investment in and we will continue to do so with the new federal government.

Ms DOW - Have you asked the current federal Labor government for funding for this project?

Mr ROCKLIFF - We are working together on a range of health services and investment.

Ms DOW - My question is specific.

CHAIR - Ms Dow, can you please let the Premier finish his answer before you ask another question?

Mr ROCKLIFF - I have been pleased to work alongside the federal government when it comes to delivering our urgent care centres, when it comes to delivering our investment in the single-employer funding model, and a range of other innovative solutions. I will continue to work with the federal government and ensure that we have not only investment in infrastructure but also key investments in innovation and other services that I have just mentioned.

Ms DOW - To be clear, Premier, my question was specific to the Launceston General Hospital stage two redevelopment, which is \$580 million. Have you asked the federal government for assistance to fund this very important project for the state?

Mr ROCKLIFF - We are committing to it.

Ms DOW - \$8 million this year.

PUBLIC

Mr ROCKLIFF - It was \$24.7 million this year and we have \$15 million for the helipad as well, including the \$24.7 million. Next year it is \$26.9 million, \$47 million and \$31.7 million.

Ms DOW - You haven't asked the federal government for money for this project?

Mr ROCKLIFF - Ms Dow, we are investing in the Launceston General Hospital. Of course, we can engage with the federal government on future planning but we have \$115 million over the course of the forward Estimates to invest in the master plan.

Ms DOW - Can you table the correspondence you have had with the current federal government around this project?

Mr ROCKLIFF - Table the correspondence?

Ms DOW - Yes, if you have written to them and discussed this, whether it be - was that a formal representation that you made to them about this project?

Mr ROCKLIFF - What I'm saying is I've been working with the federal government on a range of areas, including urgent care centres and the single-employer funder model, investing in primary healthcare.

Ms DOW - But you haven't directly asked them for the LGH?

Mr ROCKLIFF - Because we have got it in our Budget, Ms Dow. We're spending money, investing -

Ms Dow - Yes, but it's \$580 million, and there's \$8 million.

CHAIR - Ms Dow, can you let the Premier answer the question before you ask another question, please?

Mr ROCKLIFF - I will always have our discussions in the best interests of Tasmanians with the federal government. Have you asked?

Ms DOW - No. You're the Health minister, it's your responsibility to ask. Of course I am working closely with our federal -

Mr ROCKLIFF - No, obviously not. I would have thought that you might have asked as shadow minister for health. Anyway, if you haven't.

Ms DOW - I certainly have raised it with the federal health minister in the minister meetings that I've had with him.

Dr WOODRUFF - Minister, are you aware of the advice that has been released by the Victorian Chief Health Officer to Victorians to help them recently protect themselves in the COVID-19 wave that we are having at the moment? The advice consists of wearing a mask, getting a 2023 booster, letting fresh air in, opening windows and doors to reduce the spread of the virus, getting tested, staying at home if a person has COVID-19, staying at home for at

least five days, or until they have no symptoms, and talking to their doctor. Are you aware of that advice?

Mr ROCKLIFF - Well, that's advice to the Victorian people. We have our winter plan, which encourages vaccination as well and COVID-19 safe behaviours.

Dr WOODRUFF - Do you agree that's potentially life-saving advice that the Victorian Government CHO has provided?

Mr ROCKLIFF - We're managing respiratory infections during winter this year. This includes all respiratory illness, particularly influenza and COVID-19. The winter strategy aims to ensure the state is prepared to manage respiratory infections during winter, with a focus on health system preparedness, infection prevention, detection and early treatment to minimise health harms and health system strain. The strategy is accompanied by a comprehensive communication campaign, encouraging Tasmanians to take steps to reduce their risk of winter illness.

Implementation of this strategy includes the following key elements, which I'm sure you would be very interested in, Dr Woodruff: supporting health system readiness to respond to demand through enhanced statewide coordination, escalation planning and increased bed capacity where possible; influenza and COVID-19 vaccination coverage; testing and surveillance; working with priority groups such as aged care, disability care, education and early childhood to optimise vaccination coverage and to plan for and manage outbreaks of infections. We are supporting Tasmanians to access testing treatment and care by maintaining hospital capacity and providing community-based care alternatives, such as COVID@homeplus. COVID@homeplus will continue to provide a 24-hour seven-day-a-week service to provide care and support to vulnerable or high-risk Tasmanian people with COVID-19 and other respiratory infections in the community.

Dr WOODRUFF - Minister, I was talking about advice to individuals about how they can protect themselves during the current wave.

Mr ROCKLIFF - I agree.

Dr WOODRUFF - The current and ongoing level of community transmission is substantial but Public Health Tasmania hasn't released the sort of accessible high-quality advice that the Victorian Government has. The advice from our Public Health is to take personal responsibility; to keep your hands clean by washing with soap and water or by using alcohol-based hand sanitiser; covering coughs and sneezes, which doesn't protect you from other people's infections; and staying at home when you're sick. No advice on how long to stay home for, no advice not to leave if you have symptoms. Can you see that there's a problem with the quality of the advice being provided in Tasmania?

Mr ROCKLIFF - The winter strategy is supported by the comprehensive public information campaign which is currently underway.

Dr WOODRUFF - I have a copy of it here from the newspaper and that's what it says.

Mr ROCKLIFF - I remind everyone to please receive your annual influenza vaccine; keep up to date with your COVID-19 vaccinations; make early plans with your GP to access testing and antiviral medication, if eligible, before you become unwell; stay at home when unwell; maintain hand hygiene and cover coughs and sneezes; access the most appropriate care if you become unwell, including primary care, COVID@homeplus and, if you are severely unwell, please present to a hospital. We have quite a comprehensive winter strategy plan here from the Department of Health.

Dr WOODRUFF - I'm talking about advice to -

CHAIR - Order, Dr Woodruff.

Dr WOODRUFF - I'm not interrupting him; he's stopped speaking. I'm clarifying; I'm not talking about what the Government is doing. I am talking about the advice to Tasmanians during the current winter wave of COVID-19 about how they can keep themselves safe.

Mr ROCKLIFF - Yes, exactly, so the Winter Wellness campaign supports the 2023 winter plan. It uses an integrated multi-channel approach and encourages Tasmanians to take steps to minimise their risk of severe respiratory illness through winter. So, prepare by being up to date with your vaccinations. They will boost your immunity against COVID-19 and the flu to reduce severe illness if you get sick. Plan. If you are at high risk of severe illness, make a plan with your healthcare provider for access to testing and antiviral medication if you get sick, as I said. Protect yourself and others by washing your hands, covering your coughs and sneezes, and staying at home if you are unwell.

Dr WOODRUFF - Can you see that people have pointed with a great deal of concern about your repeated failure to provide advice to people to wear masks? All you talk about is to wash your hands, which is almost ineffectual in an airborne virus. Why do you continue with not telling Tasmanians to protect themselves by wearing a mask and staying at home if they have any symptoms?

Mr ROCKLIFF - I point you to Dr Veitch's media statement on 1 June regarding a number of matters. Of course, separate to that, we would encourage the wearing of masks, particularly around vulnerable Tasmanians. Masks are more commonplace now than they were pre-COVID-19. Personal responsibility is very important so we also encourage individuals to register their positive result for COVID-19 and offer registration in COVID@home, receive individual instructions on treatment and isolation. So, there is a lot of information when it comes to COVID-19, Dr Woodruff.

Mr YOUNG - Minister, last year the Tasmanian Government announced it would undertake a consultation process where children and young people will be invited to make suggestions on the new name of the current ward 4K at the Launceston General Hospital. Can you please update the committee on the progress of this project which is a key component in rebuilding the public's trust and confidence in the Launceston General Hospital?

Mr ROCKLIFF - Thank you for your question, Mr Young. Our Government has been very clear that there is nothing more important than safeguarding children while they are in

our care. We spoke about that this morning. We are committed to continuing to implementing important and necessary change to ensure the safeguarding of children.

The renaming of ward 4K at the Launceston General Hospital is part of a state wide project to give a name to all paediatric wards in Tasmanian public hospitals. As you mentioned, Mr Young, our Government is committed to undertaking consultation with children and young people. I can advise the committee this has now occurred as part of a broader workshop about improving child safety and wellbeing in health services.

I am happy to announce that through this process and consultation with the children, the new name for children's inpatient services across Tasmania will be known as the Wombat Ward. There is a wombat there for display. Thank you very much, Kath, and thank you to all our young people who have participated. As well as being informed by the ideas of children and young people, staff from across the Department of Health were also invited to have their say on the new name.

I thank all staff, health leaders and most importantly the children and young people who participated in the workshops for their valuable contribution to this process. Engaging with children and young people to inform activity and decision-making is a key component of the Department of Health's Child Safety and Wellbeing Framework, which was released in September last year. Consultation with young Tasmanians on the new name of the children's ward at public hospitals in Tasmania was important to gain their valuable input on a name that is welcoming and safe for them. The department will now start work to formalise the new name for our inpatient children services across the state.

I have examples of some young people's drawings of wombats, which I am happy to table. That's very good consultation; listening to our young people and the voices of young people is very important. I congratulate everyone.

Ms DOW - My next question is also about the Launceston General Hospital, Premier. Is there a business case that is publicly available for the Launceston General Hospital redevelopment stage 2 - the \$580 million project?

Mr ROCKLIFF - The master plan that has been produced.

Ms DOW - The business case, Premier.

Mr ROCKLIFF - Perhaps I can throw to Shane who might like to comment on the Launceston General Hospital master plan.

Mr GREGORY - The development of a master plan comes after the planning process. The significant next stage of the master plan would be the tower, which replaces the north side building. There's a number of steps to get to there, before that can be built. We will go through an extensive planning process which will inform the design, the cost and that will then develop the business plan.

Ms DOW - You haven't prepared a business plan, Premier, to provide to the federal government for funding on this project?

PUBLIC

Mr ROCKLIFF - We have a master plan and we are investing in the Launceston General Hospital.

Ms DOW - I know that you have a master plan. I am asking you about the funding and a business case for the project.

Mr ROCKLIFF - We are investing, ourselves, in the Launceston General Hospital. We can have discussions with our federal colleagues at a point in time as well, as we continue to invest. It is exciting. It is good news.

Ms DOW - I want to be clear, just one more time, Premier, and ask you directly. Have you approached the federal government for funding for the Launceston General Hospital redevelopment?

Mr ROCKLIFF - We are investing in the Launceston General Hospital redevelopment. I am happy to have positive discussions with the federal government - as we have done over the course of the last 12 months on a range of matters, including a 50/50 funding commitment for health. There is a range of areas that we can continue to ask.

Ms DOW - Will you be asking them about this project, if you haven't already?

Mr ROCKLIFF - I have written to the Prime Minister, acknowledging the broader work undertaken by the health funding reform through the mid-term review of the National Health Reform Agreement. All premiers and chief ministers would like readjustment, to provide equitable 50-50 health funding from the Commonwealth - which seems very fair to me - but I'm also content to continue to advocate of behalf of all Tasmanians in terms of infrastructure-

Ms DOW - Premier, is the redevelopment of the Launceston General Hospital a priority project for you? It sounds to me like it's not.

Mr ROCKLIFF - It is a priority project. They are all priority projects. We're investing \$41 million into upgrading the Mersey Community Hospital, right now. We've got a brand-new anti-natal-

Ms DOW - It was your signature election policy at the last state election. You said at the time you would approach the federal government for funding to make it happen, to make better services available for people across the north and the north-west. It's a major hospital for the northern part of the state.

Mr ROCKLIFF - Yes. I recognise that. We're pleased to be able to work with the federal government. You also need to recognise infrastructure investment in hospitals is staged - as you know, they're working hospitals. But we're very pleased to be able to have a new ante-natal clinic in Burnie; \$41 million redevelopment of the Mersey; \$15 million for the helipad. Other significant Commonwealth commitments currently being negotiated between the Department and the Commonwealth include the establishment of four urgent care centres across Tasmania. As I've mentioned -

PUBLIC

Ms DOW - Just to be clear, Premier, will you ask the federal government for funding for this very important project?

Mr ROCKLIFF - I will be working with the federal government in a constructive way and seeking their assistance in a number of matters - and \$20 million in Commonwealth funding for construction of a hospice, located in the of Launceston General Hospital precinct.

Ms DOW - That was Ross Hart. It's a Labor commitment.

Mr ROCKLIFF - Well, thank you. It was a commitment made by both parties at the last election, so thank you Bridget Archer and thank you, Ross, and whoever else you want to thank, it's fine.

Ms DOW - The point of this question was about the commitment that you made towards the Launceston General Hospital, Premier.

When will the Launceston General Hospital redevelopment project be fully funded, Premier? And when will it be delivered?

Mr ROCKLIFF - I just want to reinforce the fact that 50;50 funding is what we're lobbying for in health. I'm sure you support me in that.

Ms DOW - The capital, for the project?

CHAIR - Ms Dow, can you please let the Premier answer before you make another comment.

Mr ROCKLIFF - I mentioned the hospice location. There is approximately \$5 million in Commonwealth funding for the John Flynn pre-vocational doctor program. It is an extension of the Royal Junior Doctor Training Innovation Fund which is being expanded through this program to enable rural primary care rotations for intern and resident medical officers as part of the Tasmanian Rural Generalist Pathway. There is \$4 million to develop a medical research centre within the LGH. I've spoken to Bridget Archer, our local member, about the LGH redevelopment and I'd expect that a major project such as this would attract federal funding as is usually the case with major projects. But, we've got \$18 million [TBC] over the next four years committed to a new mental health precinct at the LGH. So, there's a lot happening. We're doing it in stages.

Ms DOW - Premier, will you fast-track the development of the Launceston General Hospital? There's been numerous calls, given the current state of service delivery out of the Launceston General Hospital; the need for more bed capacity; the need to do more elective surgery; the pressure on the emergency department. Will you fast track the development?

Mr ROCKLIFF - We have already been fast-tracking it. Have you been to the new women's and children's precinct?

Ms DOW - \$8 million this financial year is not fast-tracking.

CHAIR - Ms Dow, please let the Premier answer.

Mr ROCKLIFF - Brand new women's and children's precinct, is that how it's titled?

Ms DOW - I have been through. I've taken a tour of it.

Mr ROCKLIFF - And fast-tracking the helipad. Also, it's a 38-bed ward we've established at the LGH - Ward 3D. So, you could argue that we are fast-tracking.

Dr WOODRUFF - Minister, I am concerned at the degradation of the advice provided to Tasmanians about how to protect themselves from COVID-19 over the past year by your department. I have in front of me from October last year, which is only nine months ago, public health advice telling Tasmanians that if you are a positive case or contact, ask your workplace about their COVID-19 policy for your return to work. If you have COVID-19 or respiratory symptoms, you should avoid contact with high-risk settings for seven days. If you need to leave home wear a mask. You should wear a mask in indoor public places and avoid those places for seven days if you have COVID-19 or symptoms. At the moment the advice available is to stay at home when you're sick, wash your hands, cover coughs and sneezes and talk to your doctor if you're in a high-risk group.

This is a huge degradation in the advice. What is the basis for that? Maybe the Director of Public Health would like to provide some advice to us about this? We do have a hardening of evidence about the risks of infection and reinfection of COVID-19 and long COVID-19 symptoms. It was the third biggest killer of Australians last year. We are finding out more about the dangers of COVID-19 infection, not less. But, your advice is getting worse.

Mr ROCKLIFF - I don't accept any degradation of advice.

Dr WOODRUFF - Well, it is. Why don't you tell people to wear a mask in crowded settings, or stay at home if they have any respiratory symptoms?

Mr ROCKLIFF - I've always said I encourage people to.

Dr WOODRUFF - That's not the advice.

CHAIR - Dr Woodruff, can you please let the Premier answer the question?

Mr ROCKLIFF - I think you'll find that it is most likely the advice. Dr Veitch would you like to come to the table and talk about the current state of play with respect to COVID-19 advice. I referred to Dr Veitch's media statement before. That was in relation to an increase from low to moderate. That was the reason for the release, as I understand?

Dr VEITCH - In the first instance I would refer Dr Woodruff to our website, which provides the advice she is seeking. It tells us that people who have symptoms or a respiratory infection are encouraged to get tested, to stay home while they're ill, to register a positive RAT, knowing if they have a positive PCR the lab will notify it and ensure that any predetermined pathways to care are followed up. So if they are seriously unwell, or at risk of becoming unwell, they will get anti-virals quickly. They are recommended to stay at home while they have symptoms. They are recommended to wear a mask if they are outside their

home for seven days, at least, after their diagnosis. This advice is in keeping with the Victorian advice. We called it the COVID-19 risk level, but it's more useful to consider it as the winter respiratory illness risk level. COVID-19 has increased steadily since February, but we are seeing far fewer cases than we were at almost any time last year.

However, the cases had increased and we had seen a modest increase in the number of people hospitalised. We'd seen a number of infections in residential aged care, but again, nowhere near as many as we were seeing last year. We thought it was prudent to give the public advice to be more vigilant, undertake protective actions because we know that every winter, with the exception of the extraordinary winters of 2020 and 2021 when we were locked down, we'll see an Influenza epidemic go through. That will happen quite quickly.

The reason for raising public awareness last week was more than COVID; it was also to be ready for a winter epidemic of influenza which typically comes on very quickly and gets quite high, quite quickly - within two or three weeks; much more quickly than COVID has increased in recent weeks. And also there's respiratory syncytial virus which has also had a modest increase. We were providing advice to the public to reinforce a lot of the messages we provided over the last few years.

Mr ROCKLIFF - And the website says - if you test positive by RAT, register your result; stay at home while you still have symptoms, as I believe I alluded to; wear a mask in indoor public places for 7 days after your positive test; and, if you have COVID-19 or cold or flu-like symptoms, do not visit high-risk settings or people at high-risk from respiratory illness for at least 7 days. Then it goes through covering coughs and sneezes as well as saying that public health recommends wearing a face mask in crowded indoor spaces, on public transport and when visiting people who may be at risk of severe illness.

Dr WOODRUFF - Well, that's a change because when the information that I've got was printed, at the time it was raised to moderate didn't say anything like that. So, clearly, something is breaking through. We are the only state that hasn't had a chief health officer or a director of public health or premier address their population about COVID-19 in the last 6 years. We've had -

Mr ROCKLIFF - The website was updated on 4 May.

Dr WOODRUFF - Okay, fine; but we've had the Australian Chief Health Officer; we've had New South Wales only a few weeks ago; Queensland; South Australia; ACT - all the other states are addressing people about COVID-19 and associated risk levels and how to keep safe. We've had nothing from you, as the minister; nothing from the Director of Public Health in his role. There's no visibility. It might be that there's something on a public health website - and there obviously is now - but that hasn't been the case and it's not a public campaign to vaccinate. We have children under 5 years-of-age completely unvaccinated. There is no advice to Tasmanians, telling them to stay home. Will you accept that Victoria sets a good standard by advising people to stay home for at least five days if they have COVID-19 and until they have no symptoms?

Mr ROCKLIFF - It sounds like we have stronger advice, with the seven day advice I referred to there. I think it aligns -

PUBLIC

Dr WOODRUFF - And no symptoms?

CHAIR - Dr Woodruff, can you let the Premier answer your question, please.

Dr WOODRUFF - Well, there's two parts to that advice.

CHAIR - Order, Dr Woodruff. Thank you.

Mr ROCKLIFF - We also have COVID@homeplus which is a very helpful service. I know that Dr Veitch launched the winter plan and answered a number of questions at that point in time. We have a comprehensive website as well as social media for public consumption; so I believe we are doing well in terms of information presented. Tasmanians have become very accustomed to accessing their Coronavirus information

Dr WOODRUFF - But they can't get a test, for love nor money; not a PCR - thanks to you.

CHAIR - Dr Woodruff.

Mr ROCKLIFF - I think you can get a PCR test; but we're also still issuing RATs.

Dr WOODRUFF - Not publicly available.

CHAIR - Dr Woodruff, can you please let the Premier answer your question.

I will ask one myself, for the Chair. Coming back to rural and regional areas, minister - after 16 years, Dr Latt's contract expires in July, placing him in a position where he won't accept another contract due to the fact that it doesn't have many benefits. Can you please explain how this has happened?

Mr ROCKLIFF - Thank you, Mr Tucker. I know we've spoken about this on a few occasions and I acknowledge your concern about this area. I received some email coorespondence last night from [inaudible] constituents which may well have been similar correspondence that you received as well. I acknowledge the long service of Dr Latt, the local St Marys community and district hospital. Like most general practices across the country, the St Mary's general practice is a private business. Noting that general practice and primary health are the responsibility of our national government, of course our Government is acting where it can to support general practitioners to provide additional services for their communities, including through the \$8 million GP After Hours Support Initiative.

We've also partnered with the Commonwealth government to deliver a \$13 million pilot program to employ GP registrars through their training to make it easier and more attractive to train as a GP. As occurs in many rural areas of Tasmania, local general practitioners provide medical services to our district hospitals. The Tasmanian Government already provides support to Dr Latt and has continued to engage with Dr Latt in order to understand what further assistance may be provided. Perhaps if I could ask the assistant secretary, Mr Gregory, to provide further detail.

Mr GREGORY - Dr Latt runs a private practice and leases some space from the department in the St Marys Hospital. That is a private operation. In addition to that he is employed under the Rural Medical Practitioners Agreement which is an employment arrangement between him and the department and he receives a number of payments through that. As part of his lease he is provided with support services, including administration and practice management assistance, and he is able to access more if he chooses to do so.

We have been talking to Dr Latt about moving to a contract for services. He's not working under a contract at the moment so we've talked to him about moving to a contract for services where we define the services we would like him to provide and he is able to tell us what he needs in return in terms of remuneration and other support to help him do that. We've been taking to Dr Latt for a significant period of time now and he hasn't come back to us yet with a price.

CHAIR - I want to follow up with another question because this is going more broadly across the Lyons area. There is a number of GP issues throughout Lyons. Why are rural communities losing excellent qualified GPs due to being emotionally and physically burnt out as they continue to work alone, and how can the Government support these rural GPs?

Mr ROCKLIFF - It's a very good question, Mr Tucker, and it is why we are working with the federal government which has the primary responsibility for primary health care, although we're reaching in more now to support our rural areas and in your electorate of Lyons we can talk about Ouse, Westbury, Campbell Town - we've had some discussion publicly around these areas.

You're right, Tasmanians are finding it harder to access general practitioners and other primary healthcare services owing to what has been falling bulk-billing rates, decreasing GP numbers and difficulties faced by patients in finding open appointment times. Along with the AMA, I often highlight the fact that around 10 years ago 50 per cent of medical graduates were going on to become GPs and now it's around 14 per cent, I understand, so we have fewer graduates going on to become GPs, which we're trying to address in some part with a single employer model. Many Tasmanians who are unable to access adequate primary health care by themselves are going for both preventative health care and immediately necessary care or presenting to healthcare settings less appropriate for their condition and often emergency departments.

While it's a federal government responsibility, there is a very key reason why we would and are reaching into the primary health care space, often in collaboration with the federal government in more recent times, so we can reduce those presentations in the emergency department where people have not been able to access the primary health care services when needed. It's also why we have worked to improve and broaden our scope of practice when it comes to other providers as well such as pharmacies, for example, or when we're going through a scope of practice review.

We've announced some scope-of-practice matters in terms of reducing red tape around health care with pharmacies last November and again earlier this year, but paramedic practitioners, nurse practitioners and the pharmacy review are key areas as well. I know Professor Lawler has a lot of experience in this space and one of Mr Tucker's questions was

really around why this is happening in rural and regional areas. Perhaps he could shed some light further to what I have said.

Professor LAWLER - Through you, minister, the challenge of course is that there are challenges facing general practice across the country and these are magnified in rural settings. Some of the challenges that face general practice are recruitment issues in that we have a significant drop in the number of graduates entering general practice as their specialty of choice but we also have a significantly aged population within general practice, many of whom are probably not far away from retirement.

There are a number of ways in which this is being addressed, both in Tasmania and across the country. One of them is to identify the issues that are disincentives. There are issues that are relevant to general practice such as remuneration, particularly the remuneration and conditions that are enjoyed by registrars as they train, because there is a significant dip and uncertainty as people move out of the hospital system into general practice training prior to becoming a general practitioner.

That is being addressed in part through the single employer model trial, which is a collaborative arrangement between the Tasmanian and Commonwealth governments to provide for state-based employment of registrars undergoing general practice training to ensure continuity of conditions and salary. That has been very much welcomed by key stakeholders such as the general practice training colleges, the AMA and trainees themselves and that will be rolled out and developed over coming years.

The second is that the best way to ensure a Tasmanian general practice workforce is to grow our own and in large part we see a successful growth of the Tasmanian rural generalist training program. A rural generalist is a general practitioner who is trained through one of the two colleges and has a specific additional skill such as trauma care, dermatology, mental health, women's health and any number of others. By training general practitioners through our own Tasmanian Rural Generalist Scheme, we will be looking to develop and grow the locally-grown rural generalist workforce within Tasmania, which we expect to be a significant contributor to meeting rural general practice needs over coming years.

Mr ROCKLIFF - Just very quickly, Mr Tucker, in 2022-23, work to address the matters you have raised include to co-develop a model of care and a plan for urgent care services in Tasmania, working closely with Primary Health Tasmania; implementing a single employer model for GP trainees and examining the length of training contracts; building on intermediate models of care including expanding virtual care through the THS innovation in aged care service models of care; expanding health services delivered at rurally-based district hospitals; an expansion of primary health services in rural areas to include working with existing health service providers in communities; and to explore how federal funding could be better utilised for the multidisciplinary approach with allied health, nurses, paramedics and pharmacists.

This challenge has been experienced across the nation but we have to work very hard at trying to address your concerns. That includes what Mr Gregory has said in respect of Dr Latt as well.

Ms DOW - Premier, what has become very clear today is the fact that you've prioritised funding for a stadium in Hobart over the LGH redevelopment and that you've fast-tracked that project. Why won't you prioritise funding for the redevelopment of the LGH and why won't you ask the federal government for assistance with funding, or when do intend to do that? Will you fast-track this really important project that will create over 6000 jobs?

Mr ROCKLIFF - Ms Dow, we are and have been fast tracking.

Ms DOW - Do you think that is not an important economic driver for the north of the state?

Mr ROCKLIFF - The women's and children precinct, Launceston General Hospital helipad, 38-bed ward opened in recent times, we have \$24.7 million this year, \$26.9 million next year, \$47 million the year after and \$31.7 million. We are doing that. I reject your accusation around not prioritising health infrastructure. That is a ridiculous assertion. It is not based on fact. We are working with and have advocated for the equitable 50-50 health funding when it comes to commonwealth budgets.

Ms DOW - When will the long-awaited air-conditioning upgrades at the Launceston General Hospital finally be completed for the comfort of staff and patients?

Mr ROCKLIFF - I might throw to Mr Gregory on that one.

Mr GREGORY - Through you minister, we are currently in the process of finalising the design for the upgrades. They will be completed within the next 12 months.

Ms DOW - When did you decide to move away from stage 3 of the Royal Hobart Hospital redevelopment being at the repatriation centre?

Mr ROCKLIFF - We were pleased to announce the New Town development and the \$2 million that goes into the Budget on scoping and planning for what will be a significant capital investment over the course of the next number of years. We have worked this through the Budget process in discussions with our health team. There are a lot of reasons why the repatriation centre upgrade will be very difficult and challenging in terms of decanting.

Ms DOW - When did you make the decision to move away from it?

Mr ROCKLIFF - Through the Budget process.

Ms DOW - What consultation occurred with the current occupants on that site prior to your announcement being made last week that the new site was St Johns Park.

Mr ROCKLIFF - I hope you are supporting what we announced last week. Do you support that? I would encourage you to support that.

Ms DOW - My question to you was, Premier, who was consulted prior to that announcement last week? It is such a sudden change and move away from the repatriation.

PUBLIC

Mr ROCKLIFF - As part of the Budget, there is \$2 million to complete the master plan process to develop this once-in-a-generation precinct for all Tasmanians. We released the draft master plan on 31 May for public feedback. I encourage Tasmanians to have their say via the Department of Health website, which will continue to be updated with further information about opportunities to be involved.

Stage 3 of the Royal -

Ms DOW - Did you consult before you made that announcement, Premier?

Mr ROCKLIFF - Stage 3 of the Royal Hobart Hospital master plan proposed the development of the repatriation hospital site as a dedicated second campus of the Royal Hobart Hospital, providing a range of services including mental health and sub-acute care. Since the release of the Royal Hobart Hospital master plan the St Johns Park site has been considered as an alternative option for stage 3.

The redevelopment of St Johns Park precinct allows delivery of health care services in a community-based environment with access to green open spaces, parkland, paths and public transport and a more therapeutic environment to improve recovery outcomes.

Ms DOW - You can't tell me who you consulted with before you made that announcement. Maybe you might be able to tell me how much was expended on the master plan for the repatriation before you made this sudden change to then move to St Johns Park? How money was spent on the repatriation redevelopment project?

Mr ROCKLIFF - Perhaps Mr Gregory has some information about the existing services that are there and consultation. I am sure Ms Dow would appreciate that and also the question around budget for the repatriation.

Mr GREGORY - Through you, minister, in terms of consultation for the development of what is a very initial phase of the master plan, conceptual plan for discussion with the key stakeholders, we talked to stakeholders whose occupancy is directly affected. There are some stakeholders who aren't affected by the proposal. We will be talking to them next. And we have consulted with heritage bodies who have a strong interest in the site.

In terms of money expended on progressing a master plan at the Repat, we haven't spent money on that, we haven't progressed that. We have thought through how you might develop that site, which led us to the conclusion that St Johns Park was a more appropriate site for development.

Ms DOW - One of the considerations around using the Repat was about having that critical mass of clinicians that were in close proximity to the Royal Hobart Hospital. If you had doctors, for example, that had patients at the Royal but then had patients across at the Repat as well, it is a short period of time for them to move between the two facilities. It also meant there was a good critical mass of health professionals close to one another for those services.

Are you concerned that St Johns Park is quite a way away from the Royal? Has consideration been given to what that would mean for those doctors providing care?

Mr ROCKLIFF - Thank you for the question, Ms Dow. It's probably best that Mr Webster answer the question, given his responsibility around hospitals.

Mr WEBSTER - In considering St Johns Park versus the Repat, we looked at the patient at the centre of the redevelopment, the type of patients and what would be best for them. The redevelopment of the Repat originally saw Roy Fagan older persons mental health closing and moving to the Repat. We see that the modern models of care for patients, particularly older persons in mental health, should spend minimum amount of time in a hospital-type environment and we need to move them to an environment which is actually therapeutic. The gardens at Roy Fagan, if you talk to the patients, are the best feature of Roy Fagan. That model of care then says to us, if we are developing a site such as the Repat, can we develop gardens? Can we develop the outlook that we need for the patient. Unfortunately, the answer to that is no. The answer to St Johns Park is yes.

In terms of the critical mass of practitioners, we will have a size at St Johns Park, which is a campus, which will allow us to have a mass of practitioners. There will be some that need to move between the Royal in Liverpool Street and the Royal campus at St Johns Park. We accept that and we need to work through that in terms of rostering and all those sorts of things. Those same difficulties were between the Repat and the Royal in terms of where it sits and things like that.

I think the other critical issue is that, in rebuilding at the Repat, particularly given the number of vulnerable patients that are in Repat wards such as the transition-to-care unit, the palliative care unit, is it patient centred to rebuild around them, given the noise, the environment of the building of a construction site? Again, we came back to it is better to build them at another facility and then do one move from the Repat to that site.

For all of those reasons, particularly for the patient-centred reasons, we need to work with our clinicians to make this work.

Mr ROCKLIFF - St Johns Park is also closer to the Royal Hobart Hospital than Roy Fagan, for example. You'll find that clinicians will also move between Royal Hobart Hospital and Calvary, so I believe it is not too much of a barrier, Ms Dow.

Dr WOODRUFF - Premier, COVID-19 vaccinations are increasingly effective at reducing people's risk of serious acute disease. But we know now that serious illness, hospitalisations, ICU admissions are just a tiny tip of the iceberg of the impacts that COVID-19 can have on people's health. The evidence of the long-term risks of neurological diseases, neurodegenerative, advanced ageing, cardiovascular disease and impacts on every organ in the body appear to be substantially increased. COVID-19 was recorded as the third biggest killer last year and that was not from the acute infection deaths. Do you agree that the long-term health impacts, both for individuals and for the population of Tasmania, are likely to be significant from COVID-19?

Mr ROCKLIFF - I'm very interested in long COVID so I hear your question. While I'm advised that most people who have COVID-19 will recover within a few weeks, there are some that may experience long-term symptoms. Internationally, the understanding of long COVID is still evolving but I am advised that people who are up to date with their

vaccinations are less likely to experience long COVID. Vaccinated individuals who do experience long COVID typically experience symptoms that are less severe and that last for a shorter period. I would encourage all eligible Tasmanians to remain up to date with their COVID-19 vaccinations as a consequence.

In June last year, we announced a \$400 000 investment to establish the operation of a statewide long COVID navigation service. The long COVID service commenced on 19 September last year to support Tasmanians who have long COVID, access appropriate care and manage their ongoing symptoms. The long COVID navigation service has been established as an extension of the COVID@homeplus program. The objective of the service is to optimise post-COVID-19 care within primary and tertiary healthcare settings. Its goals include improved access to timely care, reducing inequalities, helping management provide guidance and decision support for primary care, delivery of coordinated and patient-centred care, and improve patient outcomes. The navigation service is available to patients statewide through a referral from their GP.

The service is an extension, as I say, of the COVID@homeplus program. It has received 108 referrals in its first six months, with 46 referred on for specialist medical assessment.

I am just wanting to find some further information without repeating what I've just said. We have the federal government long COVID inquiry report. That's the standing committee on health, aged care and sport report. It aims to improve Australia's response to long COVID, an often debilitating condition possibly affecting hundreds of thousands of Australians. I have mentioned some of the figures more locally for us.

A national long COVID parliamentary committee on long COVID and repeated COVID-19 infections made nine unanimous recommendations aimed at strengthening the Australian Government's management of long COVID. That includes the definition of long COVID for use in Australia; evidence-based living guidelines for long COVID, co-designed with patients with lived experience; a nationally coordinated research program for long COVID and COVID-19; the vaccination communication strategy; access to antiviral treatments for COVID-19; support for primary healthcare providers; focus on indoor quality and ventilation. And the committee recommended, given the multiple questions that have risen during the inquiry, that the Australian Government consider a comprehensive summit into the COVID-19 pandemic and Australia's past and current response, including by all governments at all levels. If that comes to fruition, of course we would want to participate.

We have vaccination data -

Dr WOODRUFF - Probably not one I'm interested in, thank you. My question was not about long COVID. Do you understand the distinction between what's generally termed long COVID, what you just read out, which is serious and important. But the other very serious impacts for individuals and the health system of the long-term impacts on people's organs that, without them necessarily expressing any other symptoms, can at a later date lead to a much higher risk of diabetes, Alzheimers disease, heart attacks in young people, a whole range of other diseases, events, and death that can occur because of prior COVID infection, even in the absence of any other symptoms? Do you understand that that is actually the case and what is your advice about the impacts in Tasmania on our health system with the

increased risk of what we're expecting from the evidence? Diabetes, heart attacks, Alzheimer's, ageing and a whole range of other things.

Mr ROCKLIFF - Thank you, Dr Woodruff.

Dr WOODRUFF - Do you understand the distinction between the sort of long COVID symptoms you just read out, also serious, and this other big increased risk of every other organ disease from the coronavirus?

Mr ROCKLIFF - From the way you've presented that, there appears to be a distinction and -

Dr WOODRUFF - It's obviously blurry, but yes. I'm trying to get to this part -

Mr ROCKLIFF - Perhaps I'll go to Dr Veitch first and then if Professor Lawler could add any value to Dr Veitch's comments perhaps Professor Lawler could speak as well.

Dr VEITCH - Thank you, Premier. Dr Woodruff, you're drawing attention to the distinction between people who've got a series of symptoms, often with pronounced fatigue and poor concentration, the concept of brain fog, which is a fairly familiar clinical syndrome occurring after significant illness. It occasionally occurs after influenza and in glandular fever it's a fairly familiar experience for clinicians and it appears to be relatively common after COVID infections, perhaps as many as 5 per cent, although most of those people resolve symptoms within the following six to nine months, and it is more common in people who are unvaccinated, people who've got severe COVID and people who have conditions before they get sick. That has certainly been a prominent focus.

You draw attention to the probably incompletely understood phenomenon of a range of chronic conditions, often with an underlying degenerative or vascular basis, that is currently the subject of fairly intense epidemiological and pathophysiological research. I think we're probably not in a position to be able to conclude that this is going to be a long-term, ongoing tail of serious illness, but I think we obviously have to be mindful of that possibility. We have to have our surveillance for those conditions in place. We do have surveillance for all causes of mortality associated with the last couple of years and it's certainly the case that the number of excess deaths that occurred in the course of particularly 2022 is higher than the actual number of deaths that are attributed purely to COVID.

Dr WOODRUFF - You could tell us that number, if you wouldn't mind.

Dr VEITCH - I don't have the number.

Dr WOODRUFF - I believe the number is in the order of 10 per cent higher than average years, which has been, I understand, by many people attributed to COVID.

Dr VEITCH - I think it's in the order of 10 per cent, Dr Woodruff.

Dr WOODRUFF - That's a lot, isn't it?

Dr VEITCH - I think it's very important for people to understand that every year in Tasmania around 3500 to 4000 people die, mostly of substantial age at the end of their lives, and there is year-to-year variation in the number of people who die. I think it's too soon to necessarily say that all of that 10 per cent was directly or indirectly due to COVID, but at least half of it is probably due to COVID.

Dr WOODRUFF - Other people have said that. Other Australian health experts have made that assessment. I don't put myself in that category, obviously, I'm just repeating what has been expressed and believed by the Institute of Health and Welfare who did some of that research, but thank you for that. Are you saying then - through you, minister - that you don't think the evidence being accumulated on a regular basis by *Nature*, *Science*, *The Lancet*, *Journal of American Medicine*, et cetera of the increasing risks on organs from coronavirus is a concerning or a significant matter?

Dr VEITCH - I certainly did not say that.

Dr WOODRUFF - No. What are we doing about that evidence and what it's pointing to increasingly, this consistent story of the impacts of coronavirus damaging organs and the need to protect ourselves from the virus in the first place where possible?

Dr VEITCH - I think you're drawing attention to the burden of chronic disease in Tasmania, which is a very significant issue for us. We have relatively more risk factors in our population and relatively high rates of chronic disease, including heart disease and diabetes. I think it's absolutely critical that we have, if you like, all-cause prevention and all-cause care for those people so that we don't put all of our eggs in the COVID basket. You also attend to things such as people's smoking, their behaviour that may increase their risk, and we provide a comprehensive approach to that care. I think Tasmania is a modest player in medical research but probably kicks above its weight, certainly in areas such as cardiovascular disease. We've done excellent research over the years and we would be looking to the cumulative advice from the 300 or so papers that are published every day on COVID to really understand the significance of these factors over the long term.

Dr WOODRUFF - Thank you. Minister, I'm concerned that you're equivalising the risk of being infected with the flu with being infected with the coronavirus. I'm concerned that the message of your Government is to put them in the same bucket. Of course they're both concerning but why aren't you talking to Tasmanians about the particular risks of infection and the increased risk of reinfection with the coronavirus, which is different to other respiratory viruses?

Mr ROCKLIFF - I pointed to the comprehensive information on the website including weekly statistics, testing, staying well and the matters -

Dr WOODRUFF - Does it talk about reinfection risk?

Mr ROCKLIFF - Would you like to talk about reinfection, Dr Veitch?

Dr VEITCH - The simple thing about reinfection is that just about all infections now are reinfections because we've almost all been infected at least once with COVID, so the concept of reinfection is really akin to what we experience with a range of other viruses

where we have multiple infections over our lifetime and slightly different strains of those viruses. Of course we have to be vigilant for what may occur after sequential infections. We maintain surveillance for COVID and we will continue to do so. As possibly the most intensely investigated and researched condition in human history, we're probably going to find the answer to the consequences of reinfection. Clearly, if you get reinfected you're going to do relatively worse if you are older, if you haven't been vaccinated and if you lack access to treatment. Maintaining vaccination, boosters, early access to testing and treatment are the key things to avoiding the worst consequences of future infections and reinfections with COVID.

Dr WOODRUFF - Are we still requiring hospitals to report hospital-acquired COVID cases?

Mr ROCKLIFF - In the management of outbreaks is the answer as I understand it.

Dr WOODRUFF - As in all the time?

Mr ROCKLIFF - Would you like to talk about that, Dale, or clinical services planning?

Dr VEITCH - I just note that our surveillance system captures who are reported as hospitalised.

Dr WOODRUFF - No, do we collect data on hospital-acquired COVID infections?

Dr VEITCH - We do.

Dr WOODRUFF - And report that?

Dr VEITCH - I will turn to Mr Webster on the reporting.

Mr WEBSTER - We continue to monitor outbreaks within our hospitals, including hospital-acquired infections, and that is reported internally through us and we provide that data through Public Health as well on a regular basis. We are also monitoring outbreaks in residential aged-care facilities ongoing as well.

The minister mentioned clinical services planning. I am going to the studies you're talking about and those sorts of things. It is really important that we take them into account in terms of planning for our future needs in Tasmania.

We have a process of clinical services planning that picks up those changes in the health demographics within our population. A crude example, 50 years ago we wouldn't have planned for as much dementia in our hospital systems as we have in the last 20 years with the aging population. Those sorts of changes to the system in what we can expect in disease in our population are part of our long-term plan but also part of our ongoing clinical services planning so that we know what we need within our system.

Mr ROCKLIFF - I understand, Dr Woodruff, the COVID@homeplus asked you whether you have been positive before. There will be a survey that is taken into account with clinical advice which is information on reinfection as well.

Mr YOUNG - Since coming to Government in 2014 there have been a number of key reforms across health firstly with One State, One Health System, Better Outcomes reforms and then Our Health Care Future. Minister, I understand a key component of the second stage is the Long-Term Plan for Health Care in Tasmania 2040. Can you please outline the progress of this key initiative and the benefits it will bring for Tasmanians?

Mr ROCKLIFF - Thank you, Mr Young. We released the draft of the Long-Term Plan for Health Care in Tasmania 2040 for public consultation in March this year. Our long-term plan provides a blueprint of how the Tasmanian Government will achieve a world-class innovative, integrated health care system for all Tasmanians. Today I am pleased to announce our vision for the future of health care in Tasmanian has been confirmed with a final version of the Long-Term Plan for Health Care in Tasmania 2040 released today. I will table that now.

This is an important milestone in our Government's long-term health reform agenda and focuses on delivering a more connected health system that meets the needs of Tasmanians now and into the future. The plan is supported by three regional clinical service programs outlining the services to be delivered over the next five years in response to projected needs in Tasmania's regions. The priority is to expand and enhance clinical services.

These reforms are about building a sustainable health system now and into the future by focusing on connecting care across acute, sub-acute rehabilitation, mental health, primary health and care in the community. This includes in key areas such as supporting the health of older Tasmanians and the use of health navigators to assist people in accessing the services that they need. I acknowledge and thank individuals and organisations who have prepared written submissions regarding the long-term plan and progress towards the long-term plan is already underway through initiatives such as Health Workforce 2040, a 10-year strategy for digital health transformation, improving patient outcomes and hospital master planning and 20-year infrastructure planning.

Other key initiatives that are under way or will commence shortly include a \$41 million investment over two years to develop central virtual care hubs, a plan for equitable access to endoscopy through \$38 million funding to deliver an additional 22 000 endoscopy procedures by 2027, implementing nurse-led clinics to address growing outpatient care demand and reduce wait times for public outpatient services with \$20 million funding over two years. There is a funding commitment of \$500 000 to develop a master plan for district hospitals and community health centres. The development of an implementation plan is well underway focusing on the action with the greatest immediate effect. Progress will be publicly reported.

Chair, if you wouldn't mind, I can table the clinical services profile north, clinical services profile south and the clinical services profile north-west. They are profiles for 2023-27.

Ms DOW - Premier, how many nurses graduated in Tasmania last year? How many graduate nurses were employed by the Tasmanian health system this current year? How

nursing and midwifery positions are there vacant across each region of Tasmania, both full-time equivalent and fixed-term vacancies please?

Mr ROCKLIFF - That is a good question. We will get that information for you. If I could talk about nursing and recruitment while we gather the information. We continue to increase the number of the nurses and midwives employed across the state.

From 30 June 2020 to 31 March 2023, there have been an increase of 659.24 FTEs in nursing staff across the department, representing an increase of 15.6 per cent of the total nursing workforce. Since 1 July 2022, the total nursing staff has increased by 124 full-time equivalents due largely to an increase in services across all hospital services. This increase is significant as the number of nursing staff dedicated to COVID-19 support outside of hospital services has now significantly reduced since the closure of dedicated testing clinics.

Unfortunately, at this point in time, it is not possible to draw vacancy figures from the current HR system, but once implemented, the new human resources information system will address this and other major inefficiencies and operational risks associated with the current business systems and processes underpinning human resources functions within the Department of Health.

Ms DOW - Would you agree that it's superfluous to have the number of additional staff if you don't actually know how many vacancies you've got or how many have left? It's not really a true reflection of what's happening across the health service.

Mr ROCKLIFF - I will point you to the last couple of sentences. Transition to practice nurses commencing in January each year are progressively appointed across the state to areas where they can be appropriately supported. In 2022, 327 transition to practice nurses started their careers with the Department of Health. This represents a significant increase in comparison to the previous years where 292 commenced. Subject to probation, permanent employment is offered to graduate nurses who successfully complete the transition to practice program. Specific educational outcomes are in place in specialty areas to further support the retention of transition to practice graduates.

Ms DOW - Did you say how many of those graduates did transition to practice in Tasmania? How many went on to have permanent employment with the health service?

Mr ROCKLIFF - My understanding, all of them. In 2022, 372 transition to practice nurses started their careers with the Department of Health.

Ms DOW - That was their graduate year, just to be clear? My further question was around how many were offered permanent positions with the state health service?

Mr ROCKLIFF - I will seek some information for you, Ms Dow. In the 2022-23 transition to practice program, 294 individuals accepted offers of employment over four rounds. The Department of Health is unable to provide data on which people were offered positions in the first rounds and subsequent rounds. I may have to take that question on notice. I mentioned the 327 figures, in 2022. Going back to 2013, the previous minister for health was talking about a target for 2014 of 115 graduate nurses. We have made significant inroads there.

Ms DOW - I'll put the question on notice.

My next question Premier is, what is the annual cost of nursing overtime across the health system? What are the total hours of overtime worked at each major hospital in the state?

Mr ROCKLIFF - While we are gathering that information, overtime for nursing staff increased from 3.59 per cent to 3.95 per cent 30 June 2022 to 31 March 2023.

That reflects the increased level of demand on our services. I mentioned that figure didn't I, 3.95 per cent of overtime as a percentage of total paid FTE by hospital, with the previous year, 2021- 22, 3.59 per cent. In terms of the dollar value, I'd have to seek the information on notice.

Ms DOW - I'll put that on notice.

Premier, what was the amount spent on double shifts last year, and what was the average number of double shifts done across each hospital, when it comes to nursing?

Mr ROCKLIFF - The use of double shifts is highest in areas requiring postgraduate and specialist nurses, including intensive care, emergency departments and psychiatric medicine. Alternative recruitment strategies and activities continue to be explored to support these areas. Double shifts are used in extraordinary circumstances to cover unplanned absences or personal leave, patient acuity and roster gaps. In the third quarter of 2022-23, there were 2718 double shifts worked statewide, compared to 2796 double shifts worked statewide in the third quarter of 2021-22, so there has been a decrease there.

Double shifts increased significantly from April to October, compared to the same period last year, in part due to increased attention on managing COVID outbreaks; furloughing of staff; requirements to stay home to care for children who are COVID positive; and increasing use of casual pool staff to fill roster gaps, resulting in a lack of available backfill for unplanned absences. Double shift data for 2022-23 year-to-date available data shows that across each region there were, in total:

July 2022: 1321
August 2022: 1111
September 2022: 1030
October 2022: 1174
November 2022: 964
December 2022: 1086
January 2023: 954
February 2023: 842 and
March 2023: 922.

So, there is a decrease there in double shift data.

Ms DOW - I will go to the next question because I am conscious of time, but thank you for reading that into *Hansard*. My next question is about the number of agency nurses

employed across the Royal Hobart Hospital, Launceston General Hospital, North West Regional Hospital and Mersey Community Hospital last financial year?

Mr ROCKLIFF - Thank you, Ms Dow, I might get Ms Morgan-Wicks to answer that question.

Ms DOW - And further to that, I wanted to understand what the employment costs were for each of those per annum; the costs related to accommodation and travel associated with those agency nurse positions; and what was their average hourly pay rate?

Ms MORGAN-WICKS - The usage of agency nurses is a contracted position and is not currently tracked through our current legacy HR system. It's tracked through our finance system, so we pay on invoice. I don't have that hourly rate data averaged per hospital with me today, but, as part of our implementation of the HRIS, we will be including all locums and all agency nurses as part of our HRIS system so that we can track that.

We are also looking to implement a nursing Dashboard which will include current employees; the usage of our casual nurses; and also, the demand for agency nurses, so that each hospital is then able to track that on a very available and transparent ward-by-ward basis as to the actual nursing resources that are demanded in each of our health services day-to-day, shift to shift.

Ms DOW - Thank you. The other question I had was in relation to locums, broken down across each of those areas. Are you able to provide the annual cost of locums across the health system?

Mr ROCKLIFF - Yes, I believe we are able to. We remain committed to employing nursing and medical staff through ongoing recruitment and retention strategies. Consistent with previous years, there continues to be vacancies with some specialist vacancies remaining extremely difficult to fill. This is not unique to Tasmania, and occurs both nationally and overseas. The COVID-19 pandemic saw increased costs associated with locums demanding higher rates, increased airfares, accommodation and hire car costs - none of which have decreased as a result of the pandemic.

In an ongoing effort to reduce locum costs, the department utilises a range of recruitment strategies including, where possible, recruiting permanent staff to fill vacancies; offering permanent opportunities to long-term locums; recruitment and retention allowance of 25 per cent for the north-west salaried medical practitioners; implementation of a 25 per cent market of retention allowance for psychiatrists working in the northern or southern regions; establishment of an agency-specific recruitment program enabling longer term fixed-term employment for our doctors in training, including ability to engage accredited registrars for the length of their training program and targeted campaigns through specialist recruitment agencies and specialist colleges, along with rolling advertisements for permanent vacancies.

The department has a panel of medical locum providers engaged via a tender process. The contracting of providers and the use of locum engagement business rules provides a standardised locum engagement process across the state. The current locum contract with each of the panel of providers expires on 30 September this year; and a new open tender process for the provision of locum medical licenses opened on 13 May 2023.

Ms DOW - Really interested in the cost, Premier.

Mr ROCKLIFF - Yes, okay. As at the 31 March 2023, the cost is \$57 147 193 and the percentage of that total budget is 3.49 per cent. Last year was \$52 857 039 - 2.57 per cent of total budget.

Ms DOW - Have you got them across each hospital site?

Mr ROCKLIFF - I've got across the north, north-west and south. Of the \$57 million and the locum costs, and this is as of 31 March this year, we have \$11 612 855 for the Royal Hobart Hospital, which is 1.84 per cent of the total budget; the LGH has \$16 643 266; the North West Regional Hospital has \$9 517 503; the Mersey Community Hospital has \$6 623 597; mental health has \$12 189 694; and for other locum costs that aren't specified in terms of service or region, there is \$560 278, so that brings the total to just over \$57 million.

Dr WOODRUFF - Thank you. I've got a bunch of questions, Premier, following on from previous annual reports and Estimates committees around ambulance ramping times, which have continued to worsen which is really concerning. You might need to possibly take some of these questions on notice; I'll leave it to you to work out. Could you please provide some figures for this current financial year, or do you want to bring someone to the table?

Mr ROCKLIFF - Dr Woodruff, have you asked all the questions you wanted to ask of Dr Veitch?

Dr WOODRUFF - Yes, for the moment, I think so. Thanks, Dr Veitch.

Mr ROCKLIFF - I could invite Jordan Emery up to the table -

Dr WOODRUFF - Good idea.

Mr ROCKLIFF - and congratulate him on his appointment as the new chief executive of Ambulance Tasmania.

Dr WOODRUFF - That has been formalised, has it? Congratulations.

Mr ROCKLIFF - Yes, very good.

Dr WOODRUFF - So in this financial year, by hospital, can you please tell me the percentage of patients arriving by ambulance who are transferred within 15 minutes of arrival and within 30 minutes of arrival, and the median amount of time a person waits to offload at each of the hospitals?

Mr ROCKLIFF - While we see if we can find that information. I've got some numbers here that might be of assistance.

Dr WOODRUFF - Only if it's that stuff, thanks, because I am just conscious of the time. I am happy to take it on notice, I don't need to hear more information you have, we can drown in it. I just want to know that information, thank you.

Mr ROCKLIFF - All right. Jordan, do you have the information that Dr Woodruff is seeking, or would you like to take this on notice?

Ms MORGAN-WICKS - While Jordan is looking to find that specific data, just noting on the transfer of care metric, which is unfortunately not recorded in the ROGS data for ambulance across Australia, but based on the AMA report card, Tasmania was second-best to NSW for transfer of care within 30 minutes, noting that the AMA reported us at 79.6 per cent transferred within 30 minutes, compared to NSW which was 84.8 per cent, but Jordan may have some more specific transfer-of-care data points.

Dr WOODRUFF - The specific things were by hospital, percentage transferred within 15 minutes and percentage transferred within 30 minutes of arrival.

Mr ROCKLIFF - Okay. I hope I'm reading this correctly. The Royal Hobart Hospital ambulance presentations in total -

Dr WOODRUFF - Arrivals?

Mr ROCKLIFF - from July 2022 to March 2023 were 19 092. The percentage transferred within 15 minutes was 52.2 per cent; percentage transferred within 30 minutes was 59.8 per cent; the number ramped was 9135; and the hours ramped were 17 837. If I go to the 2021-22 figures, the percentage transferred within 15 minutes was 57.4 per cent; percentage transferred within 30 minutes was 65.75 per cent; and the numbers ramped were 10 815.

For the LGH, ambulance presentations from July to year to date were 11 023. The percentage transferred within 15 minutes was 56.9 per cent; percentage transferred within 30 minutes was 62.7 per cent; the number ramped was 4753; and the hours ramped were 12 581.

For the North West Regional Hospital, ambulance presentations to March this year were 6113. The percentage transferred within 15 minutes was 78.6 per cent; percentage transferred within 30 minutes was 86.4 per cent; the number ramped was 1308; and hours ramped were 830. It seems to be a pretty reasonable improvement based on last year, although there are three months to go.

For the Mersey Community Hospital, ambulance presentations to March this year were 3615. The percentage transferred within 15 minutes was 86.4 per cent; percentage transferred within 30 minutes was 91.5 per cent; the number ramped was 490; and hours ramped were 280.

Jordan, do you have any more specific data on that?

Mr EMERY- I can provide some additional information statewide and by hospital on the average time in hospital, particularly since December last year.

Dr WOODRUFF - I was interested in the percentiles, the median wait time for a patient affected by offload delays for each of the hospitals. I wonder what the median wait time and the seventy-fifth and the ninetieth percentiles.

PUBLIC

Mr ROCKLIFF - I think Mr Webster has some information here.

Dr WOODRUFF - Excellent.

Mr WEBSTER - It may save time in that this is available on the Health dashboard and can be split by hospital and by categories of patient, but I can read it out.

Dr WOODRUFF - By the percentiles?

Dr VEITCH - No, this is the median number of minutes you asked for.

Dr WOODRUFF - The median wait time, yes, for fiftieth percentile, the seventy-fifth percentile and the ninetieth percentile.

Mr WEBSTER - On the website we do the fiftieth percentile and we do it by category, by month and by hospital, so do you want me to go through that?

Dr WOODRUFF - Is that for ramping?

Mr WEBSTER - This is the median time for transfer from the ambulance into the hospital.

Dr WOODRUFF - Yes, so the length of wait time.

Mr WEBSTER - Yes. I can read it out but it is available on the Health dashboard now.

Dr WOODRUFF - I am interested the ramping times.

Mr WEBSTER - The wait time is transfer of care delay time.

Dr WOODRUFF - So from turning up at the hospital in an ambulance to being actually admitted into the emergency department, not into the interior room but actually as an admitted ED patient.

Mr WEBSTER - Yes. Again, the wait time on the website is that time - it's not as in arriving at the hospital, it's the time from arrival to when you are seen to be admitted. As I said, I could read it out, but it is available and broken down on the website.

Dr WOODRUFF - Just to be clear, I am not talking about general people turning up to emergency departments. I am talking about people turning up in an ambulance. Not ED arrivals but ambulance.

Dr VEITCH - I don't think we have that.

Dr WOODRUFF - That's right, but it is available. That is what I am asking for. Is that possible, Justin?

Mr EMERY - Jordan.

Dr WOODRUFF - Jordan, I beg your pardon. Can we get that on notice then?

CHAIR - I am sorry, I thought he was going to take it on notice.

Dr WOODRUFF - The question was, can I put that on notice because it isn't available on the dashboard but I know it is collected?

Mr ROCKLIFF - Yes, absolutely.

Mr YOUNG - There is nothing more important than protecting our children. Can the minister please update the committee on the steps he has taken to ensure the safety of children in our hospitals and health system?

Mr ROCKLIFF - Thank you, Mr Young, and I agree with you wholeheartedly. There is nothing more important than protecting our children and we did discuss this this morning in the Department of Premier and Cabinet Estimates. This is why we instigated the Child Safe Governance Review of the Launceston General Hospital and human resources after the evidence we heard at the Commission of Inquiry. The review commenced in August 2022 and included a process for engaging with victim/survivors, established and facilitated by Dr Maria Harries, a review panel member and expert in child safety matters. The final report from the review contained 92 recommendations, which we accepted and are working to fully implement by the end of this year.

One key action being progressed is the development of a new statewide complaints management framework. The draft framework is currently in the consultation phase, with feedback already received being integrated into the next version, including the mapping of existing complaints processes across the health service.

A key component of this is a child safety and wellbeing panel who will review all child safeguarding concerns and complaints in hospitals and health services. Today I can announce that four members have been appointed to the panel, including Barbara Kingston as chair of the panel. Barbara is an experienced non-executive director and former senior executive with governance experience in national and multi-state not-for-profit businesses and public entities, including tertiary health systems.

Two further independent members have been appointed. Paula Davies is an experienced lawyer who has works in legal and policy roles with the South Australian Government and as a lawyer in private practice. Anne Stark has been a psychologist since 1974 and has practised extensively in various organisations in Tasmania and Queensland, with an emphasis on roles in family therapy and child psychology.

Importantly, the panel includes two consumer representatives, Erica Edmonds, who owns a legal practice undertaking workplace investigations and training with clients such as the University of Melbourne and Victoria Police; and Nix Berry, who grew up in Tasmania and has experience of child and health services in that context. Nix's achievements include being a student facilitator with the schools' partnership program at Latrobe University in Melbourne, working with secondary and middle school students from diverse and low

socio-economic backgrounds. Nix will bring an important consumer focus to the panel's work.

The establishment of the Child Safety and Wellbeing Panel is part of our Government's commitment to strengthening child safety in our hospitals and in health settings as well.

Ms DOW - Premier, what was the annual cost of ambulance ramping across the state last year?

Mr ROCKLIFF - We don't capture that data because that expenditure would - Dale, would you explain?

Mr WEBSTER - We don't capture that data. We employ the number of paramedics so it is a sunk cost in that the paramedics are already employed.

Ms DOW - That's quite interesting because last year we asked some similar questions around this and we were provided with a figure of about \$3 million in costs. I think it was related to those total hours, which was about 20 000.

Mr WEBSTER - We've calculated the hours ramped, which is what we have already read out at part of that data.

Ms DOW - I was pretty sure, Premier, that you were able to provide a figure last time, even if it were just for those 31 528 hours that you spoke about before, being the number of hours that ambulances were ramped.

Mr ROCKLIFF - If we had the average costs per hour, I guess we'd be able to work that through.

Mr WEBSTER - Last year that cost was drawn from some discussion occurring between the CEO of Ambulance Tasmania and I believe it was HACSU, it might have been one of the other unions. As part of those discussions, it had been calculated for that purpose. But it was actually was calculated by Mr Acker at the time so it is not something we capture as an ongoing figure.

Ms DOW - Something that you'd look to capture in the future?

Mr WEBSTER - As I said, it is a sunk cost, as I think the minister said. You can do the hours by the average wage per hour to calculate it but it is not something that is that useful in the management of Ambulance Tasmania. The critical issue is the transfer of care time, particularly because that is relevant to the patient and how we manage the patient's care through the process, rather than how we manage the staff through the process.

Ms DOW - What would be the average cost per hour?

Mr ROCKLIFF - We'll have to check that figure.

Mr EMERY - I think it would be fair to say, Ms Dow, it is highly variable. It would depend on the number of ambulances delayed at any point in time. And, of course, the target

PUBLIC

to achieve 100 per cent offloaded within 30 minutes, that initial 30 minutes wouldn't be captured because we would anticipate a period of delay to transfer a patient over to the hospital whilst triage, et cetera, takes place.

Ms DOW - Premier, will you commit to publishing monthly data on ambulance ramping?

Mr ROCKLIFF - We're increasingly putting more information on the dashboard.

Ms DOW - My question is about ambulance ramping. Could you provide that information?

Mr ROCKLIFF - We look at any improvements we can make when it comes to the dashboard.

Ms DOW - The other one that would be useful to see on there is around regional response times. You have the response times across the major centres and even that's available in the annual report. But there is nothing about regional response times, which we know are very bad at the moment.

Mr ROCKLIFF - The dashboard does provide very important overview aspects of the public health system: possible admissions, emergency departments, elective surgery, outpatients, ambulances, mental health, cancer screening and oral health -

Ms DOW - The question was about whether you would include ramping on there and regional response times. I know what's on there, Premier, thank you.

Mr ROCKLIFF - It is important because I am getting to the nub of it here. We first published the dashboard in 2015 as part of the One State, One Health System, Better Outcomes reform program. Since June 2021, the dashboard has been updated monthly, most recently on 26 May to provide statistics for the 13 months to the end of April 2023. In January this year we expanded the dashboard to provide a detailed breakdown of ED care. That includes the number of presentations by triage category and median wait time to be seen at each hospital.

In addition, the dashboard now also shows the number of patients who do not require an ambulance and who are being treated through Ambulance Tasmania's secondary triage pathway, information on births in Tasmania, patients supported through our police, ambulance and clinician early response team, and the number of enrolments through our highly successful COVID@homeplus program. Anything else that we can provide that's of interest to Tasmanians -

Ms DOW - I'm seeking a commitment from you, Premier, that you will publish that data.

Mr ROCKLIFF - We'll take that on advice and see what's possible. We have the RoGSdata that we publish which makes interstate comparisons. Tasmania has one of the fastest triple zero answer times in the country - 92 per cent answered in equal to or less than

PUBLIC

10 seconds. ACT is 95.1 per cent. South Australia is 92.9 per cent and Victoria is the worst at 65.6 per cent.

Ms DOW - Premier, we had Tasmanians ramped for over 31 000 hours at our hospitals last year. Can you tell me how many people died while they were waiting in ramped ambulances across the state last year?

Mr ROCKLIFF - I saw a media release that you put out that was factually incorrect around that matter. If I could ask Professor Lawler to speak to mortality that would be helpful.

Mr WEBSTER - Through you, minister, we have a robust process for capturing those events. They include deaths that occur within the emergency departments across our state. It is my understanding that we don't currently capture that information by location of patient, which includes either whether it's the treatment space where they are once they've been handed over to the department or by being on the ramp.

CHAIR - Premier, with city-based areas you've got your UCCs with bulk billing. What is being done to ensure our rural and regional communities have access to GPs, bulk billing, pharmacies and hospitals, ensuring population growth in these towns?

Mr ROCKLIFF - Thank you for that question. I've covered a bit of that earlier on around your last question. We can talk through that. Mr Webster if I could -

Mr WEBSTER - Through you, Premier, just to start answering that question. The single employer model that we're rolling out will roll out across the state, giving us increased coverage. Hopefully, by starting people in a pathway of learning in general practice in a rural area, we're encouraging them to stay in that area. I think Professor Lawler outlined the rural generalist pathway that's been developed, with the idea to increase the number of rural generalists, so you're staying in that rural area.

The announcement by the federal government in the federal budget to triple the payment to practitioners in rural areas to bulk bill was a significant uplift. At the time it was called the most significant uplift in that fee for 20 odd years. That will hopefully contribute to getting bulk billing in those areas. Getting the number of GPs up is also really important.

CHAIR - What about the pharmacies and the hospitals? That was part of the question as well. Can you expand on that?

Mr WEBSTER - Through you, minister, with pharmacies, we are doing a review of the scope of practice of pharmacists, increasing the use of the pharmacy as a community health hub. The minister spoke on this in an answer earlier. Increasing that, increasing the scope of nurse practitioners in rural areas so they can operate in rural areas. We are looking at community paramedics as a concept in rural areas. Increasing the number of health care workers and the scope of their work in rural areas is really important. A number of the after-hours GP grants that were given across the state by the minister through our program over the past few years have also gone to rural pharmacies.

PUBLIC

Dr WOODRUFF - Minister, one of the reasons Ms Dow and I are asking you these detailed questions is because it is not available in the Dashboard data and there's less information there than there used to be. One of the things I've asked you -

Mr ROCKLIFF - Is there?

Dr WOODRUFF - Yes. A lot of information disappeared about five years ago. I think you might remember, Rebecca? We talked about that. Here we go, we asked this question in Estimates -

Mr ROCKLIFF - We're increasing them. We're completing monthly data.

Dr WOODRUFF - You have gone back to monthly data.

Mr ROCKLIFF - No, no.

Dr WOODRUFF - It always used to be monthly. You stopped that.

Mr ROCKLIFF - No.

Dr WOODRUFF - Yes, you did.

Mr ROCKLIFF - No.

Dr WOODRUFF - You weren't Health minister; Mr Ferguson was.

CHAIR - Order, Dr Woodruff.

Mr ROCKLIFF - It was never monthly data. We were the first government to have the Dashboard, which was quarterly. The previous government of which you weren't part of, I accept that, we had to RTI the outpatient waiting list, for example. We've been very open and transparent and increasingly. It is now monthly.

Dr WOODRUFF - Let's talk about being more open and transparent. I asked you about Code Blue data last year on ramps, which is like a medical emergency. You said you weren't able to tell us. Are you able to tell us the number of Code Blue events for ramped patients in the last financial year at each of the hospitals?

Prof LAWLER - If I understand the question correctly, it was seeking specific data around Code Blue in each of the hospitals?

Dr WOODRUFF - On the ramp.

Prof LAWLER - I'm not sure that the information is gathered to that level of granularity. My understanding is that in the process of capturing deteriorating patients and the response, which is part of our response to standard 8, we have MET call or Code Blue databases in our hospitals that capture the information. I understand that it is captured to the point of department or unit location rather than the granularity of cubicle or treatment space. It's also worth noting that it's frequently the case within emergency departments that

responses to arrest or sudden deterioration may actually simply be through local activation rather than a hospital-based Code Blue activation. If we capture hospital-based Code Blue, an emergency department response may not be captured through that process. I wouldn't say that we would have the information to that level of granularity to hand.

Dr WOODRUFF - Okay. Can I get clarification through you, minister? Are you confident you do not collect information and cannot provide us with information on the ramp, not elsewhere in ambulances but on the ramp, on the number of Code Blues that are called. That is, medical emergencies that happen on the ramp before a person is admitted into the emergency department? Why do we not know this? We have been asking about this for years.

Is it possible to get that information or do you just not want to provide it?

Mr ROCKLIFF - I've been very open and transparent with that data.

Dr WOODRUFF - I know it is called; it must be recorded?

Mr EMERY- I suppose, Dr Woodruff, it's conceivable that some data might be available in a Safety Reporting and Learning System (SRLS), but not specifically as it relates to Code Blues. There might be occurrences where a Code Blue is called and a patient has had a simple faint or syncope, and that data might not necessarily be captured in the SRLS if there was not any particular harm to the patient.

Dr WOODRUFF - Okay. Mr Emery, is it something Ambulance Tasmania could look at collecting? Code Blues wouldn't be called for somebody for light reasons, they're for medical emergencies, like imminent life-threatening situations or potentially so. Can that be collected?

Mr ROCKLIFF - That's something we could consider, Dr Woodruff, as we upgrade our digital health system for example, and that data then can be much more accessible.

Dr WOODRUFF - Okay, thank you. How many SLRS, which is what Mr Emery referred to, have been made regarding situations in offload delay, or ramping, in this financial year, and how does that compare with the previous three financial years?

Prof LAWLER - One of the things I will highlight is the way we capture information through the SLRS. I take your point, Dr Woodruff; code blues aren't necessarily called for the kind of situations that Mr Emery has highlighted. But, it's important to note that Mr Emery has highlighted what we know of a situation at the end of a medical emergency, and code blues are called at the beginning. So, it may well be that code blues are called for any reason for a change in conscious state, or a change in observations, that may ultimately result in something that might not be as serious as otherwise considered.

We capture safety events of a very broad nature within the SLRS, and they are captured through a number of different categorisations: service delivery infrastructure - and certainly sometimes issues relating to ambulance off-load delay are captured in that, because there is a perception that they relate to infrastructure or service delivery matters; workplace health and safety; and what we describe as patient-client events, which are the adverse safety events that

we tend to capture for clinical issues that arise during the delivery of care. One of our challenges is that the current system that we use is a little bit difficult to search through.

As I mentioned, we can capture on the basis of where a patient is from a unit area, and it may well be that a patient on the ramp might be categorised as being within the emergency department, or they might be categorised as being under the care of ambulance. Similarly, there may not be an entry within that, that says a code blue was called, or the patient collapsed, or had a cardiac arrest. They are very different in terms of both the entry of information pertaining to the event, and also the outcome of the event. It is very difficult within the system we currently have to pull out the information you are describing.

We are in the process of procuring a replacement program for our safety reporting and learning system which will have a much greater level of granularity and searchability in order to provide us with greater intelligence about such things.

Dr WOODRUFF - Thank you, Professor Lawler. Minister what I am hearing, and what I have heard for years when I have asked this question, is that it's too hard. It's too difficult, we can't get that granularity. More and more people are being ramped for longer amounts of time in Tasmania. There is no doubt that people are dying from ambulances not coming on time; or dying because they can't get a bed in hospital; or dying while they're ramped without being able to access even the emergency department services.

If we don't collect the information about what's happening, how can we have a situation that responds and improves what is going on? If you don't even know how many people are having code blue events or any other emergency or any other problems in ambulances, then how can we respond? It's almost as though you don't want to know. What are we doing to collect the information to understand the increasingly large number of people in life-threatening conditions that are left outside of a hospital, unable to access medical emergency services - including, I need to say, for Mrs Schramm who died not even getting a bed, or any palliative care or dignity. What are we doing to make sure that no-one else suffers that indignity and the death that she did?

Ms MORGAN-WICKS - We absolutely do care about adverse patient outcomes that occur across all of our hospitals, and Professor Lawler has detailed our safety risk and learning system, our SLRS system-

Dr WOODRUFF - You can't provide it to me.

Mrs MORGAN-WICKS - which has recently been upgraded to provide information. We have been very open in terms of our digital health transformation program, which is some \$475 million over the next ten years, which we are investing to upgrade legacy and ageing health, digital infrastructure. We work together with each of our partners - for example, ASIM - in the collection of indicator data within our EDs and have committed to the ASIM indicators in terms of length of wait. We also publish a significant amount of data in the ROGS each year, noting that this is a problem, not just in Tasmania but in every single state and territory in Australia.

Dr WOODRUFF - Will you commit, minister, by the next Estimates to being able to answer all these questions? Your Government has had years where I've heard, 'Oh, it's really

hard'. The data is there; they're not being collected. Meanwhile, people are dying. These may be avoidable situations - of course they might be - but they can't be responded to if we don't know what's going on. You don't know what's going on.

Mr ROCKLIFF - I accept that each adverse event is responded to.

Dr WOODRUFF - By calling the Coroner when someone dies. That's often what happens. You didn't answer my question. Could you commit to having that information available, instead off into the never-never?

Mr ROCKLIFF - Yes, I'll answer your question. Professor Lawler, on that last point.

Prof LAWLER - I make two observations. I don't believe there is an acceptable rate of cardiac arrest or medical event which would prompt us to work harder to address ambulance ramping or offload delay. We recognise that there is not only a challenge with respect to the volume and number of ambulances ramped, but there is a clear body of evidence that indicates that there is a potential increase for adverse events in the presence of ramping. That's why we have undertaken a number of processes and reform initiatives, including the process for escalating concerns around ambulance ramping. Mr Emery and hospital leaders have worked on that over the last three to four months to ensure that both patients and their families, and also staff members, are able to appropriately escalate their concerns to those individuals within the health system that have the authority to give effect.

There is also clear work around the transfer of care policies in which there is a marker and a clear process delineated - much in line with what occurs within New South Wales - to address and escalate, not only to ensure patients are off the ramp and into cubicles earlier but also to ensure that ambulances are free and available to respond in a timely fashion to priority zero calls.

I consider it's also quite incorrect to indicate that our sole response to adverse events is to call the Coroner when people die. We certainly do call the Coroner when people die, and it's appropriate to report such cases to the Coroner in line with the act. We have an entire safety event management policy that is utilised across all of our publicly-funded health services in order to identify, categorise, investigate and respond appropriately to adverse events, not only when harm is occasioned to a patient but also when a near-miss occurs, so that we can learn the lessons and reform and advise our care delivery and structural processes.

This results in hundreds of recommendations every year. It results in root cause analysis and London Protocol investigations of SAC1 and SAC2 events, respectively. We have reporting requirements on the health services around the time limits within which they have to provide an initial update and a final response. We have regular meetings across our health services to ensure that those recommendations are not only noted but implemented.

There is a comprehensive process of safety event management monitoring and response that leads to better and safer care for Tasmanians.

Dr WOODRUFF - You can't tell us what's happening, because you don't provide the data outside the hospital.

PUBLIC

Ms DOW - Premier, when will the station and clinical services master plan that you committed at last Estimates to release in November of last year be made public, and how many positions does it say that we are short with paramedics across the state?

Mr ROCKLIFF - Which master plan?

Ms DOW - Station and clinical services master plan - the OHRC report.

Mr ROCKLIFF - Thank you for the question. We are investing at record levels across our health system including Ambulance Tasmania (AT). In April this year, a median emergency response time as of 1 June 2023 was 13.8 minutes. This is the shortest time in 18 months, and I commend all our hardworking paramedics on that result.

In relation to our commitment to employ an additional 48 paramedics across the state, I can confirm this was achieved ahead of time. We didn't stop there. Last year's budget also included funding for an additional 11 paramedics between Sorell and Huonville. We have also invested in new infrastructure, with 21 new ambulance vehicles commissioned over the last financial year, and we are continuing to work to build new stations across the state, including Bridgewater, which I was pleased to be at the opening of just a few weeks ago.

Given we were able to recruit the additional paramedics ahead of time, this meant our review of ambulance service demand could also be brought forward. Ambulance Tasmania engaged ORH to conduct a 10-year review. ORH specialises in taking an evidence-based approach to forecast future demand rates for emergency services such as Ambulance Tasmania, and today I can table a final report from ORH.

The modelling in the report was based on achieving a potential target median emergency response time of 10 minutes in urban areas and 15 minutes in rural areas over the next 10 years. Work will now occur to understand from a process improvement resourcing and infrastructure perspective what would be required for Ambulance Tasmania to reach this target by 2031-32. This is a journey we need to take our employees on with us and, as such, the department will be consulting our staff and volunteers on whether this target is appropriate and achievable given the details set out in the report.

The report also proposes three primary mechanisms to improve response performance through until 2031-32, including process improvements such as activation times, mobilisation times, time on scene, time at hospital and number of calls managed through secondary triage. Furthermore, the report proposes four potential new locations under the heading of 'location optimisation', including Legana, Cygnet, Sandy Bay and Snug.

The final proposed recommendation is to further enhance frontline resourcing, including through options such as expanded community paramedic positions and increased resourcing in identified geographic locations to respond to increasing demand over the next decade. The proposed enhancements will require careful consideration and staging, including new infrastructure, phased equipment to accommodate graduate paramedics and an assessment of operational and volunteering models to support service delivery changes.

The department will now work closely with staff, unions and our volunteers to seek feedback on the report and to create our 10-year master plan. Structured information sharing

and consultation sessions will be facilitated by the Ambulance Tasmania executive team over the coming weeks and I encourage all our valuable and valued employees who are interested to participate.

Chair, I have Ambulance Tasmania's final report by ORH titled Plan, Prepare, Perform that I have just spoken of and I will table that officially for everyone's consumption.

Ms DOW - I have another question for you, Premier, in relation to the Smithton ambulance station. I am sure you will all be aware what happened last week in Smithton where there was an elderly gentleman who waited five hours for an ambulance with a fractured hip and pelvis. I'm sure you will agree with me that is entirely unacceptable. Following that incident and the local media reporting on that, the THS provided a response to say that the Smithton station was a double-branch station rather than a single-branch station. It is in fact a single-branch station and therefore there aren't paramedics there 24/7 currently. Further to that, why wasn't there a volunteer response in place initially to provide care for that gentleman and his family? Why were they just left waiting for when the next ambulance could get to them? Why wasn't there a paramedic available in Smithton, as your Ambulance Tasmania website says there should be? Why did that happen? Why was there no volunteer response?

Mr ROCKLIFF - Thank you for the question. I acknowledge the circumstance of the gentleman and my heart goes out to his family as well. It must have been a very traumatic circumstance for their loved one. I appreciate the difficulties for the family. Smithton ambulance station operates as a single-branch station where a paramedic is on duty during the day and is on call overnight. Paramedics will also work with volunteers when available. The stations at Wynyard and Burnie will also respond to cases in the area if required.

I am advised that when a community member calls triple zero the call is triaged by a specialist emergency call taker using a medical priority dispatch system. Ambulance Tasmania triages cases is to prioritise those that require an emergency response.

Recruitment of paramedics and volunteers is a key priority for our Government and Ambulance Tasmania and we've spoken about that today. We're also implementing new initiatives such as secondary triage and community paramedics to care for low-acuity patients who may not require hospital treatment.

In addition, we are undertaking a review of ambulance service demand across the state, of course, the outcomes of which will help guide the future investments into our service which I've spoken about.

Ms DOW - That's not much help to this family, Premier. Why wasn't there a paramedic available?

Mr ROCKLIFF - Volunteer or?

Ms DOW - Both, either.

Mr EMERY - Through you, minister, thanks Ms Dow. You're correct, it is a single-branch station and we have paramedic coverage 24/7 at those locations. During the

daytime they are rostered shifts and during the night-time they are on call. Smithton experiences, if my memory doesn't deceive me, approximately two emergency incidents a day, or two triple-zero incidents a day. On occasion, those incidents can occur in quick succession or they can occur whilst a transfer is happening that takes the paramedic and volunteer out of the Smithton township. In those circumstances, as the minister said, we employ a dynamic deployment model where we might move resources from Wynyard up into the Smithton area or move resources from Burnie up to Wynyard and then stage the Wynyard resource in Smithton.

There is a unique challenge about Smithton at present in that we have nine volunteers currently working at Smithton station as a result of some very successful community volunteer recruitment campaigns, however those volunteers are currently at level 1, which is the lowest level of volunteer training. They will go through a natural progression process as they increase exposure and skills in volunteering with Ambulance Tasmania, but from a patient safety perspective, our policy is not to dispatch volunteer ambulance officers who are at level 1 alone, because there are both risks to the volunteer responding without sufficient experience and of course risks to patients as well to send an inexperienced volunteer.

That doesn't detract for a moment from the incredible service of our volunteers right across the state, and as I said, we have nine outstanding level-1 volunteers at Smithton who will continue to develop into more senior volunteers and be able to be dispatched to those cases as they build that skillset.

Ms DOW - Further to that, I was advised by the local community that some time ago there was the provision of an emergency response vehicle in Circular Head that was manned by volunteers and it was able to provide that immediate service to the community in an emergency where the paramedic wasn't able to be present or was taken elsewhere to meet demand. What happened to that resource in the community and why has that changed?

Mr ROCKLIFF - To my understanding the vehicle is still there. Perhaps the training level wasn't appropriate for the vehicle - Mr Emery?

Mr EMERY - That's correct. There are two ambulance vehicles at Smithton station, a normal Mercedes-Benz Sprinter ambulance and a four-wheel-drive. Of course volunteers could ordinarily respond in a four-wheel-drive ambulance or the Mercedes-Benz Sprinter. On this occasion, the ambulance was not dispatched with volunteers because of the skill level of the volunteers, it was not the absence of an available ambulance.

Ms DOW - I have a further question to Mr Emery, if I may, through you, Premier. I want to ask whether you think the employment of those permanent 97 paramedics across the ambulance service is enough to meet current demand; will that make a difference or is that merely filling substantive positions?

Mr ROCKLIFF - That was a commitment we made earlier this year which we will see through with the Budget. The permanent funding for 97 fixed-term Ambulance Tasmania and COVID-19 established positions we announced in February this year. Our funding for the 97 positions is included in this Budget, as I said. It includes both paramedic and operational support roles, with \$56 million over four years allocated in the Budget to fund those positions. The funding of these positions represents a significant investment in Ambulance

Tasmania and the health sector more broadly, and recognises the increasing demand for emergency medical services across Tasmania.

Ambulance Tasmania is currently working closely with the Department of Health to permanently fill the positions. Ambulance Tasmania senior managers are currently meeting weekly to transition paramedics to positions from fixed term to permanent employee. The transition of on-road paramedics and graduate paramedics who have recently attained their authority to practise as independent paramedic practitioners to permanent employment is being progressed as a matter of priority. Following the initial focus, permanent employment to paramedic supervisor, manager and specialised support roles will be progressed. Any current paramedic vacancies and those resulting from promotions will be filled from the current paramedic employment merit list and future paramedic recruitment campaign.

Mr Emery would you like to add value to that and answer Ms Dow's question?

Mr EMERY - Ms Dow, the baseline modelling produced by Operational Research in Health shows that the best response performance in the state is currently in the north west. That is unsurprising because the significant portion of the 97 COVID-19 funded positions were based in the north west of the state. Perhaps you will be aware that the current median emergency response time in urban areas in the north west of the state is about 10.5 minutes, which is a better response performance than the south and the north.

The plan, as the minister touched on earlier, describes ways to further improve response performance across the state. Organisational efficiencies are one of those measures, new locations or location optimisation is one of those measures, and resourcing enhancements are one of those measures as well.

It is not quite that simple. We need to understand exactly the models of care we would seek to employ. We have had some very significant success with our community paramedic program. That gives us some indicators of potential benefits in response performance by expanding the community paramedic program, even in lieu of perhaps increasing the number of emergency ambulances. It will ultimately land in the middle but that is where the consultation process is incredibly important so that we have the right resources for the future and the right models of care to meet community needs in the north west and across the state.

Ms DOW - Premier, how many paramedics short do you estimate that we are across Tasmania at the moment?

Mr EMERY - Ms Dow, I don't want to sound evasive. The modelling is future-focused so it looks over the next 10 years. It certainly talks about resourcing enhancements over the next 10 years and a phasing of that which aligns with certain infrastructure. We don't currently have any infrastructure at Legana, Snug, Cygnet or Sandy Bay, where the ORH report proposes new locations, so it will be a phasing of resources over that time. The reason it is difficult to give you an exact answer is because it would depend, at least in some part, on whether we adopted a community paramedicine expansion as one part of it or whether we rolled out increases in frontline paramedic positions, or a combination of both. That would determine the final number of positions over the 10 years.

Mr ROCKLIFF - Ms Morgan-Wicks would like to add value to the question.

Ms MORGAN-WICKS - The value of the ORH report, which is some really critical data and modelling which will then inform a 10-year strategic plan for Ambulance Tasmania. It identifies the business processes that we need to optimise and get better, it identifies the exact locations where, over 10 years, depending on the KPI that we are heading to. If we are going to head to a 10-minute and 15-minute response time KPI, it says, right, you need to look at these locations with these resources, which may require built infrastructure, which we then need to stage into a 10-year plan. That is why we are going to go back to our staff. ORH already consulted staff through the formation of the report. We will go back. What is their feedback in relation to this data and modelling? How can we then appropriately stage a 10-year plan for AT to get us to 2032, if that 10- and 15-minute response time KPI is actually chosen.

But we know from our own staff, who will tell us these are the numbers of paramedics that we can actually take on, particularly in terms of supporting graduate paramedics at any point in time. So we need to stage that, together with the infrastructure investment. But not forgetting the optimisation of our business processes, so activation times for all ambulances, et cetera.

Ms DOW - I have another question around the Campbell Town ambulance station, which I understand was designed with the living quarters clad in iron. That has resulted in difficulties with pagers and phone signals, and radios not working while the paramedics are on duty. That has led to them having to sometimes put the pagers outside the windows to get them to work, hanging them on the side to hear callouts. Are you aware of these problems, Premier? And what are you doing about it?

Mr EMERY - It is the first I have heard of it. I have attended Campbell Town station on many occasions and met with staff there on many occasions, and I have not had this matter raised with me previously, I'm sorry.

Ms DOW - Would you commit to following it up for me, please, Premier?

Mr ROCKLIFF - Yes, I will.

Mr YOUNG - This Budget invests \$180 million to continue the Government's digital transformation across our health system. This strong commitment to the future delivery of health services across our state has been welcomed by a number of organisations. I am interested to understand what is currently being achieved through digital technology across our health system.

Mr ROCKLIFF - Thank you very much, Mr Young. The role that digital technology can play in improving patient care, providing better data on patient management and reducing our reliance on manual processes is extremely important to our Government. This is why we are implementing our 10-year digital health strategy.

The Department of Health has a dedicated team of ICT professionals who already provide great innovations for our health system. The Health ICT report 2022-23, which I will table today, outlines a number of initiatives which have been implemented across the financial year.

Telehealth and virtual care have been a significant innovation across our health system. We have seen the implementation of COVID@homeplus as one example which has delivered benefits to the patients treated. In fact, as of 4 May 2023, 21 161 patients have received treatment through remote monitoring, equating to 127 812 patient-care days, with 207 478 observations recorded.

This financial year, we have implemented our eReferral system across the Launceston General Hospital, the Mersey Community Hospital, and North West Regional Hospital. The system provides general practitioners with a more connected and secure platform to refer patients to outpatient services.

Building on this, we will be implementing a digital outpatient management solution, or digital front door, later this financial year, which will allow patients to securely access information relating to referrals and appointments.

We have also commenced a trial of free wifi for patients and visitors within cancer services in the north and north west, and the paediatric ward at the Royal Hobart Hospital. Further services will be rolled out in the coming months.

The key part of our investment into digital health focuses on cybersecurity as well, and we have established a dedicated cyber operations function, including an instant response plan to test our cyber security readiness and resilience. We will continue the implementation of our digital health strategy to deliver more contemporary, connected and innovative care to all Tasmanians.

Chair, I have the Health ICT highlight report for the end of financial year 2022-23 from the Department of Health, which I would also to table today.

Dr WOODRUFF - Minister, is it true that there are plans being discussed for shipping containers to be used as ramp space while development work at the hospital is underway?

Mr ROCKLIFF - Are you speaking about the Royal Hobart Hospital?

Dr WOODRUFF - Yes.

Mr ROCKLIFF - Mr Gregory, can you verify the question?

Mr GREGORY - Through you, minister, there is quite a bit of work to do in laying out the staging for redevelopment at the Royal. I'm not aware that there are any plans to use shipping containers.

Dr WOODRUFF - No? Okay.

Can you provide us with some information from Ambulance Tasmania? Maybe, Mr Emery will be able to provide it. It might be too detailed to give to the committee. It could be on notice if that would work?

Mr ROCKLIFF - What you are about to ask?

Dr WOODRUFF - Yes, it's in relation to shifts within Ambulance Tasmania by shift type - like, day shift, afternoon shift, night shift, and by region. My question for the last financial year is how many shifts have gone unfilled by paramedics, by shift type and by region?

Mr EMERY - Dr Woodruff, I don't have that broken down by shift type. We'd need to gather that information.

Dr WOODRUFF - Okay. Can I put that as a question on notice?

Mr ROCKLIFF - Is that information accessible?

Dr WOODRUFF - It is, I've seen the roster before. It is collected.

Mrs MORGAN-WICKS - Yes, we do have rosters. We have thousands of rosters across the Ambulance Service, so we'd need to determine whether we have that collected in an integrated form. We're unsure whether we have that information without a significant manual roster by roster collation of the data.

Dr WOODRUFF - Thank you. I'm not sure how much work it is but I think it is incredibly important for Tasmanians to understand how many rosters are not filled by paramedics across the state. I get regular reports, particularly in regional areas, that they're not filled. We need to understand the extent of that and whether these reports are true or not.

Mrs MORGAN-WICKS - That is probably, again, one of the very valuable reasons why we are replacing our legacy and ageing HR digital infrastructure with our brand-new HRIS system. I've had feedback from staff members right across the system, from paramedics to doctors to nurses to allied health professionals into our kitchens, et cetera, in terms of their desire to see the new rostering module of HRIS implemented. The team is working very hard to get that information into the system. In addition to assistance and saving of time with rostering, the saving of time for our payroll teams in having to manually enter that information currently, we will also have a better monitoring suite for the data that is uploaded into these modules in HRIS.

Dr WOODRUFF - Thanks. I am also very excited about the future but we're just talking about what's happened in the last financial year.

Could I put on notice for that information to be provided? I know it is available because we have seen rosters. The roster master sheets do have that information available to them.

Mr ROCKLIFF - I think we're working on some information at this present time, Mr Webster, as a result of the question in the Legislative Council. Is that correct?

Mr WEBSTER - Yes. We are working on the data by location but not by day versus night shifts. We're putting that together at the moment

Dr WOODRUFF - You could provide me that information on notice for the last financial year by region?

Mr WEBSTER - By region, minister?

Dr WOODRUFF - By region, by ambulance station, yes.

Ms DOW - Do you get regular updates on that, Premier, about the vacant shifts across the state?

Dr WOODRUFF - I know that there are night shifts regularly not filled in some of these stations. We need to know that. People need to know that. We need to know the extent of under-resourcing. It's not Mr Emery's fault, it's just where you are, so we would like to see that, please.

Mr ROCKLIFF - We will work to provide that information if possible.

Dr WOODRUFF - I will put it in writing. My second question is about annual leave. We understand the paramedics in the south of the state and it might also be in the north and the north west have been frustrated by their inability to get leave granted from Ambulance Tasmania for the coming year. It seems quite a lot of staff will be banking significant amounts of leave as their requests to take it have been denied. Can you please tell me why that is happening? It is concerning, given that paramedics are under so much pressure in very intensive and traumatic work conditions.

We think it is important for people to have their annual leave entitlements taken when they need it. Can you explain why it is not happening and why you don't think it would contribute to extra burnout?

Mr ROCKLIFF - Recreation leave taken as at the 31 March 2023 was 7.42 per cent of paid FTE, which was also higher than last year's percentage of pay leave at 6.54 per cent of paid FTE. What other information do we have?

I have recreation leave by award as percentage of paid FTE. Would you like those figures?

Dr WOODRUFF - What does that mean? I am asking about the year ahead. People are asking for leave and they are not being granted. It is about people who are not getting access to the leave that they want. We understand that is the case in the south.

Mr WEBSTER - Through you, minister. The level of staff taking leave or the percentage of people on leave over the year is relevant. In 2021-22, 9.63 per cent was involved in recreation leave. So far this financial year it is 11.21 per cent, which indicates an increase in the taking of recreation leave. There is a process with rostered staff of needing to nominate and it be built into rosters. The statistics are showing an increase in the granting of recreational leave, not a decrease.

PUBLIC

Dr WOODRUFF - For the coming year, just to be really clear, through you, minister, Mr Webster, are there any staff who have been directed that they will not be given recreation leave for 2023-24?

Mr WEBSTER - I am not going to say there are no staff that have had that. As I just explained it's part of rostering so they may have asked for a period of leave that doesn't fit with the rostering for that particular period.

Dr WOODRUFF - For the whole year. I am talking about across a year where people are wanting to take their leave entitlement. I accept that they cannot get it at Easter or at Christmas. This is not what we are hearing. We are hearing it is like we will not be able to grant leave this year.

Mr WEBSTER - Through you, minister, I will pass to Mr Emery for that level of detail.

Mr EMERY - Through you, minister, and thanks Dr Woodruff. We do have processes for how paramedics and all employees apply for leave. On occasion we will see higher applications for leave during school holiday periods and over the Christmas period. We will, periodically, have to decline some applications for leave to ensure that we can continue to maintain operational rosters over that period time.

There wouldn't be an occasion where an employee would be told 'under no circumstances can you take any leave this year'. It may require a process wherein which their local leader and the paramedic negotiates on days that might work, to ensure that we don't have roster holes and no paramedic coverage because too many people are on leave at any period of time.

Dr WOODRUFF - Thank you. You are confident that that isn't the case. People haven't been told 'You can't take leave this year'.

Mr EMERY - I'd be very confident, Dr Woodruff, that no paramedic was told 'you're not entitled to take any leave this year'. It would be a negotiation with the employee about what leave blocks are available, so that we can carefully spread our resources and ensure we can provide emergency coverage.

Dr WOODRUFF - Thanks. I wanted it to be on the record that there is widespread frustration amongst paramedics - at least in the south of the state - that it's extremely difficult to get leave because of the lack of paramedics. That is really having an effect for some people on their capacity to manage the trauma and stress of their work.

In relation to the MYPOL support system, which was something that we've talked about previously to manage trauma and psychological distress, can you tell us if there's been any change, Premier, to the number of paramedics who are tapping into the MYPOL support system?

Is there any other support that is available to them?

Mr ROCKLIFF - Thanks very much, Dr Woodruff. Mr Emery?

Mr EMERY - I don't have the immediate number available for you, Dr Woodruff. In some cases, the engagement with MYPOL is confidential and independent of the employer. We continue to have a range of services available, including through the Wellbeing Unit that we share services through, alongside our colleagues in the Department of Police, Fire and Emergency Management. That includes access to specialist psychologists including a panel of psychologists that can provide that support to paramedics.

We have our critical incident stress management program or CISM, as you may have heard it referred to, which provides immediate care in the aftermath of a critical incident.

There are wellbeing program officers across the state who work within the wellbeing unit and can provide a range of referrals to a vast array of psychosocial support services, and we have four employee assistance program providers in the state that paramedics can tap into 24 hours a day, seven days a week.

The final program I'll touch on, Dr Woodruff, is we have a dedicated internal peer support officer program with 26 specially trained, peer support officers across the state. They can provide peer-to-peer support for paramedics, volunteers and, indeed, leaders and corporate staff as well experiencing mental health challenges.

Mr ROCKLIFF - Further to your Budget-related question, I'm advised there is \$1.5 million per annum in additional funding to boost health and wellbeing support for all emergency service personnel and volunteers, and all Ambulance Tasmania employees and volunteers can access Department of Police, Fire and Emergency Management wellbeing support officers.

The Department of Police, Fire and Emergency Management also provides access to and administers online physical and mental health assessments through the MYPOL. Following completion of the assessments, coaching assistance provided by Allied Health professionals and other more immediate interventions are in place for individuals, as required. In 2021, as we've discussed before, I believe, AT engaged a third-party organisation - Frontline Mind - to conduct two organisation-wide resilience scans to assess the health of the organisation.

The resilience scan survey results were shared with staff in late 2021 through communications and a series of staff consultation forums. The feedback from the forums informed the Ambulance Tasmania culture improvement action plan in July last year. AT committed to the implementation of 73 actions under the action plan which are being monitored and reported against on a monthly basis.

Mr YOUNG - The Tasmanian Government has a 10-year, \$1.5 billion health infrastructure plan to ensure that we have contemporary health facilities across the state. Can the minister please update the committee on the progress of this plan, and outline some of the projects which have been delivered for the community?

Mr ROCKLIFF - Thank you, Mr Young. You've mentioned the investment we're making and the 10-year health infrastructure plan. Today, I will be tabling the Department of Health End of Year Highlights 2022-23 document, which outlines a number of key

achievements. I will highlight some of them and, of course, I have mentioned some along the way.

The \$5.8 million antenatal clinic at the North West Regional Hospital in August 2022, has brought together a range of antenatal services from across the hospital into one location, so that women's outpatient journey from maternity is delivered at the same place.

In March this year, the slab at the Mersey Community Hospital was poured for the new E block which will house new outpatient clinics. Building works are anticipated to be completed by the end of next year.

At the Launceston General Hospital, a number of key milestones were met, including the new dedicated women's and children precinct opened last year, and the new administration and learning building which incorporates the Anne O'Byrne Education Centre, opened in November last year. In January this year, the first stage of the emergency department expansion was opened, providing an additional 28 beds. We also completed the first stage of the endoscopy short stay suite in December. In March we completed the cardiology unit redevelopment, providing a temporary space for patients and also allowing for the establishment of a dedicated sleep centre. In May, we also completed the intensive care unit expansion, providing 12 new beds.

Planning for the future infrastructure needs for Tasmania is also a focus of our Government. We have released master plans covering all of our major hospitals, and as discussed earlier, our new vision for St John's Park. Work will also commence on a master plan for our district hospitals, in line with the place-based approach to rural health services as outlined in our long-term plan. I want to thank everyone involved for delivering these critical projects for Tasmania.

Chair, we have the infrastructure services group End of Year Highlights 2022-23, and I will table four copies of that document.

Ms DOW - Premier, the people of Bicheno have been waiting a very long time for their new ambulance station. In February, you announced the site of 60A Burgess Street, Bicheno, as the proposed site.

Can you confirm for me how that site's progressing, and whether you will compulsorily acquire the site - or will there need to be commercial negotiations around the site?

Mr ROCKLIFF - Yes, most certainly. I will throw to Mr Gregory but I will just make some opening comments about Bicheno.

I was at Bridgewater the other day, and that's a terrific example of us delivering on new critical infrastructure. In the first six months of 2022-23, Ambulance Tasmania responded to 55 592 incidents compared to 52 559 in the same period 2021-22 - an increase of 5.7 per cent. I recognise that, to meet increasing demand, we need to be focused on supporting and investing heavily in AT to ensure Tasmanians can get the response they need, when they need it.

PUBLIC

The new Bicheno station will provide a contemporary purpose-built facility that will house two ambulances to service the rural region and allow for further expansion of services as the community continues to grow into the future.

In response to your question, following the community consultation processes, which included an open day information session and public display, I recently announced the site for the new station at 60A Burgess Street, Bicheno. This site is the best location to minimise response times for the ambulance service to call-outs, which data shows are primarily within the township.

I understand that a development application in relation to 60A Burgess Street has been lodged with the Break O'Day Council and the Department of Health are working through the acquisition process, however I'm advised this does not prevent the council from considering the application which is being lodged.

CHAIR - Premier, can I just have a point of clarification? Did you say Break O'Day Council? Bicheno would be in Glamorgan-Spring Bay.

Mr ROCKLIFF - Glamorgan-Spring Bay Council. My apologies.

Dr WOODRUFF - Maybe this is future thinking.

Ms WHITE - Premier, you'd be aware that the site you've selected is the same site that the community originally expressed concern about, which led to the consultation, and instead of listening to them, chosen the same site. There has been a DA put in for a completely different development, not for the ambulance station, which the council is considering. At what point do you look for an alternative site, given the proponent is actively looking to develop that precinct into a residential and retail development and is not open to the department purchasing that land to build an ambulance station?

Mr ROCKLIFF - It's been quite an involved process, Ms White, so perhaps I'll hand over to Mr Gregory to take us through the journey.

Mr GREGORY - There was some opposition to the site being used. We then ceased the compulsory acquisition process and went through quite extensive consultation. We don't normally do public consultation to talk about other people's land; that's not normally the process. We had been engaging with the landowner and reached an impasse there, so it was agreed that we would withdraw the compulsory acquisition notice while we did some further consultation.

The outcome of that further consultation was in fact quite strong support for that site as the site for an ambulance station and strong recognition of the reasons why we originally chose that site, so we have recommenced the compulsory acquisition process and the notice to treat was actually issued today.

Ms WHITE - So you will be seeking the compulsory acquisition of that site?

Mr GREGORY - Yes, that's correct.

Ms WHITE - What are the consequences there for the Government given there's been a DA lodged and I believe it's more than a \$10 million development that is proposed to be constructed on that site in terms of the compensation you'd need to pay the developer because they won't be able to build that project because you're going to compulsorily acquire that site? What change is there to the valuation?

Mr GREGORY - The valuation of the land is taken as the best and highest use of the land at the time the notice to treat is issued. That remains unchanged while there's no approved subdivision. The Valuer-General looks at the parcel of land, looks at its zoning and looks at what is its best and highest-value use, so that would not in fact change between the original notice to treat and current notice to treat.

Ms WHITE - It seems like a controversial step to take, Premier, given the proponents were serious; they've lodged a DA and they've released designs for that part of town, which is the main street, to develop retail and residential and we've got a housing shortage, particularly in that part of the state. It's quite controversial for you to proceed with the compulsory acquisition of that land when I believe there were alternative sites that were pointed out to the department. Maybe they weren't exactly the same as this one, but surely you could find a compromise that means you don't scuttle a development for Bicheno that would have created lots of jobs and brought new housing and commercial opportunity for the town?

Mr ROCKLIFF - I acknowledge there are some concerns about the use of the site. The Department of Health have always sought to work cooperatively with the owners of 60A Burgess Street, engaging in a number of discussions and preparing alternative ambulance station and land acquisition proposals for the owner's consideration. I acknowledge what you say and I also acknowledge the value of having a brand-new ambulance station to service the community as well, which needs to be taken into consideration

Ms WHITE - No doubt, but couldn't the community have had both? That is the point I'm making. Surely there could have been a compromise rather than taking this I think quite controversial step? There were also concerns about the design of the ambulance station. I believe, from memory, that the design the department had circulated was similar to the Campbell Town design and the community were really resistant to that type of development on the main street; it's an iconic tourist town and they want development there to be keeping with the location. Can you commit to working through the design matters with the community so that they don't have a tin-iron shed built on the main street?

Mr ROCKLIFF - I am advised that it's part of the development application. Shane, do you have anything further to add?

Mr GREGORY - We did a lot of work with the owner of the property and we're not acquiring the entirety of 60A Burgess Street, just a component of it, part of it. We worked very closely with the owner to minimise the acquisition take. We put forward a number of design options to the owner to minimise the amount of land we needed and at their request, we even put driveways in different locations and access roads in different locations, so we did quite a bit of work with them to minimise the impact on the residual property.

It is true that initially there was pushback against the location and the design. When we went through the broader consultation process, the vast majority of people were in favour of

the site, were quite happy with the concept design and the location, so once we went to a broader consultation with the community we got a much more favourable response than just a smaller group of people expressing opposition.

We looked at a number of sites put forward by the community. They were either zoned incorrectly, they didn't have an adequate footprint or they were in the wrong part of town. One of the sites suggested, for example, was the industrial estate several kilometres outside of town, which is a significant distance away from where the main cluster of call-outs are, and would have added a number of minutes, possibly five to 10 minutes, to the response times.

Ms WHITE - I thought they wanted to move the council depot out there and you could've used the council depot.

Mr GREGORY - The council depot doesn't have direct access to the road network. The council's buildings are built partly on crown land and partly on road reserve, which would actually require the closure of a road, and the silos and shape of those two combined lot and road doesn't actually work, and it would also drop the ambulances smack-bang into the middle of the school zone, which is something we've been trying to avoid through the whole process. There were a number of sites put to us and we looked at each of them, but they generally don't work. Sites that appear obvious on a map, when you look into the detail, don't work. There was another site we looked at that people were suggesting was very suitable, but it's an old service station with noted soil contamination.

Ms WHITE - Are the owners okay about the acquisition?

Mr GREGORY - I think the owners have made their views on the acquisition quite clear.

CHAIR - I have had a couple of members ask for a rest break for five minutes.

Mr ROCKLIFF - That sounds fantastic.

CHAIR - That's what I thought too, so we'll stop the broadcast for five minutes and come back.

Mr ROCKLIFF - Can I just clarify an answer I made today if that's all right?

CHAIR - How about you do that when we come back?

Mr ROCKLIFF - Okay, sure.

The committee suspended from 5.03 p.m. to 5.12 p.m.

Mr ROCKLIFF - I will clarify a statement I made earlier today in relation to an Indonesian trade mission by Ms Ogilvie. The advice I provided earlier today at the table was that Ms Ogilvie replaced the Defence Advocate, who had to withdraw from the mission due to a family matter. While it is correct that the Defence Advocate had to withdraw due to a family matter, the approval of Ms Ogilvie to co-lead the mission occurred a number of days prior and this was signed off by Mr Barnett, who was acting Trade minister at the time in

mid-October 2022. The Defence Advocate having to withdraw from the mission meant that Ms Ogilvie led the delegation.

We are advised that the primary reason for the high cost of fares was the late confirmation of dates for travel due to a unique opportunity arising for three Tasmanian businesses to host exhibition stands on HMAS *Adelaide* at the Indo Defence 2022 Expo & Forum - Australia's flagship industry and defence event. This event offered the opportunity for significant exposure for Tasmanian maritime and defence products endorsed by an Australian Navy event to an audience of our regional strategic partners, adding significantly greater value to the mission. It was only at the end of October that this offer was finalised with Navy and the Department of Foreign Affairs and Trade (DFAT). That, unfortunately, held up the booking of flights for those attending.

I have some questions on notice responses, Chair.

A state demographer - we have been scoping requirements for the state demographer -

Ms WHITE - I suggest that the Premier table these so that we don't eat into the Health Estimates?

Mr ROCKLIFF - Okay.

Ms WHITE - Thank you for providing them but I think unhelpful.

Dr WOODRUFF - I have some questions on paediatric COVID-19 that may or may not be appropriate for Dr Veitch. It might also be someone at the table who answers them - Prof Lawler?

It's in relation to some research published by the Journal of the American Medical Association Network Open that suggests children who acquire COVID-19 infection are at increased risk of type 1 diabetes development. Some additional research in the Journal of Paediatrics suggests there's been a significant increase in paediatric diagnoses of type 2 diabetes since the start of the pandemic.

I expect Tas Health keeps data on the incidents of diseases like diabetes, hepatitis and heart conditions in different cohorts.

Are data available on the incidents of these diseases in paediatric patients before and after the onset of the pandemic? Have we identified any increase or spike in childhood diabetes, hepatitis or heart conditions since the onset of the pandemic?

Mr ROCKLIFF - Thank you, Dr Veitch. Perhaps we could commence the answer to the question from yourself.

Dr VEITCH - I am aware of the paper that's been published on increased incidents of diabetes in young children in America, which I think has come out in the last month.

I'm aware of the paper that was published about the increased incidents of type I diabetes in young children that's been possibly attributable to COVID-19 infection. Bearing in mind that almost all young children will have had COVID-19 infection.

I am not aware of the mechanisms in Tasmania for surveillance for new onset diabetes among young children. It would be a question that we would need to seek advice from the paediatricians on their surveillance of that.

Dr WOODRUFF - We don't collect data on the incidents of diseases like diabetes and hepatitis by age cohort?

Dr VEITCH - I don't know what the mechanisms are. Neither of those are notifiable diseases under the Public Health Act, but they're certainly significant diseases and I think I would seek advice on that from the paediatricians.

Dr WOODRUFF - Could you table that advice? How would you seek advice, through you, Premier? You mean you might do this as a formal mechanism.

Dr VEITCH - It is not something I plan doing, to be honest. Surveillance in the management of paediatric diabetes is not normally within the province of public health surveillance. The first way to understand that would be a discussion with the paediatricians.

Dr WOODRUFF - That's why I thought it might be someone else at the table who could answer that question. Thanks Dr Veitch.

Mr ROCKLIFF - Perhaps Professor Lawler could?

Prof LAWLER - Through you, minister, I would think, Dr Woodruff, that it's probably worth noting that we do capture clinical and operational activity within the public health system, which is predominantly to do with presentations with disease states rather than incidents within the community.

As Dr Veitch has highlighted, there is a lot of information and data collected on specific notifiable disease of which neither of those currently stands as a notifiable disease. I think what Dr Veitch describes about gathering and understanding from local paediatricians, it is important to note that a lot of provision of such care is also occurring within the private sector which we don't necessarily capture under our report arrangements. We do code for presentations within the public sector and health activity on that basis, which is, as you would appreciate, different from incidents within the community.

Dr WOODRUFF - Just to clarify, it seems that in the state and nationally we don't collect data on the incidents of diabetes. I thought we might have done that. That's a concern.

Do we collect information on RSV and streptococcus A? Has there been an increase in the incidences of those since the start of the pandemic?

Dr VEITCH - Yes, Dr Woodruff, there has been an increase in the incidents of both those diseases, but that's because they only became notifiable diseases last year. So, we don't

have a solid time series, but we have a sense of the usual incidents and prevalence of RSV through our laboratory-based surveillance. It hasn't seen to be unusual in the last couple of years. RSV did continue to circulate in Tasmania even when, for example, influenza didn't in 2020 and 2021.

Dr WOODRUFF - My final question, probably for Dr Veitch. There is research evidence that the coronavirus can increase miscarriages, stillbirths and premature births. At least, there is suggestive evidence of that. Do we have any data we collect in Tasmania, albeit it would be a small number, on miscarriages, stillbirths and premature births? And do we have that information pre-COVID-19 and by year since COVID-19. Are we collecting that information?

Dr VEITCH - Again, I would probably have to ask what's available in the hospital system. We do have a Council of Obstetric and Paediatric Morbidity and Mortality, which is a longstanding system. I can't here and now remember the particular details that are included in that report. But that will be the first thing to turn to, and that's publicly available.

Mr ROCKLIFF - We could ask the chief nurse, Ms Douce, to come up.

Dr WOODRUFF - That would be really helpful. Thank you.

Mr ROCKLIFF - Perhaps you would just like to repeat your question.

Dr WOODRUFF - Hello, Ms Douce. My question was in relation to miscarriages, stillbirths and premature births, with suggestive evidence at the moment that the coronavirus can increase the risk of those events. Do we collect data on those miscarriages, stillbirths and premature births? And if so, is there a time series, say, of the last five years in Tasmania?

Ms DOUCE - The perinatal database that we use in Tasmania is Obstetrics Tas. For all pregnancies coming into our system and, indeed, any maternity service or private service within the state, that data is captured.

In terms of miscarriages less than 20 weeks, I'm not 100 per cent sure that we capture that, but certainly anything over 20 weeks is captured, particularly the period of gestation in terms of premature birth and stillbirth are absolutely captured through that database.

Dr WOODRUFF - Can you provide that information for the last four years, please? Probably five years.

Ms DOUCE - That would actually be captured through the council of perinatal, maternal and morbidity committee report, and that's reported annually. I think that the most recent report would be 2021.

Dr WOODRUFF - Can I put that on notice to get that information from that report?

Mr ROCKLIFF - Yes, I believe it's publicly available but, of course, if you would like to put it on notice.

Dr WOODRUFF - Thank you, I appreciate that.

Ms DOW - Premier, will you make the 40 COVID-19 positions at the Launceston General Hospital for essential support staff permanent to provide them with employment security?

Mr ROCKLIFF - We've had some discussion around the circumstances of the 40 personnel. Perhaps I could throw to Mr Webster on this one.

Mr WEBSTER - Ms Dow, what we are looking at is a move back to business as usual at the Launceston General Hospital, in the, if you like, post-pandemic. Some of those jobs will be made permanent. But they might be slightly different jobs because we are looking at the workloads, particularly around these are support roles mostly in our wards. And, of course, our configuration of wards and our number of patients in wards, all of those things have changed. We are working through a consultation process with our staff about what it will look like into the future. We've engaged with the staff, I think, in one meeting with their union representatives present. We've gone away to look at their feedback. It will probably result in a reprofiling. With the increase in staffing, we may need team leaders, we need to look at the skill mix because it was a rush to put it back in.

So, the short answer is we're not saying we won't make them permanent but we need to work through the detail to make sure we have got them in the right spots because our wards have changed over that period.

Ms DOW - Given that their contracts are complete on 17 June, that gives you 12 days to get these things in place. What is going to happen come that date? You are obviously looking at it but, really, I am asking from the Premier for a commitment to make sure those people do not lose their jobs come 17 June.

Mr ROCKLIFF - We are working through the matters.

DR VEITCH - We have given an undertaking to the relevant union that the status quo will apply while we are doing this review. As I said, it is not a case of they might get exactly the role they got under COVID-19 but we are working to figure out what the structure needs to be because the wards have changed at the LGH and the workload has changed at the LGH. So, we are working through that. We will keep the status quo while we are going through that process.

Ms DOW - I think they might argue that the workload hasn't changes or decreased, and that there is still quite a lot of COVID-19 in the hospital and around, and that the demand is increasing. But it is good to hear that they are not going to lose their jobs come 17 June.

Premier, robotic surgery is minimally invasive keyhole surgery which produces better outcomes for patients and is available at most major centres in Australia as well as the rest of the developed world. Surgeons are now undertaking their training on these robots. Neurological, general, gynaecological and some ENT surgery will only be performed robotically in the future and where the future is now.

Premier, are you aware of the need for a Da Vinci robot at the Launceston General Hospital? Have you been approached to fund this critical piece of technology? And why isn't funding in the state Budget for this technology?

Mr ROCKLIFF - I recall meeting with the College of Surgeons about this particular matter and listened to their representatives intently in terms of their advocacy for such infrastructure. Mr Webster, do you have some insight into where that is at?

Mr WEBSTER - Dr Stuart Day, our clinical director in Launceston, has spoken to me and he is exploring what it looks like for the LGH. There hasn't been a formal proposal put forward because he is in that stage of looking at, 'what could we do, what is it that we need?', those sorts of things. But we are aware and I have spent some time with Dr Day and the CE at the LGH looking at where would it be in terms of, and then feeding that back into master planning, et cetera. What I would say is that for the LGH, we are at the very early stages of exploring it and it hasn't come through as a formal proposal to Government at this time.

Ms DOW - Would there be the provision in the funding that is allocated for your elective surgery, increasing funding across the forward Estimates in the Budget, for the provision for that? Would that be able to be funded as part of the equipment that is part of that funding commitment?

Mr ROCKLIFF - I will have to seek advice on that, Ms Dow.

Ms MORGAN-WICKS - Noting that robotic surgery is a new technology request, if that is going to formally come through, I, as secretary, certainly haven't received a formal request that I am aware of. I am happy to receive a request in relation to new technology. But we do need to be very careful in terms of the implementation of new technologies across our hospitals. They do need to be considered and, I note from the Chief Medical Officer's perspective, in terms of the appropriate use, piloting, trialling and evaluation. We would also need to consider that not just at the Launceston General Hospital but also at our Grade 6 tertiary hospital site here at the Royal. Would it also apply into the new surgical upgrades, for example, that we are also putting into other hospitals?

I do not know if the Chief Medical Officer would like to comment in relation to new technology approvals. But it is not as simple as just a request coming from our head of clinical surgery. It would need to be formally evaluated and approved.

Prof LAWLER - To add to the secretary's comment, it is important also to not view these technologies in isolation. We do have a process for assessing, approving and also monitoring the introduction of new technology. This is true whether it is a new IV cannulation device that costs a \$1.20 or a robot that can run to the millions.

The challenge is also that it sits within the context not only of that device but it also needs to be supported by the infrastructure and the team around it. Individual practitioners using it need to have the appropriate level of training and a credential. Their scope of practice is associated with that.

All of those things - the initial outlay, the operational expenditure, the clinical and quality controls around it and the training and support required for clinicians using it - need to be part of a formal business case for which there is then an appropriate cost benefit analysis. This can go to the primary end points such as retention of appropriate function and return to work or good function at three to six months. It will also go to elements such as length of stay in hospital, complication rates, bleeding rates. They're all looked at in order to analyse whether the investment that goes to the initial outlay and the ongoing management provides the benefit that Tasmanians need.

Ms DOW - Premier, have the theatre lights that are held together with gaffer tape at the Launceston General Hospital been rectified?

Mr ROCKLIFF - The way you've expressed that is being 'held together'.

Ms DOW - They're disintegrating, Premier. That's the problem.

Ms MORGAN-WICKS - Through the minister, I think Shane can comment on the order in which we've been investing in upgrades in our theatre. To confirm for the record, the deterioration in the theatre lights, we understand, has been caused by the stronger infection prevention and control materials that are being used to clean the lights. We noticed that deterioration and we sought IPC advice in relation to the proper taping and use to protect but note that these lights need to be cleaned to meet IPC controls.

I think Shane can probably provide Mr Gregory some further advice about the lead time to replace the lights. We have ordered them but I understand that there is a 24-week lead time. It is our intention to replace them. The cause of the deterioration was not age or wear and tear, it was due to the cleaning regime which was newly instituted as part of COVID-19 in our theatre precinct.

Through the minister, Mr Gregory can you provide an update on the ordering of those replacement lights, noting that we have already replaced in two theatres?

Mr GREGORY - Through you, minister, we've replaced the lights in theatres one and two as part of the broader refurbishment. That was always planned and it was underway anyway, so those lights were on order. After consulting with the surgical staff and the maintenance team at the LGH to determine what were the best lights, we have put an order in for the remainder of the lights. They come out of Europe and there's about a 24-week lead time for them to arrive. They won't take long to install once they're here.

Ms DOW - Through you, Premier, was that order placed after the media story about that or were they on order prior to that happening?

Mr GREGORY - Through you, minister, I believe they were already on order prior to the media story.

Ms O'CONNOR - Premier, as you know, the Tasmanian Law Reform Institute report, *Legal Recognition of Sex and Gender* was published in June 2020, three years ago. Recommendations seven, eight, nine and 10, relating to consent for surgeries to modify innate sex characteristics, have not been addressed by your Government.

In a letter to Amnesty International, you said you were advised, and I quote from the letter -

... that no surgeries to modify the sex characteristics of children are performed in Tasmania. We understand commonwealth health data from Medicare codes and sourced from the Human Services and Health website on procedures conducted in Tasmania indicates that at least 19 surgeries were conducted here from July of 2018 to July last year on children aged 14 and younger, which would be characterised as surgeries to modify innate sex characteristics of children.

Premier, can you confirm whether surgeries to modify innate sex characteristics are in fact performed in Tasmania on some children who have variations of innate sex characteristics?

Mr ROCKLIFF - We are committed to supporting the health and wellbeing of LGBTIQ+ Tasmanians. Our Government believes that all Tasmanians have the right to be treated with respect and have access to high quality health services regardless of where they live and their circumstance.

As I also hold the portfolio of Mental Health and Wellbeing, I understand the sensitivity of these matters and the deep impact that the history of these issues has had on the LGBTIQ+ community.

Our Government values the research and analysis undertaken by the Tasmanian Law Reform Institute. I thank them for their continued work. Given the complexity of matters addressed in the TLRI's 2020 report on the legal recognition of sex and gender -

Ms O'CONNOR - No, this about children with innate variations of sex characteristics and surgery to assign a single sex or gender to them. You might have been handed the wrong brief.

Mr ROCKLIFF - No. At my request the Department of Health has undertaken detailed analysis of the report's recommendations regarding surgical intervention to alter the sex characteristics of children with innate variations of sex characteristics. I've asked our department to examine these matters and provide comprehensive advice on the action required to respond. Our Government is committed to making decisions on the basis of expert advice with a priority focus on best practice and person-centred care.

Consideration has been given to the scope of issues related to relevant medical interventions, Tasmanian jurisdictional arrangements and the needs of the Tasmanian families. We will work collaboratively with community and clinical stakeholders to ensure that any changes to current arrangements are made to advance health and wellbeing outcomes for Tasmanian families. I will have more to say on this matter.

Ms O'CONNOR - When. With respect, Premier, the question was can you confirm whether surgeries to modify innate sex characteristics are performed in clinical settings in Tasmania? This is in the context of your letter to Amnesty International where you stated

unequivocally that they were not, and then Medicare codes which indicate they in fact are. The children's rights are being breached because there are surgeries being undertaken on children with an innate variation of sex characteristics.

Mr ROCKLIFF - Okay, thank you. I did, on advice, write back to Honourable Meg Webb MLC to a very similar question. The answer I gave to that question is -

There is no concrete data about the prevalence of babies born with intersex variations in Tasmania. National estimates of prevalence of intersex conditions vary, though it is estimated at around 1.6 per cent of all births, noting that intersex traits may be identified at birth, during adolescence, or later in life.

Most infants born with intersex variations do not require surgical intervention, however, some babies are born with intersex variations which, if left untreated, can lead to serious health issues. This includes conditions such as congenital adrenal hyperplasia, which may be fatal if untreated. Conditions like these are considered medical emergencies and it may be necessary to perform surgical interventions on these patients to avoid adverse outcomes.

Paediatric surgery involving the genitals would only be undertaken in a Tasmanian public hospital if it was considered medically essential to restore, obtain or repair vital physiological function.

Ms O'CONNOR - Can I ask, Premier, in the absence of legislation that protects the rights of these children, in line with the UN joint statement on the human rights of intersex persons, the AMA, LGBTIQ+ health position statement of 2021, the Australian Human Rights Commission report about health and bodily integrity, what guidance or framework is in place in public settings in Tasmania's hospital system to surgeons who might argue that this is essential surgery when it may not be? Where's the guidance to surgeons?

Mr ROCKLIFF - I am advised that surgery to address innate variations of sex characteristics for infants and children is very infrequent in Tasmania. There are no formal policies, protocols, guidelines or frameworks guiding decision-making about surgery for minors in Tasmania's public hospitals -

Ms O'CONNOR - Did you say there are none?

Mr ROCKLIFF - other than Elective Surgery Procedures Not Routinely Performed which states gender reassignment surgery is not funded to be routinely performed by the Tasmanian Health Service unless there are exceptional clinical indications for surgery due to congenital abnormalities in children.

Ms O'CONNOR - Can I just check - did you just say there is no guidance?

Mr ROCKLIFF - No formal policies, protocols, guidelines or frameworks -

Ms O'CONNOR - Do you think that's a problem?

Mr ROCKLIFF - guiding decision-making for surgery for minors.

Ms O'CONNOR - Because no one can ask these babies whether they consent. That's the issue; which is why the Tasmanian Law Reform Institute (TLRI) recommended a suite of reforms, potentially including amendments to the Criminal Code.

Mr ROCKLIFF - I am advised, that's what we're working through. I have met with a person with lived experience in these matters as well, so I understand the heartache - to put it mildly - that's caused this particular individual.

Ms O'CONNOR - A final question on this round, Chair. This all sounds a bit casual -

Mr ROCKLIFF - No, it's not casual at all.

Ms O'CONNOR - There's no guidance for surgeons; there are no rules, apparently. There's an assertion you made in a letter to Amnesty, which appears to not be true.

Can you commit to the table today to progressing these TLRI recommendations as a matter of some urgency and, perhaps, start with clinical guidance to your surgeons?

Mr ROCKLIFF - Part of us working on this is meeting with people with lived experience, and I've met with some in the last 12 months.

Ms O'CONNOR - Yes, but there's still no guidance in place.

Mr ROCKLIFF - Dale, do you have anything to add?

Ms O'CONNOR - No, this is a question to you. Will you commit to treating this with the seriousness and the urgency -

Mr ROCKLIFF - I am treating it with seriousness.

Ms O'CONNOR - The report was handed down three years ago.

Mr ROCKLIFF - I am treating it with seriousness.

Ms O'CONNOR - What's the next step? Are you going to engage with your clinicians, your surgeons about this?

Mr ROCKLIFF - I will seek advice on the best way forward for these matters.

Ms O'CONNOR - It doesn't sound like it's really entered your thinking much.

Mr ROCKLIFF - I've met with people concerning this.

Ms O'CONNOR - Whatever.

Mr ROCKLIFF - It's not 'whatever'.

Ms O'CONNOR - Nothing's changed.

Mr ROCKLIFF - I've engaged with people who are advocates for change in this area and I will continue to work through those matters.

Ms O'CONNOR - Are you not prepared to treat it as urgent? You are too busy with the stadium. That's your problem.

Mr ROCKLIFF - Oh!

Ms O'CONNOR - It's true.

Mr YOUNG - Minister, the 2023-24 Budget includes \$15 million for a new, state-of-the-art helipad at the Launceston General Hospital. Can you please outline the progress of these important projects, especially for those in northern Tasmania who require urgent aeromedical assistance?

Mr ROCKLIFF - Thank you very much, Mr Young. It was our Government that established Tasmania's first integrated medical and search and rescue helicopter service, which resulted in quicker response times and ultimately saving lives.

We have established helipads at the Royal Hobart Hospital, the Mersey Community Hospital and North West Regional Hospital, connecting all of our major hospitals.

Services to and from the Launceston General Hospital were temporarily relocated from 1 January 2023, following an updated risk assessment of the Ockerby Gardens Helipad by helicopter operator Rotorlift, and Ambulance Tasmania. Our Government moved quickly to identify a new site for the helipad, with the preferred location on the roof of the Cleveland Street multi-storey car park. The 2023-24 state Budget provides \$15 million for the construction of the new helipad. I can now table drawings of the new helipad site and car park.

The new roof-top location will allow for the construction of a new, state-of-the-art contemporary helipad providing efficient patient transfer to emergency department ICU and theatres. It will also enable operation in more difficult weather conditions. A development application has already been lodged with the Launceston City Council, and I'm also advised that the department has already placed an order for the helipad from the suppliers. We will continue to use the Launceston Airport as the landing site whilst the new helipad is constructed in early 2024.

The new helipad will provide a sustainable long-term solution for the LGH and the northern Tasmanian community. That finalises my answer, Chair; but I have an update or a correction regarding the Bicheno development application - it will be submitted in July.

Ms WHITE - You will submit a DA in July; you haven't acquired the land yet, though. You're talking about your own DA, or the proponent's DA?

PUBLIC

Mr ROCKLIFF - I think I'll work through that. We can still submit the development application even though we haven't acquired the land, which I stated before.

Mr GREGORY - The nature of the compulsory acquisition process is that notice to treat is served and then, within a defined time line, the land comes into ownership of the Crown. The value of the land can still be negotiated and determined after that; but the benefit of a compulsory acquisition process is that the land transfers to the Crown under a defined time line.

Ms DOW - Premier, what date were the new theatre lights at the Launceston General Hospital ordered on?

Mr ROCKLIFF - What date were they ordered on? We'll need to take it on notice.

Ms DOW - Premier, doctors are threatening to take strike action across Tasmania, which is very serious. Can you tell me the last time in Tasmania when doctors went on strike or took industrial action?

Mr ROCKLIFF - I'm unable to provide that answer immediately. We're working with our doctors, our salaried medical practitioners, and of course they're due for the pay rise on 1 July.

Ms DOW - Did you meet with them Premier? They've requested to meet with you.

Mr ROCKLIFF - I'm more than happy to meet with any doctor, because I engage in visits across various health settings-

Ms DOW - But on this particular matter?

Mr ROCKLIFF - But the negotiation with the department at this present time is appropriate.

Ms DOW - So, you won't meet with them?

Mr ROCKLIFF - No. Come on, Anita; seriously.

Ms DOW - I'm just trying to be clear about what your intentions are.

Mr ROCKLIFF - No. When it comes to a teacher agreement, or to police officers or fire service personnel, we have always said we will negotiate in good faith - and we are doing that, and I'll ask Dale to provide an update on that.

Mr WEBSTER - We've been working with the AMA for almost 12 months on a detailed enterprise agreement which is very different to previous ones for doctors. What's pleasing is that 95 per cent of the issues are resolved, which will see quite significant changes to how our doctors in training work and how the system works overall. There remain two or three outstanding issues. We've given them two offers and they have now given us feedback on those offers, and we are working to resolve the remaining issues. We are hopeful of

PUBLIC

meeting with the AMA in the next couple of days, and that session will be led by the head of the State Service to, we think, finally resolve the issue.

Mr ROCKLIFF - I last met with the AMA when this matter was discussed I believe was on 24 May.

Ms DOW - Premier, you've had a series of resignations from pretty important key roles across the Health department, people that have provided years of dedicated services, some more time than others. Are you concerned about that mass exodus from the Health department and the loss of all that intellect across the department? And what are you doing about it?

Mr ROCKLIFF - We are committed to building a health system that meets the needs of Tasmanians now and into the future. I have said many times that people in our health system are our greatest asset.

Ms Dow - They're leaving.

Ms O'Connor - Lip service.

CHAIR - Order.

Mr ROCKLIFF - As of 31 March 2023, there were 30 senior executives in the Department of Health, consisting of 16 males and 14 females. Senior leadership in Health has been incredibly stable throughout the three years of the pandemic.

We are now seeing some movement as positions open up nationally and internationally. It is positive to see Tasmanian health leaders being recognised with competitive appointments as longstanding and respective staff look to new fields to further build their skills and expand their networks, and recognise the opportunity for fresh thinking and renewal that new leaders bring to improving health outcomes in Tasmania.

The secretary has announced the appointment of Paula Hyland as our new chief executive for Hospitals North West. The department has appointed associate secretary Mr Shane Gregory, increasing the senior capability and support across the department. And Jordan Emery has recently been appointed to the role of chief executive, Ambulance Tasmania. I thank all the people that work within our health service and wish the people that are leaving -

Ms O'Connor - The exodus.

Ms Dow - The mass exodus.

CHAIR - Order.

Mr ROCKLIFF - It is not a mass exodus. Talk about inflammatory language. I think it must be getting late in the day for a number of people. We thank them for their service and their dedication in what's been an extraordinary challenging time throughout the pandemic.

Ms O'CONNOR - Minister, I'd like to talk to you about the helipad on the Royal Hobart Hospital roof and the impact that it is having on residents of Hobart. We got right to information data back which shows that between September 2021 and February 2023 - that's about 16 months, there were a total 1224 landings on the helipad. About a third of them, 34 per cent, or 497, are medical emergency. Can you confirm to the committee that the 727 non-emergency flights cost about \$17.5 million?

Mr ROCKLIFF - I will have to seek some information for you on that.

Ms O'CONNOR - Perhaps there is information available which can tell the committee how much these helicopters cost to fly an hour.

Mr ROCKLIFF - Perhaps I better ask Mr Emery to come back to the table and talk about medical retrieval.

Ms O'CONNOR - We're very interested in the number of non-emergency flights that are coming in over residential areas.

Mr EMERY - Ms O'Connor, obviously, a portion of our ambulance flights by helicopter are non-emergency cases. I don't have the exact number in terms of the breakdown between emergency and non-emergency flights immediately available. There is a portion of flights that do take place that might not be strictly a life-threatening emergency but they are retrieving a patient from a very remote location across the state that might have significant injuries that require transport to the Royal Hobart Hospital for tertiary care, even if the patient is not in an imminently life-threatening state.

Ms O'CONNOR - Thank you. Our data says that a third of those flights, we have not yet established what the hourly cost is so perhaps that could be answered, 34 per cent of the flights are emergency and medical services flights, and 28 per cent search and rescue flights. Assuming that they are serious enough for a helicopter, that's fine. There is still about 30 per cent of the flights that come in over residential areas that would appear to be not for essential medical purposes.

Mr EMERY - Ms O'Connor, I would have to look at the detail further but I would say, given the centralised nature of trauma services in the state, particularly as it relates to the Royal Hobart Hospital, patients are retrieved in what we call secondary transfers, where they may be stabilised at the Launceston General or North West Regional Hospital before being flown down to the Royal Hobart Hospital.

There are also a portion of aeromedical incidents that are primary responses. We might have a primary helicopter response to an incident and the patient is immediately flown to the Royal Hobart Hospital based on the severity of their injuries. There are search and rescue missions as well which might initially be an effort to locate a patient but some 50 per cent of those missions subsequently translate into a patient.

Ms O'CONNOR - We have correspondence and feedback from people who live along the flight path. In the original documentation from TasHealth, the K-Block helipad, it said, we could expect one or two retrievals a day. Now it is three or four retrievals a day and

landings on the helipad. Why has there been such a costly intensification of the use of helicopters at the Royal Hobart Hospital?

Ms Dow - Because all the ambulances are ramped.

Ms O'CONNOR - The ambulances are ramped, yes. I have another question on this.

Mr ROCKLIFF - As I understand it, it is a clinical decision that is made.

Ms O'CONNOR - Is it a clinical decision, though?

Mr EMERY - Ms O'Connor, I would say the total number of aeromedical missions over the last three years, there is 762, this is not unique to Royal Hobart Hospital, sorry, I should say - 762 in 2019-20, 833 in 2020-21, 777 in 2021-22 and 662 through until 31 March 2023. It is predominantly clinical in terms of transfer to tertiary-level care, especially as it relates to secondary aeromedical transfers. But, of course, there are operational circumstances that might dictate the aeromedical transfer of a patient.

Ms O'CONNOR - I am happy to put it on notice. What is the hourly cost of those flights?

And, minister, are you able to explain why the flight path now takes three to four helicopters a day over residential areas in North Hobart, New Town and Glebe? And whether that is a decision that was made because it was easier to do it? Remember, minister, they used to come in down the river and land at the Domain. And now three or four flights a day are flying low over residential areas to land at the helipad. They cannot all be emergency flights.

Mr ROCKLIFF - CASA (Civil Aviation Safety Authority) springs to mind but maybe that is not the correct answer.

Ms O'CONNOR - There is a question about why the flight path now takes all these flights low over residential areas.

Mr ROCKLIFF - We would have to do an analysis to get that information.

Ms O'CONNOR - It's in an RTI that's on the public record. If you haven't analysed it in time for Estimates -

Ms MORGAN-WICKS - In having a look at the data that's been provided, Ms O'Connor, through the RTI, that has been the case since 2021. I haven't got the formal analysis to say in terms of increase or not. Certainly, back to 2021 we can see instances where there are four flights or higher a day; but also many days in which there's one; so it hasn't just occurred within the 2023 time frame, for example.

Ms O'CONNOR - Thank you, Ms Morgan-Wicks, that's the data that we have, and, Chair, if I could pursue this for a little bit longer. Do you want me to put on the record the question about the hourly cost of those helicopters? We would also like to know about the total cost of those contracts because, from looking at the data, it looks to us as though - because contracts have been entered into - there's now a default to using helicopter transport,

which is exponentially and significantly more costly than other forms of patient transport for non-emergency transport.

Mr ROCKLIFF - Thank you, Ms O'Connor. Mr Webster?

Mr WEBSTER - The hourly cost for the Bell helicopter, which is the larger helicopter, is \$ approximately 4700 per hour. For the smaller BK helicopters - we operate two of those - the cost per hour is approximately \$2700,.

Ms O'CONNOR - Thanks for your patience, colleagues at the table. I'm keen to understand this, because we still haven't got to the bottom of it, why the flight path was changed once the helipad went in so that all the choppers now come in down Campbell Street, low over residential areas, and I don't believe any noise-testing was undertaken on the impact on residents; perhaps you can disabuse me of this notion?

Mr WEBSTER - The flight path is determined by wind and all of those sorts of things and it was determined by experts. There are very big differences in wind from the high-rise in the middle of the CBD versus the Regatta Ground which was the previous landing site, so we have taken advice on that. There is a study on the noise effects and I am pretty sure that has been provided through RTI to Dr Woodruff in recent times.

To your point about the non-urgent - I take the point that the number of flights is lower this year than it was in 2018-19.

Secondly, there will be some of those non-urgent flights which are returning aircraft from the north or north-west that have been despatched and, for emergency and operational reasons, they will do a transfer back to the south or from the north-west to the north.

Ms O'CONNOR - With respect, and final question: do you think it's possible we're getting more flights from the north down here because for 18 months the Department of Health ignored Rotorlift when they said 'your existing helipad is not useable'. So, there's a delay there or a lack of capacity at the LGH because your department, Premier, basically did nothing for 18 months and now we're getting more landings down here at the Royal? That's true, by the way. I know it's true.

Mr ROCKLIFF - I beg to differ. Professor Lawler?

Prof LAWLER - There are a couple of things to note as well. There have been increased presentations to emergency departments across the state, and that's a noted trend across the country.

Ms O'CONNOR - COVID?

Prof LAWLER - Yes, pre-COVID. There will be both pre-COVID and comparing COVID-increased presentations and proportionally there will be an increase in the number of patients who need to be brought down to the Royal. They can be for time-critical situations that can only be managed in the south of the state, such as cardiothoracic or neurosurgical conditions or some complex vascular conditions. It's also worth noting that one of the initiatives that's occurred over the last 12 to 18 months is the development and

operationalisation of the state Trauma Service which operates out of Hobart, and so patients who have significant trauma are more frequently transferred to the Royal in order to obtain and benefit from that higher level of care.

In addition to Mr Emery's comments, there are patients who not only are transferred on emergency basis via rotary mode of transport because of the time issues; there are also those who are being supported and transported from rural settings to ensure that they are being escalated to the appropriate level of care.

Mr YOUNG - Minister, in response to evidence heard at the commission of inquiry, the Government established a child safe governance review, to be informed by an independent governance advisory panel. Late last year, the report from the independent panel was released with 92 recommendations made. Minister, can you provide an update for the committee on the progress of implementing these recommendations, ensuring the safety of children and young people in our health facilities?

Mr ROCKLIFF - The Government review commenced in August 2023 [? 2022? TBC 6:06:00] and was undertaken by a governance advisory panel, established under section 13 of the Tasmanian Health Service Act 2018, with Adjunct Professor Debora Picone and Adjunct Professor Karen Crawshaw appointed as independent co-chairs of the panel. The panel also included union and staff representatives, as well as independent experts with qualifications in governance, child safety, hospital administration and human resources. The panel's terms of reference were specifically to examine the governance structure of the Launceston General Hospital and human resources through a child safety lens to ensure public confidence in the safety of the service and make recommendations as to the changes required.

A process for engaging with victim/survivors was established and facilitated by Dr Maria Harries a panel member and expert in child safety matters. Dr Harries had significant engagement with a number of the victim/survivors and their families who agreed to participate in the process. To support the implementation of the recommendations, seven working groups have been formed, based on the themes of the report, to be directly responsible for implementing recommendations. Working groups are chaired by the relevant department of health executive and report progress back to the health executive as the overarching steering committee. [all names checked]

Key actions that have occurred in response to the recommendations include:

- Engagement with the local Launceston community to hear their concerns;
- Implementation of measures to improve staff communication, such as staff forums, distribution of weekly newsletters and engagement with the Consumer and Community Engagement Council;
- Mandating child safeguarding training for all Department of Health staff, including contractors and volunteers, which is to be completed by 30 June 2023. I am advised that as of 30 May 2023, 14 239 staff have completed their training;
- Building on the existing work of a statewide complaints management oversight unit, which has responsibility for the review of reports of inappropriate behaviour or misconduct by our employees;
- The transformation of health HR, including the development of a new HR service delivery model that meets the contemporary needs of the department;

- Refreshing the leadership structure at the LGH, including the appointment of a new Executive Director of Medical Services, a new Director of Nursing for the Women's and Children's Service and a new Nurse Unit Manager for our children and paediatric service.

The former co-chairs of the review are continuing to work with the department in an advisory role to ensure independent oversight of the implementation, along with Dr Harries. The Government is committed to ensuring all recommendations are implemented to improve child safety in our hospitals. I can also table the Child Safety Governance Review update from June 2023, Chair, if that's okay with you.

Ms WHITE - Premier, in an earlier answer to this committee, you indicated that a notice to treat had been issued for the property at 60A Burgess Street, Bicheno. The landowner of that property was unaware of that notice to treat being issued until it was referenced in this committee today, do you think that's a very respectful way to do business?

Mr GREGORY - I'll seek information on that, minister.

Ms WHITE - And further to that - Premier, I understand your department has a meeting scheduled for 20 June with the Glamorgan-Spring Bay Council to discuss this project, where the council, as I understand, was going to propose an alternative site. You've announced today that you'll proceed to acquire the land at 60A Burgess Street and submit a DA in July. Do you regard that as a poor way to engage in consultation with the local government representatives for that area, given they were going into that meeting in good faith, proposing an alternative site?

Mr ROCKLIFF - I understand that we have been engaging extensively with the community on this matter. Any further update on that, Mr Gregory?

Mr GREGORY - We've been involved in a number of conversations over the last twelve months with council. We've talked about a number of alternative sites and I've been involved in meetings with the general manager and the mayor over that period of time, and we've talked about alternative sites. They put some ideas forward, and we explained why we wouldn't be able to use those sites.

Ms WHITE - Are you still going to take that meeting on 20 June with them? Sorry; I don't know if it's you; I should ask the minister.

Mr ROCKLIFF - We always continue to meet with stakeholders, including local councils.

Mr GREGORY - I'll need to confirm the details of that meeting. It's not a meeting that I would have been involved in; that would be our infrastructure team.

Ms DOW - Premier, I want to talk to you about your commitment to 298 new beds across the hospital system. I'm a bit confused about how many of these are actual, physical beds; how many are going to be purchased from the private system; and how many are hospital in the home beds. There's been lots of discussion about these beds, but it's very

difficult to understand where they will be, when they will be delivered, and how they will be funded. So, how many of the 298 beds committed to be opened by 2023-24 are open now?

Mr ROCKLIFF - I will throw to Mr Webster or Ms Morgan-Wicks in a moment. At the 2018 election, the Government gave a commitment to open and staff 298 additional beds, statewide, by 2024. As at 31 March 2023, there have been 231 additional hospital beds opened in our health system since July 2018, comprising: 188 beds in our major public hospitals, with 129 beds at the Royal Hobart Hospital - including the hospital in the home program; 45 beds at the LGH; 14 medical beds at the North West Regional Hospital; and 43 new beds through the private sector partnerships. These investments will help to free-up capacity in our public hospitals. These beds are adding a real increase in the capacity of our public hospital system, including supporting our four-year elective surgery plan.

There are further planned bed openings by the end of June 2023, with the opening of three new sleep study beds at the Royal Hobart Hospital and a further two older persons mental health beds at the Hobart Clinic. In addition, an arrangement to access six beds at St Helens Private as part of the public-private partnership fund will cease. This is in an environment where we are boosting mother and baby services at the Royal Hobart Hospital, and opening new beds in the community, and at the recently re-opened Peacock Centre.

Based on these changes, we expect that 230 additional hospital beds will open as at the end of June 2023, across the four major public hospitals and through public-private partnerships. Over the following 12 months, the THS plans to open an additional 68 beds comprising: 42 beds for the Royal Hobart Hospital, with eight emergency medicine unit beds, 30 new sub-acute beds, 2 new ICU beds, and 2 new cardiology beds; an eight bed hospital in the home service for the LGH; seven new medical beds for the North West Regional Hospital; and 17 virtual-care beds statewide.

In addition to these permanent openings -

Ms DOW - Premier, the commitment that you made at the time gave the impression to the Tasmanian community that these would be physical beds. That they would be new beds that would be opened across hospitals. Clearly some of them are private hospital beds and some of them are Hospital in the Home and some of them are virtual beds. How many physical beds?

Mr ROCKLIFF - They're still physical beds.

Ms DOW - But surely you must understand what I mean. People would have been given the impression that they were going to be all these wards full of beds. That's just simply not the case. It's semantics.

Mr ROCKLIFF - Well look at the TASU, we've opened up 24 beds there, if my memory serves me correctly, 38 beds at ward 3D in Launceston General Hospital. We are opening significant numbers of beds. In addition to these permanent openings, 10 new general medical beds will be opened to support winter flow before the space is closed to allow A block staff to decant in preparation for construction. Offsetting these beds opening is the planned cessation of the six-bed mental health short-stay unit at the Royal Hobart Hospital as the cohort of patients will transfer to the reopened Peacock Centre.

Ms DOW – The 70 additional beds by 30 June 2024, are they private beds or are they public beds?

Mr ROCKLIFF - Well, I'll just complete my answer. The new beds are fully funded in this Budget and the final type and model of care for each initiative is to be determined with close consultation with hospital clinicians and local management. This will deliver the Government's commitment to open 298 beds statewide. These are fully funded bed openings. There are other movements in beds that happens in our health system so as to meet demand day to day.

Ms DOW - Are they funded in this line item being beds and demand for major hospitals?

CHAIR - Order. Ms Dow can you please let the Premier finish.

Mr ROCKLIFF - Flex beds or same-day beds. There have also been changes relating to Royal Hospital, with a cessation of contracted services at Huon Regional Care and temporary closure at Beaconsfield while staff are recruited for after-hours medical support. The most recent statistics released by the Australian Institute of Health and Welfare showed that Tasmania had 2.93 hospital beds per 1000 in 2021 -

Ms DOW - What do they say about elective surgery wait times?

Mr ROCKLIFF - With 1583 beds at that time, sitting well above the national average of 2.46 beds per 1000 people. It was the highest of any state, with only the Northern Territory having a higher rate. We have continued to open a significant number of additional beds since that report. As a result of the impact of COVID-19, we have invested in new virtual care initiatives, such as our highly successful COVID@homeplus program which has seen more than 33 000 enrollments.

Mr WEBSTER - Through you minister, it is really important that in terms of beds that we actually go to models of care. In Mental Health Hospital in the Home, we have a successful program which is based on a model of care where the medical and clinical input is similar to a hospital but the setting is more conducive to recovery. Similarly, when we look at development of Youth Hospital in the Home for the north west, which is under our CAMS program, it is a very similar model of care. The beds could be in the hospital, but the model of care says it's actually more appropriate.

The other area that we're developing in those spaces over the next period is geriatric hospital in the home, if you like. I don't think it's going to have that term. It's about older persons not coming into the hospital environment but getting the level of care of the hospital environment. That's really important in the older person context where we know that after five days in an acute hospital, the decline is quite rapid. If we can move those hospital beds and provide the exactly the same levels of services, but have it in an environment that is more conducive to recovery then that's a better model of care.

Ms DOW - I understand that. I was making the point that I feel that the announcement by the Government was disingenuous because people thought that they would be physical

beds. I think that's the commitment that you made. That's changed significantly in the time that's elapsed.

Mr ROCKLIFF - No. Our long-term health plan is about the right care in the right place at the right time. And care in the community as well.

Ms DOW - Can you explain to me, Premier, where those beds will be funded from. Will they be funded out of the line item 'Beds and demand major hospitals' in which there are details about them, or will they be funded out of the \$250 million trust account for the Royal Hobart Hospital redevelopment, which is Budget Paper 1.

Mrs MORGAN-WICKS - Through the minister, I will confirm if our chief financial officer has anything further to add to my answer, my understanding is that the significant proportion of funding or the uplift is from the 'Beds in demand' Budget line item. In relation to virtual care beds there is also a separate line item for an increase in virtual care investment.

Ms DOW - Will that partnership funding from the federal government, that \$250 million, be used for those beds, just to be clear?

Mrs MORGAN-WICKS - Through the minister, if it is a reference to the \$250 million that was invested in K block in previous budgets, that has funded -

Ms DOW - There is \$250 million now.

Mrs MORGAN-WICKS - or funded the construction of K block, part of the 231 beds that the minister has just detailed are included in K block.

Ms DOW - A further question before I finish which is related to this item. The funding that is there for beds and demand, how much additional funding is there to respond to the increase pressure that will be on the hospital due to COVID-19 and respiratory illness over winter. For example, what happened in Hobart over Christmas when there were such significant staff shortages and there was a code yellow called at the Royal Hobart Hospital. In previous budgets there has been provision for that increasing demand, what component of this line item is attributed to that? What is attributed to beds?

Mrs MORGAN-WICKS - Through the minister, our strategy around beds had to be changed through COVID-19. It was a very deliberate strategy that was announced publicly in terms of the beds that we would be opening at each hospital site but also in community case management facilities. That was to open every available that we could identify and also safely staff our hospitals prior to the borders opening. We then still had some further bed opening after the borders opened.

Each year we look at that number of beds we have already opened and our future infrastructure plans. We do have changes. For example, at the Launceston General Hospital we have had, over the past 12 months, works to our acute medical unit. Those rooms came offline and we had to try to replace them elsewhere. There are changes in our hospital environment that impact on the exact location of beds and the availability of those beds.

PUBLIC

With that line item we look at the budget every year. We look at the current cost of the beds and every year for the past three or four years we have provided a budget submission through to Treasury on the current costing of the beds we have available and the ones that we are planning to open according to the time line of our infrastructure works.

Ms DOW - Does that include an allocation, through you minister, for increased staffing to reach that demand?

Mrs MORGAN-WICKS - The money you see there in 'Beds and demand' in the main is staffing.

Mr ROCKLIFF - You cannot make the beds without the staff.

Ms DOW - That was why I was asking.

Mr ROCKLIFF - That is why we gradually opened up ward 6 or the TASU.

Ms DOW - It doesn't talk about that in the Budget, it only talks about beds.

Mr ROCKLIFF - If you have the staffing then you are able to open beds, which is why we gradually worked up to the 24 beds from the original, there were probably less than 10.

Dr WOODRUFF - Hobart Airport is being upgraded. It is getting longer runways for flights from the Antarctic and elsewhere. In that period, it won't be useable at night, which means the Royal Flying Doctor Service won't be able to land with patients.

The Government, I understand, will have to use helicopters to move patients around the state instead of using the Royal Flying Services fixed wing planes. What is the likely additional cost to tax payers and the Health budget? How long is it going to go on for? How many extra flights can we expect at the Royal Hobart Hospital?

Mr ROCKLIFF - Thank you. I'll seek some advice on your question in terms of restricted runway access, which is the first part of your question, and the cost of.

Dr WOODRUFF - It is the cost, the number of extra helicopter flights that are expected at the Royal Hobart Hospital.

Mr ROCKLIFF - I see, helicopter flights as a result of fixed wing not being able to land in southern Tasmania.

Dr WOODRUFF - As I understand it, the RFDS lands at the airport and then patients are transferred to the Royal Hobart. And as I understand it, that won't happen and at night they will have to be moved by helicopter. Are you aware of this?

Mr ROCKLIFF - Thank you very much for the question. I am aware of the runway upgrade because that was a commitment of the federal government.

Mr WEBSTER - Obviously aware. But in relation to the exact number through this period, that's hard to calculate given that we don't pre-plan these sorts of flights.

What we will do, and again, whether or not that adds to helicopter flights, generally speaking the helicopter is used for different reasons to the RFDS. They are two different services, if you like. The RFDS is more of a non-emergency retrieval rather than an emergency retrieval, overall. I'm not saying it isn't. We would have to work out, to answer your question, we can average it out and things like that but we will need to look at the detail that you've asked.

Dr WOODRUFF - Thank you. I can expect that you have very good figures on exactly how many flights landed in the last two years at Hobart Airport in dark hours. So, you would have a very good estimate at least of the last couple of years. Have you done any modelling of the cost to the taxpayer in the Budget for this? What is going to happen?

Mr WEBSTER - Obviously, Cambridge Airport takes smaller aircraft as well, so there could be an alternative there. The other side of it is because a lot of these flights are scheduled, there is the possibility of scheduling them to be during daylight hours.

Dr WOODRUFF - Can I put that question on notice about last year's and what your estimates are: have you done an estimate of the number of additional landings at the Royal Hobart Hospital by helicopter at night as a result of that and the cost?

Mr ROCKLIFF - We will seek further information, Dr Woodruff. Place that on notice.

Dr WOODRUFF - Thanks, that will be great.

This is another question. It's about K-Block and the water and lead contamination issue, the conversation about the quality of the drinking water in K-Block. Are patients in K-Block at the Royal Hobart Hospital now able to use taps in their rooms for drinking water supply?

Mr GREGORY - The issue we had during the commissioning of K-Block related to lead and nickel associated with new tap fittings and water sitting in the pipes for a long period of time prior to the commissioning. That was all resolved during the commissioning process for K-Block. I can confirm that we continue testing just as a routine process, testing water for a range of things. I can confirm that 100 per cent of the taps that are designated for drinking water are passing and 98 per cent of the taps that are not for drinking water are passing the standard.

Dr WOODRUFF - In answer to my question, are patients advised that they can brush, drink water, et cetera from taps running in their rooms? Are they given any advice not to do that?

Mr GREGORY - They can certainly use the water for brushing their teeth, rinsing and showering. Where there are any particular taps that are designated as not for drinking water, they are very clearly labelled. They are generally washing taps, generally the taps where medical staff scrub down and so on.

Dr WOODRUFF - How many taps are designated not for drinking or not for using for potable water?

Mr GREGORY - We will check that number.

Dr WOODRUFF - Thank you.

I have a question now about medicinal cannabis. How many patients in Tasmania are currently prescribed medical cannabis products under the Controlled Access Scheme? And what funding is provided within the current Budget for this to continue, to support some of Tasmania's most vulnerable patients?

Mr ROCKLIFF - Thank you, Dr Woodruff. I will get that information for you. I know this is of great interest to you most years.

Our Government made it possible for all Tasmanian GPs to prescribe and all Tasmanian community pharmacies to dispense medicinal cannabis. Like all other Australian states and territories, Tasmanian GPs are required to seek approval from the Therapeutic Goods Administration (TGA) to prescribe medicinal cannabis through an online application form. A significant barrier to patients accessing cannabis products is the cost, as these products are not subsidised by the Pharmaceutical Benefits Scheme (PBS) and can cost thousands of dollars per year. I am advised that medicinal cannabis products are generally not included in national prescribing guidelines recognised by medical colleges and societies. I understand that for this reason, many GPs may be reluctant to prescribe these products, as any benefit may be outweighed by side-effects and costs.

In very specific circumstances, medical specialists in the Tasmanian Health Service can apply for approval to prescribe proven medicinal cannabis products for their patients for subsidised access via the Controlled Access Scheme.

It is possible that additional medicinal cannabis products will be registered by the Therapeutic Goods Administration once evaluated as safe, effective and of sufficient quality. Should additional products be registered by the TGA, they may be investigated for cost-effectiveness and potential funding by the PBS, which would make them more affordable.

To date, 20 applications have been approved -

Dr Woodruff - Currently prescribed?

Mr ROCKLIFF - for subsidised access to an unregistered cannabis product through the Tasmanian Health Service.

Dr WOODRUFF - We have arguably Australia's safest, most effective and highest quality medicinal cannabis-making business right here in the state. It's a great irony that some of the poorest people who need this effective treatment have to pay a really high premium for a product which is made here in Tasmania. No comment about the business and the price they're charging. But are you considering working with the local licensed medicinal cannabis producers to provide a more affordable medical cannabis product for Tasmanian patients?

Mr ROCKLIFF - I wouldn't intervene on that process. My advice is that it's the role of the TGA to work through these matters.

Dr WOODRUFF - Okay, so you haven't had any engagement with them. That's a pity.

Your Government entered into an MOU with the New South Wales Government in 2015. That was a promise to engage in medical research and clinical trial of products in relation to medicinal cannabis. Have you provided any funding or undertaken any further work with the New South Wales Government in relation to clinical trials, medical research to improve effective treatments for Tasmanians?

Mr ROCKLIFF - Thank you, Dr Woodruff. I will seek advice from Prof Lawler. Are you aware of such matters?

Prof LAWLER - Sorry, would you please repeat the question for me, Dr Woodruff?

Dr WOODRUFF - I understand that there was an MOU signed between the New South Wales Government and the Tasmanian Government in 2015 in relation to clinical trials and medical research in general around medicinal cannabis. It was an agreement to engage in partnering on those sorts of activities. I am wondering if there has been any funding or work undertaken with the NSW Government in relation to that.

Prof LAWLER - I'm not aware of that, I'm not aware of any activity that has been undertaken on that basis.

Dr WOODRUFF - Okay. Is it possible to check whether that MOU exist, what's the status of that MOU?

Mr ROCKLIFF - I couldn't tell you the status of the MOU off hand at all.

Dr WOODRUFF - Has it expired? Would you be able to check, please?

Mr ROCKLIFF - Yes, we can check.

Mr YOUNG - Minister, there's been record amounts of investment in health infrastructure in this year's Budget. That investment in a modern health infrastructure helps to build a health system for Tasmania's future. Could you please provide an update on the Royal Hobart Hospital pharmacy redevelopment?

Mr ROCKLIFF - Thank you, Mr Young. I have had the pleasure of visiting many of our hospital pharmacy departments during my time as Health minister and meeting the hard-working pharmacists, pharmacy technicians and other support staff. The last time, if my memory serves me correctly, it was in the Mersey hospital. I am always very impressed with the knowledge and dedication of our entire health workforce, including our hospital pharmacy workforce.

The Royal Hobart Hospital pharmacy is an example of why investing in bricks and mortar is important, as it allows us to further grow our capacity and deliver new and enhanced services. The Royal Hobart Hospital pharmacy has grown substantially since it

moved into its current location 25 years ago, and the facilities required to formulate the ever-increasing volume of medications are reaching the end of their operational life. Many medications, particularly those used for chemotherapy, require formulation into patient-specific doses before they are administered as an infusion. The THS currently requires the majority of its medication formulation to be undertaken by interstate commercial providers. This is higher cost and is reliant on daily air freight from Victoria.

The 2023-24 Budget provides additional funding to deliver a new, purpose-built sterile medication production facility at the Royal Hobart Hospital. This facility will be a contemporary, safe, high-capacity, sterile production unit that is able to meet patient throughput needs for the hospital. This will build substantial on-island capacity and significantly reduce the reliance on mainland manufacturers, resulting in reduced costs, enhanced safety and increased reliability of supply. Currently, delays to daily flight schedules may impact services, including chemotherapy delivery.

I am advised that tenders closed in April this year, with construction due to commence in late 2023 and completion in 2025. As the Society of Hospital Pharmacists of Australia president has said, this funding is one of the most significant funding investments in public hospital pharmacy in a generation. President Tom Simpson says:

This visionary investment will ensure Tasmanians have reliable access to cutting-edge cancer medicines that can only be prepared in a contemporary chemotherapy production centre.

So, thank you very much for your question.

Ms DOW - Premier, I've written to you about my concerns on behalf of my constituents about the long waiting times for people from north west Tasmania to be transferred to Launceston to have a cardiac angiogram done. People I have had conversations with have been very seriously unwell and, in some instances, almost discharged home very unwell had they not put their case so soundly to have further investigations done.

Premier, can you tell me how many people from the north west and the west have died waiting to be transferred to Launceston for a cardiac angiogram?

Mr ROCKLIFF - I am not sure we will have that detail here, Ms Dow.

Ms DOW - I might go to my next question. Given the high rates of cardiac disease across the north-west coast, more so than any other population group across the state, what is your Government doing to make sure that access is ensured for those people who are having to wait long times, which ultimately can have very dire consequences for them and their family?

Prof LAWLER - I am not sure whether Mr Webster would also have some information to add. There are a number of elements that are being undertaken. Firstly, obviously, and we have discussed this previously, the most effective way to address cardiac disease is through prevention and effective primary care. We've already discussed around the work that's underway to increase general practitioner presence within our rural settings. That's through the single-employer model and also through the training of our home-grown rural generalists,

and working with local communities and also with providers of general practice services to ensure that there's effective primary-care cover for rural and regional areas.

It's also important to note that there are significant linkages between the north west and the north, most notably, obviously, between the Mersey Community Hospital and the North West Regional around the transfer of patients as and when they require urgent angiography or cardiac intervention to the Launceston General Hospital. Alongside that, we've undertaken significant policy work to ensure that those transfers are expedited and the patients are able to transit through the Emergency Department as quickly as possible, particularly if they require urgent coronary intervention, including through angiography and angioplasty.

There's also significant conversation occurring between the hospitals and between those regions of the health service around how outreach services can support areas within the north west. That goes to discussions that occur in the service planning space around what services can be extended across the north and north west, and also what services are most appropriately delivered and duplicated in each region.

Mr ROCKLIFF - Thank you very much, Prof Lawler.

Mr WEBSTER - Thank you for this question. We actually are going through a process of reviewing our cardiac services across the state. There is no doubt that we have a very good interventionist model at the LGH. It's that next level down that your questions are about. We need to make sure that we're delivering services there as well. We've engaged Professor Andrew MacIsaac, an expert from St Vincents Hospital in Victoria, to advise us on how we might deal with those, I don't want to call them next level down, but the non-interventionist category of cardio patient. What is the best model of care that we can provide given, as you highlighted, the particular issues, the level of heart disease that's prevalent in the north-west community. How might we best deliver that safely, given that we don't have a Level 5 or a tertiary hospital in the north west. We're expecting the outcomes of that review relatively quickly and we will work to implement that review fairly quickly as well.

Ms DOW - The other point that's been made to me is that people just aren't getting the intervention or the diagnostics that they need in a timely manner either. That was the case that I was referring to. They are very seriously unwell with coronary blockages but that's not being detected in a timely way, which is very serious.

Mr WEBSTER - That's exactly what I'm talking about - that diagnostic level rather than when we know what the issue is. We're doing that fairly well but where we are doing the diagnostics, we need to improve that. That's why the advice from Prof MacIsaac, it is really important for us to follow that through and -

Ms DOW - What sort of time frame do you allocate to that, Mr Webster?

Mr WEBSTER - That review has been underway now for some weeks and we've met with him to get his oral report. But he has undertaken to put that in writing so that we can move forward fairly quickly. We see this as an urgent requirement.

Ms DOW - It is, absolutely. Thank you.

Premier, a few weeks ago we raised the case of Shakoda and her daughter, who was told she'd be waiting four years to see an ear, nose and throat specialist. She is having significant developmental delays, speech difficulties, learning difficulties.

At the time the response provided by the department was that there were opportunities being looked at for children to be flown to Melbourne to see ear, nose and throat specialists. In the time that's lapsed since then, how many children have been flown to have the urgent appointments that they desperately require? If not, why not? Further to my question, obviously the specialist waitlist is far too long. You have talked about nurse-led clinics as a way of reducing that and people are getting access to an assessment quicker. When's that going to happen? In the meantime, what do those people do?

Mr ROCKLIFF - Thank you for the question, Ms Dow. We acknowledge the challenges in meeting the significant demand for ENT outpatient appointments and surgical services in Tasmania. There is a shortage of ENT specialists across Australia and Tasmania is not unique in this. In addition to this, ENT locums are also in extremely short supply. These challenges have been recognised at the national level through the Health ministers' meeting.

The Department of Health is taking action to address the waitlists and the waiting times for both the elective surgery waitlist and the outpatient waitlist. A specialist group has been working to identify and develop alternative models of care. They are currently exploring outsourcing options with Victoria. The working group builds upon the work that has already been commenced through the outpatient transformation clinic. Assuring efficient and appropriate migration, assessment and registration processes for internationally trained health practitioners are part of the strategies that under way -

Ms DOW - When will that service be available for those children, Premier?

Mr ROCKLIFF - Supporting international professional recruitment including for medical specialists like ENT surgeons. This includes the independent review of health practitioner regulatory settings, the Kruk review and the Migration review and ensuring the appropriateness of college assessment processes for internationally trained medical specialists and considering local workforce supply where the location and number of college-trained positions -

Ms DOW - When will Shakoda's daughter get the ear, nose and throat specialist review that she really desperately requires? That was my question.

Mr ROCKLIFF - We will address that important matter under the leadership of the health task force. Outpatient transformation funding this year is clinical.

Mr WEBSTER - Through you, minister. The taskforce we have working on ENT, coming up with alternative models of care including the possibility of interstate is happening right now. As the minister said, the outpatient wait list, clinical input, if you like, is funded in this year's state Budget, so we would be hopeful of getting some results fairly quickly through the taskforce input. This is a clinical input to tell us how we're going to deal with this using alternative model such as audiologists, nurse practitioners, those sorts of things. Or can we purchase this interstate? As the minister noted, there is a shortage of ENTs right across

Australia. That is why we have prioritised that task force getting us the advice. It is funded in this year's state Budget.

Ms DOW - So will you have to go through a process of recruitment for your nurse-led clinics? How long will that take?

Mr WEBSTER - Through you, minister. We are in the process of identifying nurses that are already employed to be involved in this. If we do need to recruit, then yes, there is a bit of a lag. We have had a start ahead of the announcement in the state Budget by getting our taskforce together to give us that advice in advance.

Ms DOW - One final question to the Premier relates to maternity services in the north west. Can you provide a date in December when that service will transition back to the Tasmanian Health Service?

Mr ROCKLIFF - The date I have in my mind where that will transition is 4 December.

Mr YOUNG - Minister, everyone would agree that our paramedics and volunteer ambulance officers do an amazing job caring for our community day in and day out. Minister, can you please discuss the investments being made into the critical infrastructure needed to allow our paramedics and volunteers to continue caring for the community?

Mr ROCKLIFF - Thank you, Mr Young. I agree with what you've said about paramedics and volunteers. They do an incredible job. We all back them and support them. Our responsible management of the economy over the past nine years allows us to invest heavily into Tasmania's health system, including Ambulance Tasmania.

We have built on our commitment to deliver 48 additional paramedics across the state with a further 11 paramedics employed between Sorrel and Huonville in recognition of the demand that these stations are seeing. This is just one piece of the puzzle. As you've said, infrastructure is just as important as the staff we have running our services. Our Government is delivering both.

In the last financial year we have seen 21 new ambulance vehicles delivered into service. This builds on the 30 new vehicles commissioned in 2021-2022. Our Government has also committed to delivering several new stations across the state with two new super stations, one at Burnie, which is currently under construction with an estimated completion date of April 2024, and one at Glenorchy. We expect to award a contract for that this month.

We are also delivering a further six stations in rural and regional areas. It was great to be in Bridgewater a few weeks ago at the opening of the new \$2.6 million ambulance station. The state-of-the-art four-bay ambulance station includes training room facilities, improved amenities, improved infection control measures.

We're building more just like these around the state. Currently we have stations under construction in Queenstown and Beaconsfield, both of which we anticipate to be completed by the end of this year. This Budget provides an additional \$5.6 million over two years to progress stations at Longford and Oatlands, with a further station planned for Bicheno. I look

forward to our projects being delivered to improve emergency care to communities across Tasmania,

Mr GREGORY - Through you, minister, I just need to correct some information I provided earlier on the LGH theatre lights. Theatres one and two are part of a broader refurbishment project that was already under way. Lights for theatres three and four were purchased in December and installed in April and approval to expand the scope of works to replace all the theatre lights, that's theatres 5 through 9, was given in February. We've been working with the supplier Drager to have those lights delivered.

Ms DOW - Thank you.

Dr WOODRUFF - Premier, following up on my questions earlier about the extension to Hobart Airport and the -

Mr ROCKLIFF - The runway

Dr WOODRUFF - Yes, the runway and the impacts on the Royal Flying Doctor Service night landings.

Mr ROCKLIFF - I stand to be corrected, but it'd be the pavement, the quality of the pavement rather the extension.

Dr WOODRUFF - Whatever the problem is, I understand that -

Mr ROCKLIFF - Speaking of the pavement, because the runway extension's already been completed. This is about landing potentially international aircraft into Tasmania.

Dr WOODRUFF - Have you had any conversations with the Cambridge airport, which you mentioned before as the alternative to land RFDS flights there? I understand they received a federal government grant to repair their runway due to the impact of heavy firefighting aircraft. Could Cambridge handle RFDS traffic? You probably understand their fixed wing planes can also be reasonably large and heavy. There's also the possibility of an issue with low-intensity lighting at Cambridge airport in terms of its appropriateness for night landings?

Mr ROCKLIFF - I was out at Cambridge Airport the other day with a tourism announcement. The Leader of the Opposition was there as well. I didn't speak about the matters you have raised, with any representatives of the Cambridge Airport at that point in time. It was the first time I had visited the Cambridge Airport. Mr Emery, do you have anything to add?

Mr EMERY - Dr Woodruff, we will work closely with the Royal Flying Doctor Service, as we ordinarily do. We have runway maintenance that happens periodically at Hobart International Airport and we have a number of contingencies in place, including transferring patients by road as well as transferring patients by rotary wing or helicopter. That's not unusual for us. We will continue to work with RFDS on suitable options including the phasing or timing that those transfers might happen.

PUBLIC

To Mr Webster's earlier point, we can focus on some of those scheduled flights not taking place overnight and coordinate arrangements around airport closures as we ordinarily do.

Dr WOODRUFF - Thank you, Mr Emery, that is why we asked the question about the potential extra helicopter cost.

Premier, I have a different question now, it is about people in the north and particularly the north-west and the lethality of jack jumper bites for some people. The jack jumper desensitisation program which is run from the Royal Hobart Hospital, is very effective and lifesaving. It is only available for phase 1 at the Royal Hobart Hospital and that requires a weekly visit, for three months.

After that, the maintenance is available elsewhere. They must be stable on the treatment before the care is transferred to the North West Regional Hospital in the lowest category. Have you investigated, is there any scope for the program to be expanded to allow for people to access that phase 1 at the North West Regional Hospital?

Mr ROCKLIFF - And in the other regions?

Dr WOODRUFF - To the North West Regional Hospital would be good, because there are a lot of jack jumpers all over the state - and bees, wasps and other insects that cause an anaphylaxis response.

Mr ROCKLIFF - The jack jumper anti-venom therapy in Tasmania is improving further, with the program having recently expanded to become a statewide specialist allergy unit based at the Royal Hobart Hospital. Clinics are now available five days per week, and there are 280 patients currently receiving venom therapy in Tasmania. I did see a matter around the north-west.

This initial treatment phase, as you mentioned, has never been available at the Launceston General Hospital or the North West Regional Hospital. As a highly specialised clinical therapy, it is provided exclusively at the Royal Hobart Hospital to ensure the highest standards of patient safety and availability of the necessary clinical expertise.

A jack jumper therapy maintenance clinic is also available at the North-West Regional Hospital when patients are stable on treatment. Generally, after three months from commencing treatment they have the option to transfer their treatment to the North West Regional Hospital. Providing access to a sustainable clinic at the Launceston General Hospital is under investigation, and financial assistance is available for patients from the north and the north-west to access the initial phase of treatment at the Royal Hobart Hospital through the patient travel assistance scheme. Our inability to replicate what is in the south, in terms of the clinical expertise required, to the north-west is preventing us from having a phase 1 service.

Dr WOODRUFF - Thank you, but through you minister, did you say that you were investigating something for the Launceston General Hospital, and is there a time frame for that?

PUBLIC

Mr ROCKLIFF - Providing access to a sustainable clinic at the Launceston General Hospital is under investigation; that is the information that I have. I can seek that information in terms of time frame and exactly what the Launceston General Hospital service will look like, as well.

Mr ROCKLIFF - It's a maintenance service - the same as the north-west.

Dr WOODRUFF - There's no investigation for expanding the phase one capacity?

Mr ROCKLIFF - No, not that I'm aware of.

Mr WEBSTER - This is extremely successful and, yes, it is run out of the Royal - but it's run for the whole state. In the last few years, there haven't been any deaths from jack jumper bite and, as someone with a jack jumper allergy, I can really appreciate that.

Dr WOODRUFF - I have one of them in my family, too.

Mr WEBSTER - It is very successful. It is highly specialised and the Royal runs this service as a statewide service. As the minister said, it's because of specialisation that we wouldn't be able to replicate that in all three regions.

Ms DOW - I didn't think that the service was available at the North West Regional Hospital prior to COVID-19. There used to be a service that was provided years ago.

Dr WOODRUFF - It could've been the maintenance phase, not the beginning phase.

Mr ROCKLIFF - Not the full-on one, I don't think.

Mr WEBSTER - As I understand it, it is the maintenance one, which stopped and then started again.

Time expired.

The Committee suspended from 7.01 p.m. to 7.05 p.m.

MINISTER FOR MENTAL HEALTH AND WELLBEING

CHAIR - We are now entering the Budget Estimates for the ministry of Mental Health and Wellbeing. I welcome the Premier back again with his staff. Could you please introduce the persons at the table, names and positions for the benefit of *Hansard*? And would you also like to make a brief opening statement for us?

Mr ROCKLIFF - Yes, thank you, Chair. With me is Kathrine Morgan-Wicks, the secretary of the department, Shane Gregory, the associate secretary, Dale Webster, the deputy secretary of Hospitals and Pharmacy, and George Clark, deputy secretary of Community Mental Health and Wellbeing.

Our Government has a long-term plan to transform the mental health system, and alcohol and drug services, and to boost preventative health measures for the benefit of all Tasmanians. Central to this delivery is the contemporary mental health and alcohol and drug facilities and infrastructure, and reorienting the way mental health services are delivered to ensure contemporary integrated models of care.

I became Mental Health minister in 2019. We've provided additional funding of more than \$330 million for key initiatives to enable reform in both the community sector and in public mental health, and alcohol and drug services.

In 2020, we kicked off the reforms with \$64.6 million for both child and adolescent mental health and adult mental health. Importantly, we locked in the future funding for 27 new mental health beds at the Peacock Centre and St Johns Park, delivering a new way of operating with a stronger community focus. The year 2020 also saw the commencement of the alcohol and other drugs reform agenda.

In 2021, a further \$109.8 million was provided to kick off a mental health emergency co-response pilot, Mental Health Hospital in the Home, the continuation of the alcohol and drug residential rehabilitation beds, and we locked in permanent funding for the significant child and adolescent reforms, including to enable the development of three new services for our most vulnerable kids.

In 2022, we added another \$71.6 million, locking in the mental health co-response service in the south and providing permanent funding to grow and transform our Older Persons Mental Health Service. With the opening of the Southern Remand Centre, we also provided additional funding to provide more health services at Risdon Prison.

Mental health and suicide prevention have now been elevated to a Premier's priority, a first in Tasmania, and I am pleased to say the 2023-24 Budget ensures we can continue all of this important work for years to come.

Highlights of this Budget include funding of \$48 million over four years under enabling high-quality mental health reforms and associated initiatives to address key areas in the Tasmanian mental health reform program. This includes: the permanent establishment of the acute care team; a general practice liaison clinician; a lived experience workforce and commencement of a broader workforce development team; implementation of the new Tasmanian suicide prevention strategy; and a two-year trial of a mental health emergency response service in the north west.

There is \$24 million provided to permanently fund a dedicated statewide Tasmanian eating disorders service to deliver specialised care and a suite of programs for Tasmanians of all ages experiencing and impacted by an eating disorder. In 2023-24, the Budget also continues our \$7.5 million investment over two years with implementation of the reform agenda for the alcohol and other drugs sector in Tasmania, which aims to reduce the harms associated with the use of alcohol, tobacco and other drugs.

We are well progressed with our \$20.7 million plan to build two new mental health facilities in southern Tasmania. It was exciting to recently open the first of the two, the

Peacock Centre, which provides 12 short-stay beds and a range of other care and support services.

The second facility, at St Johns Park, will include 15-short-stay beds and a similar range of services as the Peacock Center, as well as a \$7.5 million 12-bed eating disorders treatment centre collocated on the site. The tender for this development was recently advertised and construction is expected to commence later this year.

We recognise that preventative health is the key to reducing demand on our state's health system now and into the future. This is occurring through our ongoing commitment to the Healthy Tasmania Five-Year Strategic Plan and our successful Healthy Tasmania Fund. Our investments in this year's Budget are designed to ensure seamless integration between in-patient and community-based services so that people can get the right support in the right place and at the right time. Thank you.

Ms DOW - Premier, have you discussed extending the closing date at St Helen's Private Hospital with Healthscope? And have you tried to delay the closure of St Helen's?

Mr ROCKLIFF - Originally, there was a time frame that was shorter than the time frame proposed. We did request that that time frame be extended by three weeks.

Ms DOW - What was the original time frame?

Mr ROCKLIFF - It was 2 June. We worked with Healthscope to extend that time frame to the now closing date, so it was pleasing that they were able to accommodate our request.

Ms DOW - Have you had any further discussions about extending it further, given the significant change that it will be for people accessing those beds?

Mr ROCKLIFF - Thank you for the question. It is a good one. Mr Webster?

Mr WEBSTER - We had a series of discussions once Healthscope flagged the closure of St Helen's. We looked and obviously that did result in the slight delay. But they had an imperative which was commercial, which we talked through with them. We talked through alternative delivery through Healthscope as well. Unfortunately, they couldn't accommodate that.

Ms DOW - Could you explain a bit more about that, what that alternative delivery would have been?

Mr WEBSTER - For instance, the moving of their ECT operations from St Helen's Private to Hobart Private was one of those discussions, and moving of the mother and baby beds from St Helen's to Hobart Private. But, again, they chose not to do that for their commercial reasons, so we've quickly worked to come up with other ways of delivering the services.

PUBLIC

Ms DOW - Premier, under your contractual arrangements with St Helen's around the brokered beds you've had there in the past, when was the last time the Government had an audit of the facilities done? And what did it say?

Mr ROCKLIFF - Thank you for the question. Mr Webster?

Mr WEBSTER - The St Helen's Private Hospital, as a health services establishment, needs to be, I think the term is licensed. That's done through another part of the department at arm's length from the health service itself so that the licence isn't impacted by the health service. I'm not aware of when the licensing team last did an assessment of them.

In addition to that, St Helen's would need to be accredited by the national safety and quality in health services commission, which is the same as every other facility, both state and private.

Ms DOW - Premier, did you take any immediate actions to try and stop them from closing or talk about perhaps taking over those services, taking over the buildings?

Mr ROCKLIFF - They made the decision to close for their commercial reasons. We, through Dale, negotiated a later closing time, which was at least pleasing. And we've been working on ensuring that as much of the services as possible and that continuity of care of the Mother and Baby Unit will be an example of that. Other services would be provided either in the public system, and in partnership with other providers such as the Hobart Clinic. We have been really engaged with Healthscope and the Hobart Clinic to see -

Ms DOW - Are you concerned, though, about the loss of beds? It's going to have a huge impact for people, not being able to access nearly 40 inpatient mental health beds that won't be available anywhere else across Tasmania.

Mr ROCKLIFF - Of the 31 beds, the occupancy is around 19.

Mr WEBSTER - St Helens Private Hospital is not the treating team of the patients. It's actually the clinicians - the psychiatrists, the psychologists, et cetera. It's really important that those clinicians engage with the patients to work out the pathway for treatment into the future.

Firstly, the cohort of patients at St Helens aren't necessarily the same patients who would come to a public hospital. So, the first thing we did was work with our partners in the health system to see what capacity they had to take up - the minister mentioned the Hobart Clinic, for instance. They have some capacity, so that was there.

We realised from the data - which was deidentified data - that some of the postcodes of the patients were from the north and north-west, so we have engaged with other providers in the north and north-west to alert them to that and see what capacity they have.

Ms DOW - What is the occupation, for example, at the Rivendell Clinic at the North West Private Hospital? What is their occupancy like?

Mr WEBSTER - The last time I engaged with them, they had an occupancy of 11. They have a development application (DA), either before council, or just approved by council - I can't remember now - to expand the capacity of Rivendell to 24. Rivendell specialises in PTSD and trauma, which are both claims that have come through from workers compensation and from private health insurance, et cetera. It is very good that they have that plan for expansion. As I said, that is a live plan and the DA had been lodged with the Burnie Council. I'm not sure but I think it was approved.

Ms DOW - Premier, where are the TMS machines going to go across the state? Will they be available in the north of the state as well, given there's an opportunity to do that now?

Mr WEBSTER - At the present time, before 23 June, there are four TMS chairs, treatment points, in the state. All four of them are located in the south - three at the St Helens campus, and one at the Hobart Clinic campus in Rokeby.

After 23 June, we are fairly certain that we will have four chairs still in the state. The Hobart Clinic is indicating they have increased their chairs to two, and a private provider is looking to provide two. They will continue to be in the south.

However, in the functional briefing that we're doing for both the north-west mental health precinct and the north mental health precinct, the TMS and the capacity to include TMS within the suite of services in those precincts is actually part of our planning.

In addition to that, given the level of uncertainty going forward from the 23 June, we are looking at a safety net program within the public system - us purchasing a chair and having it available. We're working on the clinical model of care that would support that. We haven't decided on a location for that. We're looking at, is four sufficient? Is that the way forward, or if that's not sufficient and not going forward, does the public health system need to be involved in that?

Dr WOODRUFF - Premier, I've listened closely to your responses in parliament to the questions we've asked, and I'm listening to what you and Mr Webster are saying now. I suppose what I hear is you never address the reality that on 23 June, the majority of services that are provided to people – life-saving services to many - will not be available anywhere else in the public health system. They just will not be.

People who are patients, and people who are psychiatrists working there now, they want you to say those words. They want you to be honest about the fact that we're in a desperate situation, and you are there with them and you're open to every option. Why aren't you being honest about the fact that there will not be TMS available for months or years because you can't whistle up the places?

Why aren't you being honest to the two psychiatrists who, between them, have 38 years of experience and maybe 3000 patients, and who have told us, publicly, that they're closing their books because there is nowhere else for them to deliver their services?

Can you be honest and say, yes, this is a terrible situation, we don't have the solutions, we want to hear your ideas?

Mr ROCKLIFF - We're working on the solutions, Dr Woodruff. I have been very honest around the real challenges that this situation has presented. I have spoken many times about it and I'm very concerned. We are extremely disappointed in Healthscope. Discussions are underway about establishing the TMS chairs for the two private providers, including the Hobart Clinic, as I think Dale mentioned.

In the longer term, TMS services are also being considered within the north and the north-west mental health precincts. ECT is a medical procedure used to treat psychiatric conditions and involves the administration of a carefully controlled electric current to the brain. ECT aims to reduce the severity of depressive and psychotic symptoms.

ECT is available within the public health care system in Tasmania, delivered in the Tasmanian health service, and via the Hobart Day Surgery, with the patients under the care of the Hobart Clinic. I'm advised that the capacity of these services, as an example, can be expanded to match any increase in demand.

The Hobart Clinic, I'm advised, are in discussions with private psychiatrists and the Department of Health about the transition of inpatient and day programs from St Helens Private Hospital to their centres in the Hobart CBD and Rokeby.

We acknowledge also that some of the patients who would normally receive inpatient treatment and care at St Helens will not necessarily have the level of acuity typically seen in public inpatient services as St Helens is not a gazetted facility. Their needs and preferences regarding their treatment and care may be better provided within alternative subacute or community-based environments, such as the newly opened 12-bed Peacock Centre, the Hobart Clinic, the Mental Health Hospital in the Home, Detox at Home or other settings.

In addition to these options, a further 27 beds will come online with the completion of a new St Johns Park facility in Newtown in 2025.

Dr WOODRUFF - Please give us the dignity of not parroting things you've already said. We only have another 35 minutes left to ask these questions. We've heard all this before. You know yourself there's a gap between 23 June 2023 and 2025.

Dr John Saul, the head of the AMA, called out the sort of rubbery numbers that you're using on St Johns Park, and he said:

When you look at the 2019 master plan, we were looking at 85 beds being promised for the repatriation development.

When you look at this proposed St Johns facility, all that's mentioned so far is an additional 15 beds and 12 beds for eating disorders, and you've been announcing that every year, I think, since I've been an MP.

What he says is we've lost 31 beds at St Helens and, sadly, the numbers don't add up. When Mr Webster says this cohort of patients are not necessarily those that would come to a public health hospital, I'm telling you, they may well be, because of the desperation of not having services on 23 June.

What are you going to do about it?

I would love to hear you really reaching into the suggestions that have been offered, like from Dr Tim Lane, about getting medi-hotels for some of those mothers who are receiving treatment at the moment. He says it very clearly: they do not need a paediatric ward in a hospital. Have you investigated that option?

Mr ROCKLIFF - We're working on a range of options. We understand the seriousness of the situation, Dr Woodruff. It is a huge challenge and we want as much care to continue as practicably possible. You mentioned Dr Saul, who I have a lot of respect for. He mentioned bed numbers and what he thought may well be the situation. Mr Webster would you update the committee on that.

Dr WOODRUFF - It is a huge loss of beds.

Mr WEBSTER - There is no loss of beds with the move from repat to St Johns Park. Dr Saul has mixed up different options. We were always Roy Fagan to repat, now we're moving it to St Johns Park. Same number of beds. The repat was going to have the mental health in-patient unit from the Royal. The same number of beds will move out to St Johns Park.

The 15 mental health beds and eating disorders beds are in addition to those that are in the master plan and were planned originally for repat, now planned for St Johns Park. We haven't reduced them from 85 to 15. We have moved them from the repat site to the St Johns Park.

Dr WOODRUFF - But they're negative 31 beds that are no longer at St Helen's.

Mr YOUNG - Minister, I understand all health services are accredited against national standards. Can you please outline how staff are working to continuously improve and deliver quality mental health services?

Mr ROCKLIFF - Thank you, Mr. Young. We're committed to ensuring continuous improvement within our statewide mental health services, so the clients are provided with the very best care and treatment and that staff are supported. Part of this commitment to patient-centred care is to ensure we recognise and address any aspect of our mental health services that need additional focus.

Like all health services, the Statewide Mental Health Services is assessed against the National Safety and Quality Health Service. These assessments are an excellent way for SMHS to ensure best practise and the operation of its service and to identify any areas that need more attention. Our Statewide Mental Health Services has recently achieved three-year accreditation. I understand assessors made special mention of the excellent culture that they found within the Statewide Mental Health Services and commended all staff for their positive attitudes and willingness to participate. Staff excellence within the Statewide Mental Health Services will also be celebrated through a new annual award recognising those people who provide exemplary care to consumers and their outstanding contributions to the service.

The Maggie Award has been launched with a call for nominations open across four categories. Nominations can be made by consumers, carers, Statewide Mental Health Services staff and other key stakeholders in recognition of the exceptional efforts of staff members. The award is named in part to honour the late Dr Margaret Tobin, an internationally renowned and inspirational leader in the field of mental health services in Australia. It recognises the symbolism of the magnolia flower representing perseverance, dignity, strength, compassion, dedication and commitment.

Award recipients will be announced as part of the inaugural statewide Mental Health Services Mental Health Innovations Conference and Maggie Awards which will be held in September. I would like to acknowledge the hard working Statewide Mental Health Services staff once again, who are dedicated advocates for their clients. I sincerely thank them for the valuable work they do every day. Thank you for the question, Mr Young.

Ms DOW - How many mother and baby beds will be provided at the Royal Hobart Hospital? We've heard conflicting reports of two or three. What will be the configuration of those beds?

Mr ROCKLIFF - I'll just come to that. I have mentioned this a number of times. It's three and was it 2.7?

Mr WEBSTER - The mother and baby unit is a three-bed unit but it's also a unit that's used for other purposes, such as transition to home. For instance, a baby that has been in the NPICU and we need to work with the parents about transitioning to home, will also go through that unit. There will be a natural triage process that goes on.

I go back a step, there are two categories of mother and baby admissions. One is where the mother is admitted because of mental ill health and needs help overcoming that, but needs to interact with the baby at the same time. The other is where the mother needs psychosocial support around settling the baby, sleep deprivation and things like that.

The unit will be used in a triage sense. What are our highest priorities in the categories that could be admitted? The public hospital use of St Helen's was 0.4 on average. That means one or two every few weeks.

We've based it on that, but we have that extra capacity. We are configuring it as a single room with the baby in the room with the mother. However, it is possible that the baby would be out of the room if that's clinically what's indicated because of the mother needing that time to recover. One of the reasons we have it close to the paediatric unit is it will allow us to manage the mother and baby as two separate admissions on occasions, or a mother and baby as a unit coming into the area. There are a number of choices in that.

Dr Lennie Woo is our state clinical director. He formerly worked at St Helen's in the mother and baby unit. Dr Michelle Williams, our lead paediatrician and Professor Sean Beggs are overseeing the development of model of care. I should also mention Dr Honor Pennington. I've forgotten the fifth doctor from perinatal infant mental health. I'll hear about that later.

Those doctors are putting their minds to making this work. We see this as an interim solution. We need to work with the community to make sure that overall we are servicing Tasmania's total community of mother and baby needs - both those with mental ill health and those where the issues are psychosocial, or using the old term mothercraft issues. Importantly, we have those services in our service suite. We have our parenting centre and our parenting service through the Child Health and Parenting Service, which will have input. We have our Perinatal and Infant Mental Health Service which is very strong in the south. The last federal government put in additional money to that sector to build that up in the primary care sector.

Ms DOW - Are those perinatal services now currently available on the north west and north?

Mr WEBSTER - Yes. We have services in the north and north west. We need to grow them, but they do exist.

Ms DOW - There's obviously a substantial loss of private beds as well. You only have three public beds there, but there were eight. That's a huge loss for parents across Tasmania. You say this is just a point in time and that there's a long-term plan around provision of this service.

Premier, will you commit today to the Tasmanian people that you'll roll out additional mother and baby beds across the state that perhaps are more community-based rather than in an acute setting. We want to see some commitment from your Government that this is not the end of the story, and that those will be available; and they'll be much more equitable for women and families right across the state.

Mr WEBSTER - We are talking about the mother and baby being an interim solution. We need to work with not just our clinicians but the broader clinicians, as you mentioned - Dr Jones who is engaged with us; psychiatrists et cetera - on what this looks like for the state. You are right, the best setting is more community setting than it is our hospital setting. We need to build that, and we need to make sure that's available in the private sector as well. The six or seven months that we have the interim solution in place allows us to work with all the clinicians to come up with what is the final look solution; noting that the primary care sector primarily sits with the Australian Government. We will be working with the Australian Government and Primary Health Tasmania to look at that model of care and what are the pathways.

Ms DOW - Have you made an approach to the federal Government about funding a different model of care in Tasmania, when it comes to mother and baby units?

Mr ROCKLIFF - That may well be a conversation that we will have, given the work on the interim solution. We spoke about a broader solution for the future, with the round table that we had the other day with the AMA, ANMF, HACSU and the Mental Health Council of Tasmania.

Ms DOW - Just on meeting with stakeholders - I understand that a group of GPs has made an approach to your office. I mentioned this in Parliament and you did commit to meeting with them; but to date they haven't been able to secure a meeting with you. They

want to talk about a future model of care for Tasmania, and how general practice might be involved in developing that. Would you give a commitment to meet with them?

Mr ROCKLIFF - It goes with PHT on this matter as well. I have met with the AMA, HACSU, the Mental Health Council of Tasmania and the ANMF; and the AMA represent GPs. We are willing to engage, and we will.

Dr WOODRUFF - Premier, in relation to the St John's Park precinct which apparently is going to be completed in 2025.

Mr ROCKLIFF - No.

Dr WOODRUFF - No?

Mr ROCKLIFF - One stage will be.

Dr WOODRUFF - The eating disorder and the other 15 beds, is that right? Okay. It does sounds as though it is nice; it's got this parkland and green spaces and so on, which should be a nice place to recuperate and heal. However, it has also been promised to Kickstart Arts. Josh Madgwick, who is with Kickstart Arts, has been working for some time now on a trauma history interpretation. You are probably aware that that precinct has a very dark past, dating back to the 1830s. It was the original location of Queen's Orphan Asylum and it housed Aboriginal children and children from poor families, in squalid conditions, between 1833 and 1879; 46 years of horror. Then it housed the New Town Invalid Asylum and was a treatment centre for tuberculosis and people with polio. It has the two historic buildings which, I understand, were handed in 2018 to Kickstart Arts and they were going to do a trauma history.

Can you tell me what's happened to that commitment to Kickstart Arts, and how they say, according to reports on the ABC, that they do not know what is happening with the redevelopment of the heritage buildings that they are going to occupy; they have not been informed of any plans to move them out; they put considerable time and effort into revitalising those buildings. What is happening?

Mr ROCKLIFF - We have committed \$2 million to the master plan, which will engage with all key stakeholders in the community on the various stages, including the eating disorders service. I might throw to Dale first.

Mr WEBSTER - If you are familiar with the site, the 2025 building, that is stage one, is in fact on the upper side above the where the obelisk is; it is on the top side of that. It is a site that had sheds et cetera, which were demolished and it has been cleared now for a couple of years. The DA has gone through the process, and we have gone to tender on that building. It is nowhere near the historic buildings on the site. The closest is a hall that is occupied by the Friends of St Johns; that is the closest building to the 2025 mental health building.

All the other buildings are not buildings, they are coloured blocks on a site, so that we can go through a consultation process about what goes into the future. It is a master plan over a long period of time, but the commitment for the St John's mental health building and eating

PUBLIC

disorders service has been in existence now for a number of years and, as I said, it is well-advanced, to the point of going to tender.

Mrs MORGAN-WICKS - We have a lease with Kickstart Arts in relation to about four buildings on the site, noting that nothing in the proposed master plan vision is impacting on those particular buildings or on the tenancy of Kickstart Arts. We will engage with them as co-tenants in relation to the site and what the impact is going to be. As part of that master plan, we have clearly outlined the significant and dark history of the site; but, importantly, health has had a relationship with St John's Park, with the buildings, and with the property itself for a very long time. As part of this, we are trying to push this forward as a positive redevelopment of the site, that we can invest in and achieve a further 50-100 years use of the site - noting that, at the moment, we believe the use is not optimised.

Dr WOODRUFF - Thank you. There is obviously a crossed wire with Kickstart Arts about their understanding about the announcement that was made about the-

Mr ROCKLIFF - Definitely be engaging with Kickstart Arts, yes.

Dr WOODRUFF - Yes, good. I don't know the history of this, but it is said that more than 400 children have may been buried there at the orphanage. It certainly would have been the case, they would not have been taken very far when they died, in the 1830s. That is a horrific, traumatic part of Tasmania's history that needs to be deeply investigated and respected, and not covered over or in any way ignored in the process.

Mr ROCKLIFF - I could not agree more.

Mrs MORGAN-WICKS - That's why conservation management plans are so critical and liaising with the Friends of the orphanage. We are very aware of the locations of the burial land that is there, and the impacts on the site. That is being taken into account by the architect that we have had involved, who has also been closely involved with the Friends of the orphanage and key stakeholders. We are very sensitive to that heritage, but we also note that this is a site that does need to have its history called out but sensitively shared with the community. It is an ideal location to actually have sensitive trauma-informed care provided for the Hobart community.

Dr WOODRUFF - Thank you. Minister, do you have a time frame for stage 2 and 3 of the development? When do you expect them to be completed?

Mr ROCKLIFF - We will work through that in the master plan process. The idea of investing in the master plan is to do just that, and then stage the redevelopment over a period of time. Hopefully at the completion of the master plan we will have very clear ideas and time frames for stages.

Dr WOODRUFF - Thank you. Do you accept that stage 1 - which, if I am correct, is due to be finished at the end of 2025, you said?

Mr ROCKLIFF - The first part of 2025.

PUBLIC

Dr WOODRUFF - Do you believe there is any risk that this would be completely disrupted with your plans to build a stadium? You have these ideas that you can build a billion-dollar stadium in four years, when you cannot build ambulance stations around the state. It is a heroic idea, and one of the many concerns is that it will suck everything out of the construction sector around the state.

Mr ROCKLIFF - It is a very long bow.

Dr WOODRUFF - Not really. Actually, I think it is a very short bow. Anyone who is involved in construction would know this. Are you confident that construction will be completed on time?

Mr ROCKLIFF - Yes, I am confident. We have some major construction with the Bridgewater Bridge. I mentioned this morning, Dr Woodruff, the infrastructure budget of the last budget was I think \$5.6 billion. This one is just under \$5 billion over the next four years, so I am very confident we will do our absolute best.

Dr WOODRUFF - You can ring-fence the budget- but you cannot ring-fence the trades. You cannot do everything immediately, with a limited number of people in Tasmania, can you?

Mr ROCKLIFF - We come out to tender. Health have had good success to bring on infrastructure, so I am confident. Thank you.

Ms DOW – Premier, I want to ask about the 27 mental health beds that you did promise a very long time ago - I think back in 2018 or 2019. When will they finally be delivered? I know a component of that has been, through the Peacock Centre, but there are still a number outstanding. This is one example of a capital works project that has been pushed out for years.

Mr ROCKLIFF - Thank you. All mental health beds that were planned as part of the Repatriation Hospital redevelopment were delivered at the St John's Park site. Delivery of a 27-bed mental health centre will be the first stage. This will include 15 additional short-stay mental health beds, a safe haven mental health integration hub, and a recovery college classroom. A new 12-bed residential eating disorder treatment centre will be co-located, and will have the capacity for day programs and a consulting hub. Construction tenders are currently being invited for this stage, with construction due to begin later this year, and completion in 2025. The Peacock Centre is open - 15 to go.

Ms DOW - Premier, 2025 is a long time from the original announcement.

Mr ROCKLIFF - If you are talking about challenges with the Peacock, of course -

Ms DOW - No, I am talking about the overall project.

Mr ROCKLIFF - Twelve was part of the 27, is that correct?

Ms DOW - It is. It is the Peacock Centre.

Mr ROCKLIFF - We had that fire on the Christmas Eve of 2021 which was a significant setback. We have only just more recently just opened it up, which is going very well, I understand.

Ms DOW - Another thing you have been talking about for a while is the roll-out of the PACER trial in the north-west. You have announced that quite a few times now. In the state of the state last year, it was meant to be up and running in the first quarter of this year; that time has passed. Can you give me a date when the PACER trial will be up and running in the north west, please?

Mr ROCKLIFF - Sure. A key driver of our mental health reform program is to provide alternative services for people to avoid acute hospitalisation where this is not necessary. While there will always be a need for acute mental health facilities, we are also supporting more community-based and early intervention mental health support options to reduce reliance on hospital emergency departments, which are not always the best environment for a person in crisis or living with a mental illness, as I've said a number of times.

In the 2021-22 budget, we committed \$5.1 million over two years to pilot PACER in southern Tasmania. This is a co-response service involving mental health clinicians, paramedics and Tasmania Police officers. Recognising the early success of this, which has seen some 1800 Tasmanians supported in its first year, with 76 per cent of those able to stay in the community, in last year's budget we allocated \$9 million to continue this important initiative. It is now known as the Mental Health Emergency Response.

As the next phase in evolving to a statewide model, we've committed funding under the 'Enabling high-quality mental health reforms' line item in this year's Budget to commence a two-year trial through to June 2025 of an equivalent service in the north west. Progress has been made towards operationalising Mental Health Emergency Response in the north west. A service-designed workshop was held late last year with key stakeholders from Statewide Mental Health Services, Ambulance Tasmania, Tasmania Police, Flourish, and Mental Health Family and Friends Tasmania. The interdepartmental committee has now approved the model. The Mental Health Emergency Response in the north west will consist of a total of 7.6 FTE, including mental health clinicians located in Burnie, Devonport and secondary triage, as well as a clinical educator embedded in Tasmania Police and Ambulance Tasmania. Recruitment to these positions is now underway and I anticipate this service will be operational in the first half of the 2023-24 financial year, so this year.

Ms DOW - From the first quarter to the first half. We haven't got long to go. It's a great service and it should be rolled out.

Mr ROCKLIFF - It's a great service; thank you. I am very proud of it.

Ms DOW - Let's get it happening in the north west.

Mr ROCKLIFF - Yes, in the north west.

Ms DOW - My other question to you, Premier - and Mr Webster made reference to this in his earlier contribution around mental health in the home beds and the youth beds that are

looking at being established in the north west. You will be well aware of the need for those beds and that service, Premier, through issues that have been raised by the local community, particularly from as far as Circular Head. What does that model look like, when will it be up and running, how many of those beds will there be and how will they be supported?

Mr ROCKLIFF - An extension of the Mental Health Hospital in the Home investment to develop a service for young people aged 16 to 25 years in the north west represents our ongoing commitment to this area and also the provision of greater community mental health supports. The Mental Health Hospital in the Home service has been operational in southern Tasmania since March 2019. The service has improved access to acute care for people who are assessed as suitable to receive care in their home environment, which ultimately has a positive impact on patient flow at our hospitals.

From 2023-24, we are providing \$8.5 million over two years for a youth Mental Health Hospital in the Home pilot in the north west. It will be based on the existing southern service model and modified for the needs of youth aged 16 to 25, drawing on the best-practice models for this cohort from other jurisdictions. In the north west, the youth pilot program will enable young people who may have otherwise been hospitalised to receive intensive short-term mental health intervention and support in their own home.

The Child and Adolescent Mental Health Service (CAMHS) mental health reform program is progressing. This initiative with the north west service plan is to be operational late this year, 2023.

Mr WEBSTER - The CAMHS team leader, the extended team leader to cover this, and the statewide youth psychiatry consultant have been engaged, and they are working on the model of care to implement this. It will build to 12 beds across the north west. It will be a virtual ward of the North West Regional Hospital but fall under the guidance of the statewide specialist clinical director, Professor Brett McDermott.

Ms DOW - Thank you. My last question is around workforce development across the mental health sector. Last year, it was identified that there was a shortage of about 350 professionals across mental health in the state. We don't really go to workforce in the Health Estimates part, but there is no doubt that there is a significant issue around recruitment and retention of the health workforce.

What specific actions are you taking in relation to the mental health workforce around retention and recruitment? And are you looking at any types of incentive programs right across the health system as a way of not only attracting people to work in Tasmania but making them want to stay, Premier, because a lot are going.

Mr ROCKLIFF - This has been a subject that I have engaged on with the Mental Health Council of Tasmania regularly, particularly when I attend their round tables twice a year.

The last four financial years we have seen an increase of just over 105 full-time equivalents in statewide mental health services. The next few years we will see significant ongoing reform arising from the mental health reform program, the alcohol and other drugs reform agenda, child and adolescent mental health reform, and older persons mental health

reform. We're pleased to be expanding our mental health and alcohol and drug workforce for the crucial investment that it is.

To grow the workforce, including the lived-experience peer workforce, there will be continued and concerted recruitment and retention activity, and a focus on workforce development and renewal. Our statewide mental health service continues to explore options to expedite the recruitment process to ensure services are maintained and that reliance on agency staff and locums is minimised. These options include the introduction of a number of developmental positions, such as assistants in nursing, transition-to-practice nurses, entry-level positions for allied health professionals. The service consistently advertises vacant positions interstate and overseas. Statewide mental health has increased the number of transition-to-practice positions by 25, and are filling these positions throughout 2023. There is a dedicated staff member within the mental health reform program who works exclusively on recruitment and provides extra assistance to interstate and international inquiries.

We are also excited to be supporting the growth of our mental health peer workforce, which I have mentioned a number of times. The appointment of the mental health peer workforce coordinator is a joint initiative with Primary Health Tasmania and reflects the importance of ongoing collaboration across the broader mental health sector to create an integrated mental health system.

A key action of the Rethink 2020 Implementation Plan is to develop a joint-workforce development strategy, with public, private, primary and community sectors, along with education and training providers, and this work is under development.

Ms DOW - When will that be completed, Premier?

Mr ROCKLIFF - I'll come to that in a moment. Supporting and developing the workforce is one of the reform directions in the reform agenda of the alcohol and other drugs sector in Tasmania. It has been identified through sector-wide consultations. An alcohol and other drugs workforce development strategy will be included in the AOD reform program implementation plan 2023-24.

We're also investing \$5 million to expand the health hub at TasTAFE's Alanvale site to include a new facility that will deliver training and workforce development for workers in the alcohol and other drugs, mental health and youth sectors as well. I have been advised by the office of the Minister for Skills and Training, Felix Ellis, who is responsible for this work on the TAFE campus, that the design for the hub is undergoing final consultation, with construction expected to commence in the second half of 2023.

I mentioned the Mental Health Council of Tasmania. We have supported them with funding to monitor and collect data on the impacts of COVID-19 in the community mental health sector workforce. They've released their COVID-19 impacts on the community mental health workforce report that's informing the Government's ongoing planning and response to supporting the mental health and wellbeing of Tasmanians.

The focus on addressing the key priorities in the report and immediate actions required from the report include integrated workforce planning to support recruitment and retention, prevention and early intervention measures to address increased service demand, upskilling and diversifying the mental health workforce, fostering and supporting the wellbeing of staff,

PUBLIC

equipping mental health services to respond to ongoing COVID-19 impacts, and data collection and monitoring to inform an effective response.

Mr WEBSTER - Through you, minister, the workforce development strategy, which is a co-piece of work between all the players, is set for delivery in 2023-24. I'd like to add the funding of the peer workforce training hub, which is a joint initiative of PHT and the Tasmanian Government with the Mental Health Council.

Ms DOW - Because currently TAFE's not providing Certificate IV in that are they?

Mr WEBSTER - Not as yet, but this is part of the Allendale expansion.

Time expired.

The Committee adjourned at 8.01 p.m.